

PassReady Prep — NCE Question Bank

Generated: 2026-10-04 · Showing: In review · Total items: 200

How to review

✓ **KEY** marks the correct option. Every option shows why it is right or wrong; check both the key and each distractor.

Mark each item **Approve**, **Revise** (note what to change) or **Retire**. Approved items are published from Admin → NCE Questions.

🔍 **Check** notes mark specific facts or citations the writer was less than certain of — verify those first.

Check: one clearly best answer · factual accuracy (theorists, dates, test properties, code sections) · distractors plausible but defensibly wrong · no answer given away by length or wording · references support the key.

NCE domains (NBCC 2023 content outline)

Professional Practice and Ethics 19 · Intake, Assessment, and Diagnosis 19 · Areas of Clinical Focus 47 · Treatment Planning 14 · Counseling Skills and Interventions 48 · Core Counseling Attributes 13 — of 160 scored items

In this copy

Professional Practice and Ethics **24** · Intake, Assessment, and Diagnosis **24** · Areas of Clinical Focus **59** · Treatment Planning **17** · Counseling Skills and Interventions **60** · Core Counseling Attributes **16** · **39** flagged 🔍

Professional Practice and Ethics

24 items · NCE weight 12% (19 of 160 scored items)

1. nce-s-eth-073 · Professional Counseling Orientation and Ethical Practice · record keeping and documentation · easy · In review

A counselor at a community clinic is reviewing her progress notes before an internal audit. Which entry BEST reflects what belongs in a progress note?

A. A near-verbatim transcript of everything the client said during the hour

Transcripts are excessive and expose more private material than the record needs.

B. Client presentation, interventions used, response, risk status and plan ✓ **KEY**

A progress note documents observable presentation, the service provided, the client's response, risk and the next steps.

C. The counselor's guesses about the private lives of the client's coworkers

Speculation about third parties is not clinically necessary and should not be recorded.

D. A list of the counselor's own feelings and frustrations about the client

Personal reactions belong in supervision or consultation, not in the clinical record.

Rationale: Progress notes should be timely, accurate and objective, recording what services were provided and why, how the client responded, any risk findings, and the plan. ACA B.6.a asks counselors to create and maintain records needed to provide professional services. Excess detail, speculation about others and counselor feelings add risk without clinical value.

References: Wiger (Documentation) — Progress note content: presentation, interventions, response, risk and plan · ACA Code of Ethics — B.6.a Creating and Maintaining Records and Documentation

☐ Approve ☐ Revise ☐ Retire Notes:

A week after writing a progress note in the clinic's electronic record, a counselor realizes she entered the wrong medication dose that the client reported. How should she correct the error?

A. Delete the original note and write a new one dated to the session day
Deleting and back-dating destroys the original entry and can look like falsification.

B. Edit the dose in the original note quietly so the record reads correctly
An unmarked change hides that the record was altered and when.

C. Leave the note as written, since signed entries may not be changed later
Known errors should be corrected; leaving false information harms care.

D. Add a dated, signed addendum noting the error and the correct dose ✓ **KEY**
An addendum corrects the record while keeping the original visible and showing who changed it and when.

Rationale: Records are corrected in a way that keeps the original entry visible and marks the correction with the date and the counselor's signature: a single line through the error on paper, or a dated addendum or amendment in an electronic record. Erasing, overwriting or back-dating can be viewed as falsifying records. Accurate correction protects both care and the record's integrity.

References: Wiger (Documentation) — Correcting errors: late entries and addenda that preserve the original entry · ACA Code of Ethics — B.6.a Creating and Maintaining Records and Documentation

☐ Approve ☐ Revise ☐ Retire Notes:

3. nce-s-eth-075 · Professional Counseling Orientation and Ethical Practice · record keeping and documentation · hard · In review

A counselor saw a couple in joint sessions and also met once with each partner alone. After the couple separates, one partner, a 42-year-old man, asks for a copy of "the whole file." The other partner has not given consent. What does the ACA Code of Ethics direct?

A. Release only the parts of the record that relate directly to him ✓ **KEY**
B.6.e limits access in multiple-client records to the parts that relate to the requesting client, without other clients' confidential material.

B. Release the entire file, since he took part in most of the sessions
The file includes his partner's confidential material, including her individual session.

C. Refuse all access, since couple records belong to the counselor alone
Clients have a right to reasonable access to their records; a blanket refusal is not supported.

D. Release the file only after his former partner reviews and edits it
Letting one client edit the record is improper and does not protect either client.

Rationale: ACA B.6.e states that in situations involving multiple clients, counselors provide individual clients with only those parts of records that relate directly to them and do not include confidential information related to any other client. Setting rules for couple records at the start, including how individual sessions are documented, prevents disputes later.

References: ACA Code of Ethics — B.6.e Client Access

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor promised a client she would call him back with a referral by Friday. On Friday, her schedule is overloaded, but she makes the call before leaving work. Which of Kitchener's moral principles does this action MOST directly reflect?

A. Justice

Justice concerns fairness and equal treatment, not keeping a specific promise.

B. Veracity

Veracity is truthfulness; the act here is honoring a commitment.

C. Fidelity ✓ KEY

Fidelity means being faithful to commitments and promises made to clients.

D. Autonomy

Autonomy is respect for the client's right to make his own choices.

Rationale: Kitchener's principles include autonomy, beneficence, nonmaleficence, justice and fidelity, with veracity often added in later work. Fidelity refers to honoring commitments and being trustworthy, which builds the trust the counseling relationship depends on.

References: Forester-Miller & Davis (Practitioner's Guide to Ethical Decision Making) — Moral principles: autonomy, justice, beneficence, nonmaleficence, fidelity (and veracity)

☐ Approve ☐ Revise ☐ Retire Notes:

A nonprofit agency fills open counseling slots by giving priority to clients with private insurance. A counselor argues that slots should go first to clients with the most severe needs, regardless of payer. Which moral principle is the counselor MOST directly invoking?

A. Justice, the fair allocation of services and resources ✓ KEY

Justice concerns fairness and equitable distribution of services, which is the issue raised.

B. Fidelity, keeping promises made to current clients

No promise to a particular client is at stake in the policy dispute.

C. Veracity, being truthful with clients about services

The concern is how services are allocated, not honesty in disclosure.

D. Autonomy, respecting each client's freedom of choice

Clients' self-determination is not the issue in how slots are assigned.

Rationale: Justice means treating people fairly and distributing services equitably; treating clients differently requires a rationale that is relevant to the difference. Prioritizing by ability to pay rather than clinical need raises a justice concern, and counselors may advocate within the agency for fairer policies.

References: Forester-Miller & Davis (Practitioner's Guide to Ethical Decision Making) — Moral principles: justice as fairness and equitable treatment

☐ Approve ☐ Revise ☐ Retire Notes:

A competent 55-year-old client decides to leave a stable job to open a risky small business. She is not in danger, but her counselor believes the move will cause her great stress and financial harm and is tempted to keep steering sessions toward talking her out of it. Which two principles are MOST in tension?

A. Justice and fidelity, since the client may lose her income

Neither fair distribution nor a broken promise is at issue.

B. Veracity and justice, since the plan affects her family too

Truthfulness and fairness are not the competing values in this choice.

C. Fidelity and veracity, since she has set her goals for therapy

No commitment or truthfulness conflict drives the counselor's dilemma.

D. Autonomy and beneficence, since her choice and welfare differ ✓ **KEY**

Her right to self-determination conflicts with the counselor's wish to do good for her.

Rationale: Autonomy is respect for a competent client's right to make her own decisions; beneficence is the duty to promote her welfare. When a client's free choice differs from what the counselor believes is good for her, and no serious harm is foreseeable, autonomy generally prevails. The counselor can explore consequences openly without imposing her own values (ACA A.4.b).

References: Forester-Miller & Davis (Practitioner's Guide to Ethical Decision Making) — Moral principles: autonomy versus beneficence · ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor holds a full, unencumbered license in her home state, which belongs to the Counseling Compact. She obtains a privilege to practice in another member state to see a client there by video. Whose practice laws govern her work with that client?

A. Only her home state's laws, since the privilege rests on that license

The privilege is based on the home license, but practice follows the remote state's laws.

B. The laws of the member state where the client is located during care ✓ **KEY**

Under the Compact, a privilege holder must follow the laws and rules of the remote state where the client is located.

C. Only federal rules, since telehealth across states is federally run

Counseling practice is regulated by states; no federal license governs it.

D. The Compact Commission's own national practice act for counselors

The Commission administers the Compact; it does not replace state practice acts.

Rationale: The Counseling Compact lets a counselor with an unencumbered license in a home member state practice in other member states through a privilege to practice. While using that privilege, the counselor is subject to the laws and regulations of the remote state where the client is located, and that state may act against the privilege. ACA H.1.b likewise notes that distance counselors may be subject to the laws of the client's location.

References: Counseling Compact — Privilege to practice: adherence to the laws and rules of the remote state · ACA Code of Ethics — H.1.b Laws and Statutes

► **Check:** Compact privileges were not yet being issued in every member state when written; confirm current operational status and the "law of the client's state" framing against the Compact text.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor holds an associate-level counseling license that, under her state's law, requires practice under an approved supervisor. She also holds the NCC credential. A friend urges her to open a solo private practice, saying "the NCC shows you're qualified." What determines whether she may practice independently?

A. Her NCC, since national certification overrides tiered state licenses

National certification is voluntary and does not authorize practice or override state law.

B. Her degree, since a CACREP master's permits independent counseling

A degree is a requirement for licensure, not a legal authorization to practice.

C. Her state's law for her license tier, which limits independent work ✓ **KEY**

The scope and conditions of each license tier are set by state law; an associate license requires supervision.

D. Her own judgment of her skills, since competence is self-assessed

Competence matters ethically, but self-assessment cannot expand a legal scope of practice.

Rationale: State licensure law defines who may practice counseling and under what conditions, including tiered licenses for counselors still completing supervised experience. National certification such as the NCC is a voluntary professional credential and does not grant the legal right to practice. ACA C.2.a also directs counselors to practice only within the boundaries of their credentials.

References: State law — Counselor licensure tiers: associate licenses and supervision requirements · ACA Code of Ethics — C.2.a Boundaries of Competence

► **Check:** Associate-level license titles and supervision rules vary by state; confirm the general framing.

☐ Approve ☐ Revise ☐ Retire Notes:

A state's counselor law is described as a "title act" rather than a "practice act." What does a title act do?

A. It limits who may use a protected title such as "licensed counselor" ✓ **KEY**

A title act restricts use of the protected professional title to people who hold the credential.

B. It limits who may perform counseling services for a fee in the state

Restricting the activity itself is what a practice act does.

C. It sets the standards that graduate programs must meet to accredit

Program standards are set by accreditors such as CACREP, not by state title laws.

D. It requires every counselor in the state to hold national certification

National certification is voluntary and is not created by a state title act.

Rationale: A title act protects the use of a professional title, so only credentialed people may call themselves, for example, a licensed professional counselor. A practice act goes further and defines the scope of practice, restricting who may perform the regulated activities. Most states now regulate both the title and the practice of counseling.

References: Gladding (Counseling: A Comprehensive Profession) — Credentialing: licensure, certification, title and practice acts

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who works with many clients facing eviction testifies before a state legislative committee in support of a bill to expand tenant protections. In the ACA Advocacy Competencies, which domain does this work fall under?

A. Client/student domain, through direct client empowerment

Client-level work helps an individual build self-advocacy skills or acts for one client.

B. School/community domain, through community collaboration

Community collaboration joins local groups; legislative testimony targets public policy.

C. Clinical domain, through individual treatment planning

There is no clinical domain in the competencies; treatment planning is not advocacy.

D. Public arena domain, through social/political advocacy ✓ KEY

Influencing legislation and public policy is social/political advocacy in the public arena.

Rationale: The ACA Advocacy Competencies describe three domains: client/student, school/community and public arena, each with an "acting with" and an "acting on behalf" level. Testifying to lawmakers to change policy that affects many clients is social/political advocacy in the public arena. ACA A.7.a supports advocacy at individual, group, institutional and societal levels.

References: ACA Advocacy Competencies — ACA Advocacy Competencies: client/student, school/community and public arena domains · ACA Code of Ethics — A.7.a Advocacy

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices that several clients from the same apartment complex report asthma flare-ups from mold, and the landlord ignores repair requests. Which action BEST represents systemic rather than client-level advocacy?

A. Help one client draft a letter to the landlord about her own unit

Supporting one client's self-advocacy is valuable but stays at the client level.

B. Teach each affected client coping skills for the stress of the mold

Coping skills address individual distress, not the conditions causing it.

C. Work with a tenants' group and housing officials on building repairs ✓ KEY

Collaborating with community groups to change the conditions affecting many tenants is systems advocacy.

D. Refer each client separately to a legal aid office for their claims

Individual referrals address each case one at a time rather than the system.

Rationale: Client-level advocacy helps a particular client access resources or self-advocate. Systemic advocacy targets the policies, institutions or conditions that create barriers for many people, often by collaborating with community groups. When counselors see a pattern across clients, the MSJCC and ACA A.7.a support intervening at the institutional and community levels, with client consent for any case-specific action.

References: MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions: institutional and community levels · ACA Code of Ethics — A.7.a Advocacy

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old client who is Deaf tells her counselor that her employer keeps refusing to provide an interpreter for staff meetings. She is articulate, knows her rights, and wants to handle it herself, but feels discouraged. Which advocacy approach BEST fits this situation?

A. Call the employer's human resources office on the client's behalf

Acting for a client who wants to self-advocate undercuts her autonomy and needs consent.

B. Help her identify strengths and plan her own self-advocacy steps ✓ **KEY**

Empowerment, acting with the client, fits a capable client who wants to advocate for herself.

C. Focus sessions on accepting the workplace as it currently operates

Encouraging adjustment to discrimination ignores the systemic barrier she faces.

D. File a formal discrimination complaint to a federal agency for her

Filing for her overrides her stated wish to act on her own behalf.

Rationale: At the client level, the ACA Advocacy Competencies distinguish empowerment (acting with clients to build self-advocacy) from advocacy on their behalf. When a client is able and wants to act for herself, the counselor helps her recognize strengths, identify resources and plan steps. ACA A.7.b requires client consent before counselors advocate on a client's behalf.

References: ACA Advocacy Competencies — Client/student empowerment: acting with clients before acting on their behalf · ACA Code of Ethics — A.7.b Confidentiality and Advocacy · Gladding (Counseling: A Comprehensive Profession) — ACA Advocacy Competencies: client/student empowerment

☐ Approve ☐ Revise ☐ Retire Notes: _____

13. nce-s-eth-085 · Professional Counseling Orientation and Ethical Practice · counselor impairment and self-care · easy · In review

A counseling intern says self-care is "a personal lifestyle choice that has nothing to do with ethics." How does the 2014 ACA Code of Ethics treat counselor self-care?

A. As a private matter outside the scope of professional standards

The Code directly links self-care to meeting professional responsibilities.

B. As required only for counselors who are already showing impairment

Self-care is framed as ongoing and preventive, not only a response to impairment.

C. As a duty that applies only to supervisors overseeing trainees

The expectation applies to all counselors, not only supervisors.

D. As part of the professional responsibility to serve clients well ✓ **KEY**

The Code expects counselors to engage in self-care to best meet their professional responsibilities.

Rationale: The introduction to Section C of the ACA Code states that counselors engage in self-care activities to maintain and promote their emotional, physical, mental and spiritual well-being to best meet their professional responsibilities. Standard C.2.g adds that counselors monitor themselves for impairment and refrain from offering services when impaired.

References: ACA Code of Ethics — Section C Introduction (self-care); C.2.g Impairment

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor in solo practice is diagnosed with a progressive neurological illness. She is fully able to work now but knows sudden decline is possible. Under the ACA Code of Ethics, what should she do to protect her clients?

A. Prepare a plan naming a colleague to handle clients and records ✓ KEY

C.2.h calls for a plan for transfer of clients and files if the counselor becomes incapacitated.

B. Close her practice right away, since any illness is an impairment

A diagnosis alone is not impairment; she may practice while able, with planning and monitoring.

C. Tell each client her full diagnosis so they can decide whether to stay

Full medical disclosure is not required and may shift focus from clients' needs.

D. Rely on her family to contact clients if she becomes unable to work

Family members are not qualified to manage confidential records or clinical transitions.

Rationale: ACA C.2.h directs counselors to prepare a plan for the transfer of clients and the distribution of records to an identified colleague or records custodian in case of incapacitation, death or termination of practice, often called a professional will. Under C.2.g, she continues to monitor for signs of impairment and limits practice if it appears.

References: ACA Code of Ethics — C.2.h Counselor Incapacitation, Death, Retirement, or Termination of Practice · ACA Code of Ethics — C.2.g Impairment

☐ Approve ☐ Revise ☐ Retire Notes:

15. nce-s-eth-087 · Professional Counseling Orientation and Ethical Practice · ACA Code of Ethics structure · easy · In review

A new clinical supervisor wants the section of the 2014 ACA Code of Ethics that covers supervisor gatekeeping, evaluation of supervisees and counselor educator duties. Which section should she read?

A. Section D

Section D, Relationships With Other Professionals, covers colleagues, employers and consultation, not supervision duties.

B. Section E

Section E, Evaluation, Assessment, and Interpretation, covers testing and assessment of clients.

C. Section F ✓ KEY

Section F, Supervision, Training, and Teaching, addresses supervision, evaluation and gatekeeping, and counselor educator duties.

D. Section I

Section I, Resolving Ethical Issues, covers ethics complaints, conflicts with law and cooperation with committees.

Rationale: The 2014 ACA Code has nine sections: A The Counseling Relationship; B Confidentiality and Privacy; C Professional Responsibility; D Relationships With Other Professionals; E Evaluation, Assessment, and Interpretation; F Supervision, Training, and Teaching; G Research and Publication; H Distance Counseling, Technology, and Social Media; and I Resolving Ethical Issues. Gatekeeping and supervisee evaluation appear in Section F.

References: ACA Code of Ethics — Section F Supervision, Training, and Teaching (F.6.b Gatekeeping and Remediation)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor asks whether the 2014 ACA Code of Ethics tells counselors exactly which decision-making model to use when facing a dilemma. Which statement is accurate?

A. It requires the Forester-Miller and Davis model for all ethics cases

The Code names no single required model.

B. It expects a credible model able to bear public scrutiny, not a set one ✓ **KEY**

The Code asks counselors to use a credible model whose application can withstand scrutiny.

C. It ranks its standards so higher sections settle conflicts automatically

The Code does not rank its standards to resolve dilemmas mechanically.

D. It leaves decision making entirely to each employer's written policy

Employer policy does not replace the counselor's ethical decision-making duty.

Rationale: The Purpose section and standard I.1.b note that no single decision-making model is best for every situation, so counselors are expected to use a credible model that can bear public scrutiny of its application. Counselors also consult, document their reasoning, and draw on ethical principles and the Code's standards.

References: ACA Code of Ethics — Purpose; I.1.b Ethical Decision Making

☐ Approve ☐ Revise ☐ Retire Notes: _____

17. nce-s-eth-089 · Professional Counseling Orientation and Ethical Practice · NBCC Code of Ethics · medium · In review

A counselor's National Certified Counselor (NCC) credential lapsed eight months ago after she did not recertify. Her website, business cards and email signature still list "NCC." What is the ethical concern?

A. She is misrepresenting a credential she no longer holds ✓ **KEY**

Listing a lapsed credential misleads the public about her qualifications.

B. There is none, since she met the standard when she earned it

A credential may be claimed only while it is current and in good standing.

C. It is a concern only if a client is harmed by her services

Misrepresentation is a violation whether or not harm results.

D. It is a concern only for her state license, not certification

The issue is her claim of national certification, which NBCC governs.

Rationale: Both the NBCC Code of Ethics and ACA C.4.a require counselors to represent their credentials accurately. Continuing to list a lapsed certification misleads clients and the public. She should remove the designation at once or complete the steps to reinstate it.

References: NBCC Code of Ethics — Accurate representation of credentials and qualifications · ACA Code of Ethics — C.4.a Accurate Representation

► **Check:** Confirm that the current NBCC Code of Ethics addresses misrepresenting a lapsed credential; the exact directive number is not cited.

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor who just passed the NCE posts a list of questions she remembers from the exam in an online study group to help friends who test next month. How is this viewed under NBCC standards?

A. As acceptable, since the items came from her own memory

Recalled items are still secure exam content; the source does not change that.

B. As acceptable, as long as she posts the items anonymously

Anonymity does not make sharing secure content ethical.

C. As a concern only if her friends end up passing the exam

The breach occurs when content is shared, whatever the result.

D. As a breach of exam security that can put her NCC at risk ✓ **KEY**

Sharing secure exam content violates NBCC standards and may lead to sanctions.

Rationale: NBCC requires applicants and certificants to protect the security of its examinations, and disclosing recalled exam content is a violation that can lead to sanctions such as denial or revocation of certification. Exam security protects the validity of the credential for every counselor who holds it.

References: NBCC Code of Ethics — Examination security and integrity of the certification process

► **Check:** Confirm that the NBCC Code of Ethics (rather than only the candidate agreement) addresses examination security.

☐ Approve ☐ Revise ☐ Retire Notes:

A newly hired high school counselor wants to join the national professional association for school counselors that publishes a national model for school counseling programs. Which organization is it?

A. AMHCA

The American Mental Health Counselors Association serves clinical mental health counselors.

B. ASCA ✓ **KEY**

The American School Counselor Association publishes the ASCA National Model.

C. NBCC

NBCC is a certification board, not a membership association for school counselors.

D. CACREP

CACREP accredits graduate programs; it is not a membership association.

Rationale: The American School Counselor Association (ASCA) is the national association for school counselors and publishes the ASCA National Model, a framework for comprehensive school counseling programs, along with its own ethical standards for school counselors.

References: Gladding (Counseling: A Comprehensive Profession) — Professional organizations: ASCA and the ASCA National Model

☐ Approve ☐ Revise ☐ Retire Notes:

A state counseling board wants to join a national organization made up of state counselor licensing boards that works on issues such as license portability and consistent regulatory standards. Which organization fits this description?

A. CSI

Chi Sigma Iota (CSI) is the counseling honor society for students and professionals.

B. ACA

The American Counseling Association (ACA) is a membership association of individual counselors, not of boards.

C. AASCB ✓ KEY

The American Association of State Counseling Boards (AASCB) brings together state counselor licensing boards on regulatory issues such as portability.

D. NBCC

The National Board for Certified Counselors (NBCC) certifies individuals and develops exams; its members are not state boards.

Rationale: The American Association of State Counseling Boards (AASCB) is the organization of state counselor licensing boards. It works on regulatory consistency, licensure portability and public protection. ACA serves individual members, NBCC grants national certification and develops exams, and CSI is the profession's honor society.

References: Gladding (Counseling: A Comprehensive Profession) — Credentialing bodies: AASCB, NBCC and state licensure boards

☐ Approve ☐ Revise ☐ Retire Notes:

21. nce-s-eth-093 · Professional Counseling Orientation and Ethical Practice · confidentiality and its limits · hard · In review

A 15-year-old client reveals that she has been cutting her thighs and has had thoughts of "not waking up." Her father emailed the counselor that morning demanding to know what his daughter talks about. She begs the counselor not to tell her parents anything. What should the counselor do FIRST?

A. Complete a risk assessment, then plan with her what parents must learn ✓ KEY

Risk must be assessed first; safety information is then shared with parents with the teen involved.

B. Reply to the father's email with a full summary of what she disclosed

Disclosure before assessing risk and without the teen's involvement shares more than needed.

C. Keep the disclosure private, since she asked that nothing be shared

A minor's request cannot override the duty to protect when safety is at stake.

D. Tell her sessions will stop unless she tells her parents herself today

Threatening to end services is coercive and risks abandonment of an at-risk teen.

Rationale: Self-injury and passive suicidal ideation call first for a structured risk assessment. If the assessment shows risk, ACA B.2.a permits disclosure to protect the client, and with a minor the counselor involves the teen in deciding how parents are told, sharing only what is needed for safety (B.2.e). The father's general demand for content is handled separately, balancing parental rights with the teen's privacy (B.5.b).

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm; B.2.e Minimal Disclosure · ACA Code of Ethics — B.5.b Responsibility to Parents and Legal Guardians

☐ Approve ☐ Revise ☐ Retire Notes:

A father in individual counseling for depression asks his counselor to testify as an expert in his upcoming custody trial and to recommend that he receive primary custody. He says, "You know me better than anyone." What should the counselor do?

A. Agree, since a long alliance gives her the best basis for an opinion
Familiarity from treatment is not a basis for a forensic opinion and creates a dual role.

B. Agree if the other parent and both attorneys consent to her testimony
Consent of others does not cure the conflict between therapeutic and forensic roles.

C. Write a letter supporting custody instead of appearing at the hearing
A custody recommendation in writing is the same forensic opinion in a different form.

D. Explain she cannot give a custody opinion and refer him to an evaluator ✓ **KEY**
E.13.c bars evaluating current clients for forensic purposes; a neutral evaluator is the proper source.

Rationale: ACA E.13.c states that counselors do not evaluate current or former clients, clients' romantic partners or clients' family members for forensic purposes. A treating counselor lacks the neutrality and full information a custody evaluation requires. If legally compelled, she may testify as a fact witness with appropriate consent, describing treatment rather than recommending custody.

References: ACA Code of Ethics — E.13.c Client Evaluation Prohibited · ACA Code of Ethics — A.6.e Nonprofessional Interactions or Relationships (Other Than Sexual or Romantic)

☐ Approve ☐ Revise ☐ Retire Notes:

A client with adjustment disorder learns his insurer will pay for more sessions only for "a serious diagnosis." He asks his counselor to record major depressive disorder, and the practice manager says, "Everyone does it." What should the counselor do?

A. Record major depression, since the client has consented to the code
Client consent does not make a false diagnosis to an insurer honest or legal.

B. Keep the accurate diagnosis and discuss other payment or care options ✓ **KEY**
Diagnoses must be accurate; the counselor can explore sliding fees, self-pay or referral.

C. Record major depression only until the client's finances stabilize
A time-limited false diagnosis is still fraudulent billing.

D. Follow the manager's direction, since the practice bears the billing risk
Employer pressure does not excuse misrepresentation; the counselor is personally accountable.

Rationale: Counselors provide proper diagnoses (ACA E.5.a) and are accurate, honest and objective in reports to third parties, including insurers (C.6.b). Upcoding a diagnosis to obtain payment is insurance fraud and can also harm the client through an inaccurate permanent record. The counselor addresses the financial barrier through legitimate options instead.

References: ACA Code of Ethics — E.5.a Proper Diagnosis; C.6.b Reports to Third Parties

☐ Approve ☐ Revise ☐ Retire Notes:

A client signed a release last month letting her counselor share records in her divorce case. On the same day, the counselor receives a subpoena for the records from the spouse's attorney and a signed letter from the client revoking the release. What should the counselor do?

A. Send the records, since the release was valid when the subpoena was issued

Revocation applies to future disclosures; nothing has yet been released in reliance on it.

B. Ignore the subpoena, since the client has now withdrawn her permission

A subpoena is a legal document that must receive a timely response.

C. Honor the revocation, respond to the subpoena, and seek a court order ✓ **KEY**

The revocation stops voluntary release; the counselor responds without disclosing and asserts privilege.

D. Send a summary only, since that limits what the spouse's side will see

Any disclosure now lacks valid consent or a court order.

Rationale: A client may revoke an authorization in writing, and the revocation applies to any disclosure not yet made. A subpoena alone is not a court order, so the counselor responds in a timely way, notifies the client, asserts privilege on her behalf, consults an attorney, and discloses only what a court orders, limiting it to what is necessary (ACA B.2.c and B.2.e).

References: ACA Code of Ethics — B.2.c Court-Ordered Disclosure · HIPAA — 45 CFR 164.508(b)(5) Revocation of authorizations

► **Check:** Subpoena response and revocation handling vary by state and court rules; confirm the general guidance.

☐ Approve

☐ Revise

☐ Retire

Notes:

Intake, Assessment, and Diagnosis

24 items · NCE weight 12% (19 of 160 scored items)

25. nce-s-ass-073 · Assessment and Testing · norm- vs criterion-referenced tests · easy · In review

A parent is reading her son's score report from a statewide mathematics test. Which statement on the report is a norm-referenced interpretation?

A. He answered 34 of the 40 test items correctly

A raw score of items correct makes no comparison with other test takers, so it is not norm-referenced.

B. He scored higher than 70% of same-grade students ✓ **KEY**

Comparing his score with a norm group of same-grade students is the defining feature of a norm-referenced interpretation.

C. He met the state proficiency standard in math

Meeting a fixed proficiency standard compares him with a criterion, not with other students.

D. He mastered 9 of the 10 fraction objectives tested

Mastery of stated objectives is a criterion-referenced interpretation of what he can do.

Rationale: A norm-referenced interpretation describes a score by its position relative to a norm group, as with percentile ranks and standard scores. A criterion-referenced interpretation describes performance against a fixed standard or set of objectives, regardless of how others performed. The same test can support both kinds of statement, so the question is what the score is being compared with.

References: Hays (Assessment) — Norm-referenced and criterion-referenced score interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

26. nce-s-ass-074 · Assessment and Testing · norm- vs criterion-referenced tests · hard · In review

A counselor educator is building a criterion-referenced mastery test for a crisis-intervention training module. After instruction, 96% of trainees answer one item correctly. The item covers an essential safety skill named in the module objectives. What should the developer do with this item?

A. Drop it, because an item this easy fails to spread scores apart

Spreading scores apart is a norm-referenced goal; a mastery test is not built to rank trainees.

B. Rewrite it so that about half of all the trainees will miss it

Targeting a difficulty near .50 maximizes variance for norm-referenced tests, not for mastery testing.

C. Replace it with an item that better separates the top trainees

Separating high from low scorers serves ranking; it would drop coverage of a required objective.

D. Keep it, since it measures a required objective most mastered ✓ **KEY**

A criterion-referenced item is judged by its match to the objective; high success after instruction is the desired result.

Rationale: Norm-referenced tests favor items of moderate difficulty that discriminate among examinees, because their purpose is to rank people. Criterion-referenced tests sample the content domain defined by the objectives, so an item tied to an essential skill stays in the test even when nearly everyone passes it after good instruction. Low score variance is expected, and even welcome, on a mastery test.

References: Hays (Assessment) — Criterion-referenced test construction; item difficulty and discrimination

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor plans to use a nonverbal ability test whose manual reports norms that were gathered about 30 years ago. When comparing today's clients with that norm group, what is the MAIN interpretive concern?

A. Scores may run high, since average performance has risen over time ✓ KEY

Population gains in ability-test performance mean old norms set a lower bar, inflating current standard scores.

B. Old norms pose no problem when internal consistency is still high

Reliability says nothing about whether the norm group still represents the people being tested.

C. Scores may run low, since older norm groups were more test-savvy

This reverses the documented direction; outdated ability norms tend to overstate current standing.

D. Outdated norms alter raw scores but leave percentile ranks intact

Percentile ranks are computed from the norm group, so they are exactly what outdated norms distort.

Rationale: A norm-referenced score is only as meaningful as the norm group behind it. On ability tests, average performance in many populations rose over the 20th century (the Flynn effect), so clients compared with an old norm group tend to receive inflated standard scores and percentiles. Counselors should check the age, size, and makeup of the norm sample and prefer recently normed editions.

References: Hays (Assessment) — Evaluating norm groups: recency, representativeness, and size

► **Check:** Flynn effect framing: gains have slowed or reversed in some countries in recent decades. SME to confirm the "may run high" wording is appropriately hedged.

☐ Approve ☐ Revise ☐ Retire Notes:

A college counseling center uses a regression equation to predict first-year GPA from an admissions test score. A counselor wants a statistic that shows how far students' actual GPAs typically fall from their predicted GPAs. Which statistic should she use?

A. The standard error of measurement of the admissions test

The SEM describes how obtained test scores vary around a person's true score, not prediction error for a criterion.

B. The standard error of the mean for the sample of GPAs

The standard error of the mean describes how precisely a sample mean estimates a population mean.

C. The standard error of estimate for the regression line ✓ KEY

The standard error of estimate is the typical spread of actual criterion scores around predicted scores.

D. The standard deviation of the admissions test scores

This describes how spread out the predictor scores are, not how accurate the GPA predictions are.

Rationale: Three standard errors are easy to confuse. The standard error of measurement concerns a single test and the error around an individual's true score; the standard error of estimate concerns prediction and the error around a predicted criterion score; the standard error of the mean concerns sampling and the error in a sample mean as an estimate of the population mean. Predicting GPA from a test calls for the standard error of estimate.

References: Hays (Assessment) — Standard error of measurement vs standard error of estimate · Erford (Research and Evaluation in Counseling) — Standard error of the mean; regression and prediction error

☐ Approve ☐ Revise ☐ Retire Notes:

Two anxiety scales use the same score metric and have the same standard deviation. Scale X has a reliability coefficient of .95, and Scale Y has one of .70. Why will Scale X have the smaller standard error of measurement?

A. Higher reliability increases the spread of the obtained scale scores

The two scales have the same standard deviation; reliability does not enlarge score spread.

B. Less of its score variance is error, so scores sit nearer true scores ✓ **KEY**

SEM = SD times the square root of (1 - r); as r rises, the error portion and the SEM both shrink.

C. Reliable scales have more items, and the SEM is divided by item count

The SEM formula uses the SD and reliability coefficient; there is no division by the number of items.

D. Higher reliability means a stronger correlation with a criterion

Correlation with a criterion is validity evidence, and it is not what determines the SEM.

Rationale: The standard error of measurement is computed as SD multiplied by the square root of (1 minus the reliability coefficient). Reliability is the share of observed-score variance that is true-score variance, so the remainder is error. With equal standard deviations, the scale with .95 reliability leaves only 5% error variance versus 30%, giving a smaller SEM and narrower confidence bands around an individual's score.

References: Hays (Assessment) — Standard error of measurement and its relation to reliability

☐ Approve ☐ Revise ☐ Retire Notes:

On a test battery where each subtest has a standard error of measurement of 4, a 12-year-old earns 103 on Verbal Reasoning and 109 on Spatial Reasoning. His parent asks whether spatial reasoning is a real strength. Using bands of plus or minus 1 SEM, what should the counselor say?

A. The bands overlap, so the gap may reflect measurement error ✓ **KEY**

The bands run 99-107 and 105-113; overlap means the true scores may not differ.

B. The 6-point gap exceeds one SEM, so it is a genuine strength

Both scores carry error, so a gap slightly larger than one SEM is not enough to call the difference real.

C. The gap can be judged only after converting both to percentiles

Changing the score metric does not remove measurement error from either score.

D. Error bands apply to composite scores, not to subtest scores

Every score, including a subtest score, has measurement error and can be reported with a band.

Rationale: Reporting scores as bands rather than points reminds everyone that each obtained score contains error. A common rule of thumb is that when two scores' bands overlap, the difference should not be treated as meaningful; when they do not overlap, a real difference is more likely. A more precise approach uses the standard error of the difference, which is larger than either SEM because both scores contribute error.

References: Hays (Assessment) — Confidence bands; comparing two scores with the standard error of measurement

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old graduate student completes the NEO-PI-3 as part of career counseling. Her Extraversion score is very low, and her Neuroticism score is average. She asks whether this means something is wrong with her. What is the BEST interpretation to offer?

A. It suggests social anxiety that should be assessed further

Low Extraversion is not a measure of anxiety, and her Neuroticism score, which taps distress, is average.

B. It shows emotional instability and a tendency toward distress

Emotional instability and proneness to distress describe high Neuroticism, not low Extraversion.

C. It points to difficulty cooperating with and trusting others

Trust and cooperativeness belong to the Agreeableness domain, not Extraversion.

D. It reflects a reserved style and a liking for quieter settings ✓ KEY

Low Extraversion describes a reserved, independent person who prefers less stimulation; it is a normal trait.

Rationale: The NEO-PI-3 measures normal-range personality on the five-factor model. Low Extraversion (introversion) means a person tends to be reserved, prefers smaller groups or solitude, and seeks less stimulation; it does not mean unhappiness or anxiety. Distress proneness is captured by Neuroticism. Interpreting traits as normal variation, and relating them to the client's goals, is the appropriate use of the profile.

References: Hays (Assessment) — NEO Personality Inventory-3: five-factor domains and interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client took the Myers-Briggs Type Indicator (MBTI) at work two years ago and was typed ENFP. Taking it again in counseling, she is typed INFP. Her Extraversion-Introversion preference score was close to the midpoint both times. What BEST explains the change?

A. Her basic personality has shifted a great deal over the past two years

A small move across the midpoint does not show major personality change.

B. The second result is invalid, so the first type is her true type

Nothing suggests an invalid administration; neither type is more "true" when the preference is unclear.

C. Splitting a continuous score into two types can flip near the middle ✓ KEY

The MBTI sorts scores into either/or categories, so small shifts near the cut point change the reported type.

D. A change in type like this shows that she is in a period of crisis

The MBTI is not a measure of distress, and a type change near the midpoint has no clinical meaning.

Rationale: The MBTI reports four dichotomous preferences that combine into 16 types, but the underlying preference scores are continuous. People whose scores fall near the dividing point can be assigned a different type on retest, which is a well-known limit of type measures. The MBTI is used for self-understanding and career exploration; it is not designed for diagnosis or for selection decisions.

References: Hays (Assessment) — Myers-Briggs Type Indicator: type scoring and limitations · Sharf (Career Development Theory) — Use of the MBTI in career counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses the Sixteen Personality Factor Questionnaire (16PF) in premarital counseling. Which description of this instrument is accurate?

A. A normal-range personality measure built through factor analysis ✓ KEY

Raymond Cattell derived the 16PF's primary factors through factor analysis of trait descriptors.

B. An inventory empirically keyed to identify psychiatric patient groups

Empirical keying against clinical groups describes the MMPI tradition, not the 16PF.

C. A projective task in which clients describe ambiguous inkblots

Describing inkblots is the Rorschach, a projective test; the 16PF is a self-report questionnaire.

D. A type indicator that sorts people into Jungian categories

Sorting people into Jungian types describes the MBTI, not the 16PF's trait dimensions.

Rationale: The 16PF, developed by Raymond Cattell, is an objective self-report inventory of normal adult personality. Its 16 primary factors were identified through factor analysis and are summarized in broader global factors that resemble the Big Five. Because it describes normal traits rather than psychopathology, it is often used in career, couples, and personal growth counseling.

References: Hays (Assessment) — Sixteen Personality Factor Questionnaire (16PF): primary and global factors

☐ Approve ☐ Revise ☐ Retire Notes:

During intake, a 45-year-old client mentions that she saw a counselor at another agency for depression three years ago. The counselor would like to review those records. What is the BEST way to obtain them?

A. Phone the former counselor and ask for a verbal case summary

An informal call without the client's knowledge or authorization bypasses her control over her information.

B. Have her sign a release naming the agency and records sought ✓ KEY

A written, informed release specifying the source and the information requested respects her rights.

C. Ask her to describe that treatment and skip the records entirely

Her recollection is useful, but prior records add diagnoses, dates, and responses she may not recall.

D. Request the full file, including all notes, as routine practice

Requesting everything by default ignores the client's consent and the principle of seeking only what is needed.

Rationale: Prior treatment records can clarify past diagnoses, treatments tried, and response to care. The counselor should explain why the records would help and obtain the client's written authorization that names the releasing agency, the recipient, the information sought, and its purpose. Limiting the request to what is relevant respects privacy, and some material, such as psychotherapy notes, carries extra protection.

References: ACA Code of Ethics — B.6.g Disclosure or Transfer · HIPAA — 45 CFR 164.508 Uses and disclosures requiring an authorization

► **Check:** HIPAA permits provider-to-provider disclosure for treatment without authorization (psychotherapy notes and Part 2 records excepted); the key is framed as best practice. SME to confirm the HIPAA citation fits.

☐ Approve ☐ Revise ☐ Retire Notes:

A client on probation asks her counselor to send attendance and progress information to her probation officer. The release she signs and dates names the counselor, the probation officer, the information to be sent, the purpose, and the required notices on revocation and redisclosure. Under HIPAA, which required element is still missing?

A. A notary's seal that verifies the client's signature

HIPAA does not require notarization for an authorization to be valid.

B. The diagnosis that supports her referral to treatment

A diagnosis is not a required element; the form must describe the information, not justify the treatment.

C. A second signature from a witness at the counseling agency

A witness signature is not one of the required core elements of a valid authorization.

D. An expiration date or event that ends the authorization ✓ **KEY**

A valid authorization must state when it expires, either as a date or as a defined event.

Rationale: Under the HIPAA Privacy Rule, a valid authorization includes a specific description of the information, who may disclose it, who may receive it, the purpose, an expiration date or event, the client's signature and date, and statements about the right to revoke and the potential for redisclosure. A release missing its expiration is defective, so the counselor should have the client complete it before sending anything.

References: HIPAA — 45 CFR 164.508(c) Core elements and required statements of a valid authorization

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor is assessing a 74-year-old widower for possible cognitive decline. He says he manages fine. His daughter, who visits three times a week, describes missed medications and getting lost while driving. His son, who lives overseas and speaks with him by phone monthly, says he "sounds like his old self." How should the counselor weigh these reports?

A. Average the two accounts, since each child knows him well

Averaging ignores how differently placed the two informants are to observe his daily functioning.

B. Defer to his own report, since he is a competent adult client

People with cognitive decline often lack awareness of their deficits, so self-report alone is unreliable here.

C. Weigh the daughter's direct observations and seek testing ✓ **KEY**

Her frequent, specific, firsthand observations carry more weight, and objective testing can corroborate them.

D. Discount the daughter's report, as caregiver strain biases it

Possible bias is worth noting, but it does not erase concrete, firsthand observations of safety problems.

Rationale: When collateral reports conflict, counselors weigh each informant's opportunity to observe, how recent and specific the observations are, and any reasons for bias. Brief phone calls can mask deficits that show up in daily tasks such as managing medications or driving. Rather than picking a side, the counselor uses the conflict to guide further assessment, such as cognitive screening and a medical evaluation.

References: Hays (Assessment) — Collateral sources: evaluating informant reports · DSM-5-TR — Major and mild neurocognitive disorders: Criterion A (concern of the individual, a knowledgeable informant, or the clinician)

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor reviewing a psychological report notes that the Rorschach was formally scored, while the Thematic Apperception Test (TAT) was interpreted. Which statement BEST compares how these two projective tests are typically scored?

A. The Rorschach has normed coding systems such as Exner's and R-PAS; TAT stories are usually read clinically

✓ KEY

The Rorschach is commonly coded with normed systems, while the TAT is most often interpreted without a single standard system.

B. The TAT is coded with Exner's Comprehensive System; the Rorschach is read only through clinical intuition

This reverses the tests: Exner's Comprehensive System was developed for the Rorschach, not the TAT.

C. Both are scored by computer against national norms, so examiner training matters relatively little

Projective tests require extensive examiner training, and the TAT lacks a single computerized normed scoring.

D. Neither test has any formal scoring system, so both rest entirely on the examiner's personal judgment

The Rorschach has detailed, research-based coding systems, so it does not rest on judgment alone.

Rationale: Exner's Comprehensive System brought standardized administration, coding, and norms to the Rorschach, and the Rorschach Performance Assessment System (R-PAS) later built on that work. The TAT has several research scoring approaches, but in practice it is usually interpreted qualitatively by examining themes, needs, and relationships in the stories. This difference affects how much weight each result can bear in an assessment.

References: Hays (Assessment) — Projective personality assessment: Rorschach scoring systems and the TAT

► **Check:** Formal TAT scoring systems exist (e.g., SCORS, need-achievement coding) but are not uniformly used; SME to confirm the "usually read clinically" framing.

☐ Approve ☐ Revise ☐ Retire Notes:

A middle school principal asks the school counselor to "give some tests" to a seventh grader who is "having problems." What should the counselor do FIRST?

A. Give a broad battery so that no possible problem is overlooked

Testing without a focus wastes time, burdens the student, and may produce results no one can use.

B. Choose the instruments the school already owns and knows well

Convenience should not drive test choice; the instrument must fit the question being asked.

C. Clarify the specific question the assessment needs to answer ✓ KEY

A clear referral question determines what to assess and which instruments are appropriate.

D. Begin with a personality inventory to screen for any disorder

Picking an instrument before knowing the concern puts the tool ahead of the purpose.

Rationale: Assessment begins by defining its purpose. A vague request such as "having problems" must be turned into a specific referral question, for example about attention, mood, learning, or behavior, before any instrument is chosen. The question then guides test selection, the information sources needed (including parent consent and input), and how results will be used.

References: Hays (Assessment) — The assessment process: clarifying the referral question; selecting instruments

☐ Approve ☐ Revise ☐ Retire Notes:

A vocational rehabilitation agency asks a counselor whether a 30-year-old client meets criteria for intellectual developmental disorder. Which assessment plan BEST answers this referral question?

A. An individual intelligence test alone, since IQ defines the disorder

DSM-5-TR requires deficits in adaptive functioning as well; an IQ score alone cannot establish the diagnosis.

B. An individual intelligence test plus a standardized adaptive measure ✓ **KEY**

The diagnosis requires both intellectual and adaptive deficits, so both must be measured directly.

C. A group achievement test plus a brief cognitive screening instrument

Group achievement tests and brief screens are not adequate for a diagnostic decision about intellect.

D. A personality inventory plus an interest inventory for job matching

These may help with vocational planning, but they do not address intellectual or adaptive functioning.

Rationale: Intellectual developmental disorder requires deficits in intellectual functions, confirmed by clinical assessment and individualized standardized testing, together with deficits in adaptive functioning, with onset during the developmental period. Severity levels are defined by adaptive functioning. A sound plan therefore pairs an individually administered intelligence test with a standardized adaptive behavior measure and a developmental history.

References: DSM-5-TR — Intellectual developmental disorder: Criteria A-C and severity levels · Hays (Assessment) — Selecting instruments to fit the referral question

☐ Approve ☐ Revise ☐ Retire Notes:

A test manual reports a reliability coefficient of .80 for a self-esteem scale. Which interpretation of this value is correct?

A. About 64% of the variance is true variance, found by squaring .80

Squaring applies to a correlation between two different measures; a reliability coefficient is read directly.

B. About 80% of the people who take the scale receive accurate scores

Reliability describes score variance in a group, not the share of individuals scored accurately.

C. The scale correlates .80 with an outside self-esteem criterion

A correlation with an outside criterion is a validity coefficient, not a reliability coefficient.

D. About 80% of observed score variance reflects true-score differences ✓ **KEY**

A reliability coefficient estimates the proportion of observed-score variance that is true-score variance.

Rationale: In classical test theory, a reliability coefficient is interpreted directly as the proportion of observed-score variance attributable to true-score differences. A coefficient of .80 means about 80% true variance and 20% error variance. Unlike a validity coefficient, it is not squared, because it already represents a ratio of variances.

References: Hays (Assessment) — Interpreting reliability coefficients: true and error variance · Erford (Research and Evaluation in Counseling) — Classical test theory and reliability

☐ Approve ☐ Revise ☐ Retire Notes:

An aptitude test correlates .35 with supervisors' later job performance ratings. A colleague tells the counselor the test is worthless because this figure is so far below the test's .90 reliability. Which response is MOST accurate?

A. Modest validity coefficients are common and can still aid prediction ✓ KEY

Validity coefficients are typically much lower than reliability coefficients and can still improve decisions.

B. The test explains about 35% of the variance in job performance

Shared variance is the coefficient squared, so .35 explains roughly 12% of criterion variance.

C. A validity coefficient must equal the reliability to be of any use

Reliability sets an upper limit on validity; validity is not expected to reach it.

D. The coefficient shows that the true reliability of the test is .35

A test-criterion correlation is validity evidence; it does not re-estimate reliability.

Rationale: A validity coefficient is the correlation between test scores and a criterion. Because criteria such as supervisor ratings are themselves imperfect and many factors affect performance, validity coefficients are usually far lower than reliability coefficients. Squaring .35 gives about .12, so the test accounts for roughly 12% of criterion variance, which can still add useful information when combined with other data.

References: Hays (Assessment) — Validity coefficients: interpretation and typical size · Erford (Research and Evaluation in Counseling) — Coefficient of determination (r squared)

☐ Approve ☐ Revise ☐ Retire Notes:

Two cultural groups differ in average scores on a clerical aptitude test. Research shows the test predicts later job performance equally well for both groups, using the same regression line. A hiring manager concludes that the test is biased. What is the BEST response?

A. Any gap in group means is itself proof that a test is biased

Group mean differences can reflect real differences in opportunity or preparation, not only test bias.

B. A mean gap alone does not show bias; equal prediction argues against it ✓ KEY

Bias is a technical question; equal prediction across groups is evidence against predictive bias.

C. Equal prediction proves the test is fair in every possible use and setting

Fairness is broader than bias and includes access, consequences, and how scores are used.

D. Bias can be judged only by a panel reviewing items for offense

Content review helps, but statistical evidence such as differential prediction is central to judging bias.

Rationale: Test bias is a psychometric property: systematic error that makes a test measure or predict differently for different groups, studied through item functioning and differential prediction. Test fairness is a broader judgment about equitable treatment, access, and the consequences of test use. A mean difference alone does not establish bias, yet a technically unbiased test can still be used unfairly, so both questions deserve attention.

References: Hays (Assessment) — Test bias and test fairness; differential prediction · Sue & Sue (Counseling the Culturally Diverse) — Cultural bias in assessment

☐ Approve ☐ Revise ☐ Retire Notes:

A client with low vision takes a timed, standardized reading-comprehension test in large print with extended time as an accommodation for her disability. How should the counselor handle the results?

A. Score it with the usual norms and make no note of the changes

Leaving out the accommodations would hide a departure from standard conditions that affects interpretation.

B. Discard the results, since accommodated scores are unusable

Accommodated results can still be informative; the code calls for noting conditions, not discarding results.

C. Note the changed conditions in the report and interpret with care ✓ **KEY**

Documenting nonstandard conditions and qualifying interpretation is the ethically required approach.

D. Lower her score to offset the advantage given to her by the extra time

Ad hoc score adjustments have no psychometric basis and would distort the results further.

Rationale: Accommodations for disability are often necessary so that a test measures the intended skill rather than the disability. Because norms were built under standard conditions, the counselor notes the accommodations in the interpretation and treats the comparison with norms cautiously; results may be designated as of questionable validity when conditions differ substantially. Some publishers provide guidance on which accommodations preserve comparability.

References: ACA Code of Ethics — E.7.a Administration Conditions · Hays (Assessment) — Testing accommodations for individuals with disabilities

☐ Approve ☐ Revise ☐ Retire Notes:

To save session time, a counselor wants clients to complete a symptom inventory online at home before intake. The publisher validated the inventory only for proctored administration in an office. Under the ACA Code of Ethics, what should the counselor do?

A. Allow home use, since online forms are equal to paper forms

Equivalence across formats and settings must be shown; it cannot be assumed.

B. Allow home use once the client signs an informed consent form

Consent does not make an instrument valid for a use it was not designed or validated for.

C. Allow home use but double the time limit to offset household distractions

Changing the time limit adds another departure from standard conditions rather than solving one.

D. Have clients take it under supervision, as it was validated that way ✓ **KEY**

Instruments not designed and validated for self-administration should not be used unsupervised.

Rationale: The ACA Code of Ethics directs counselors not to permit unsupervised use of an assessment unless it was designed, intended, and validated for self-administration or scoring. Counselors using technology to administer tests must also ensure the platform functions properly and gives accurate results. Convenience does not justify a use that the validity evidence does not support.

References: ACA Code of Ethics — E.7.d Unsupervised Assessments; E.7.c Technological Administration

► **Check:** Confirm ACA 2014 section numbers E.7.c (Technological Administration) and E.7.d (Unsupervised Assessments).

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is reviewing a personality inventory profile with a 50-year-old client, who looks worried when she sees that one scale is elevated. Which approach to communicating the results is BEST?

A. Explain the scale in plain terms and ask how it fits her life ✓ KEY

Clear language and inviting her reactions make the results understandable and collaborative.

B. Read the computer narrative aloud so she hears the full report

Automated narratives are generic and technical; the counselor is responsible for interpretation.

C. Skip the elevated scale so that she will not feel more upset

Withholding results disregards her right to know and leaves her worry unaddressed.

D. Hand her the profile to study at home and talk next session

Leaving her alone with an elevated score and no explanation invites misunderstanding.

Rationale: Clients have the right to know their results and the basis for the counselor's conclusions. Good interpretation uses everyday language, presents scores as estimates and one source of information, links findings to the client's goals, and invites the client to compare the results with her own experience. This turns the profile into a shared working tool rather than a verdict.

References: ACA Code of Ethics — E.1.b Client Welfare · Hays (Assessment) — Communicating assessment results to clients

☐ Approve ☐ Revise ☐ Retire Notes:

46. nce-s-ass-094 · Assessment and Testing · screening instruments (PHQ-9, GAD-7, AUDIT, CAGE) · medium · In review

A counselor at a veterans' clinic gives a client the PTSD Checklist for DSM-5 (PCL-5) at intake and again every few weeks during treatment. Which description of the PCL-5 is accurate?

A. A structured clinician interview that is the standard for diagnosis

This describes the Clinician-Administered PTSD Scale for DSM-5 (CAPS-5), not the PCL-5.

B. A self-report symptom measure used to screen and to track change ✓ KEY

The PCL-5 is a brief self-report of DSM-5 PTSD symptoms used for screening and monitoring.

C. A checklist of the traumatic events a client has been exposed to

Listing exposure to events is the job of a trauma-exposure checklist, not this symptom measure.

D. A projective measure that uncovers unconscious trauma conflicts

The PCL-5 is an objective self-report scale with rated items, not a projective technique.

Rationale: The PCL-5 is a 20-item self-report measure in which clients rate how much each DSM-5 PTSD symptom has bothered them in the past month. It is widely used to screen, to support a provisional diagnosis, and to monitor change across treatment as part of measurement-based care. A positive result should be followed by a full clinical assessment rather than treated as a diagnosis on its own.

References: VA/DoD CPG — PTSD guideline: screening and measurement-based care with the PCL-5 · DSM-5-TR — Posttraumatic stress disorder: Criteria B-E

☐ Approve ☐ Revise ☐ Retire Notes:

In early sessions with a couple, a counselor draws a diagram of three generations of both partners' families, marking marriages, divorces, deaths, illnesses, substance use, and close or cut-off relationships. What is the main assessment purpose of this tool?

A. To produce a standardized score for each partner's satisfaction

A genogram is a qualitative map, not a normed instrument that yields scores.

B. To decide which partner is mainly to blame for the couple's ongoing conflict

Systemic assessment looks at patterns and context rather than assigning individual blame.

C. To screen each partner for a personality disorder before treatment

A genogram maps family structure and patterns; it is not a diagnostic screen.

D. To reveal family patterns across generations that shape current issues ✓ **KEY**

Genograms show repeating patterns, alliances, and cutoffs that may bear on present problems.

Rationale: The genogram, rooted in Bowen family systems theory, maps at least three generations of a family's structure, events, and relationships. As an assessment tool, it helps counselor and clients see recurring patterns, such as conflict, cutoffs, illness, or substance use, that may be carried into the current relationship. It is also a collaborative way to gather family history.

References: Goldenberg (Family Therapy) — Bowen family systems: the genogram as an assessment tool

☐ Approve ☐ Revise ☐ Retire Notes:

On the Strong Interest Inventory, a 26-year-old client's highest General Occupational Theme is Artistic. Within that theme, her Basic Interest Scale for Culinary Arts is high, while those for Writing and Mass Communication and for Visual Arts and Design are low. How should the counselor interpret this?

A. The low scales cancel the theme, so artistic work does not fit her

One high Basic Interest Scale shows a real artistic interest; the low scales only narrow where it lies.

B. The Basic Interest Scales reflect skills, and themes reflect interests

Both scales measure interests; self-rated skill confidence is assessed separately.

C. The theme is broad, and the scales show which artistic areas she favors ✓ **KEY**

Basic Interest Scales break each broad theme into specific areas of interest.

D. The Basic Interest Scales compare her with workers in each occupation

Comparison with satisfied workers in specific occupations is the role of the Occupational Scales.

Rationale: The Strong Interest Inventory reports results at several levels. The six General Occupational Themes follow Holland's RIASEC model and describe broad interest patterns; the Basic Interest Scales break those themes into more specific interest areas; the Occupational Scales compare the client's likes and dislikes with those of satisfied workers in particular occupations. Reading the Basic Interest Scales within a theme helps the counselor pinpoint which kinds of work fit best.

References: Sharf (Career Development Theory) — Strong Interest Inventory: General Occupational Themes, Basic Interest Scales, Occupational Scales · Hays (Assessment) — Career assessment: interest inventories

► **Check:** Confirm that Culinary Arts, Writing and Mass Communication, and Visual Arts and Design are Basic Interest Scales under the Artistic theme in the current Strong edition.

☐ Approve ☐ Revise ☐ Retire Notes:

Areas of Clinical Focus

59 items · NCE weight 29% (47 of 160 scored items)

49. nce-s-clf-178 · Assessment and Testing · neurocognitive disorders · hard · In review

Over 2 years, a 57-year-old engineer has made crude remarks to strangers, lost interest in his grandchildren, shown little concern when his wife cries, and begun eating sweets compulsively. He still recalls recent events and finds his way around town without trouble. Which diagnosis BEST fits?

A. Major neurocognitive disorder due to Alzheimer's disease

Alzheimer's disease typically begins with prominent decline in learning and memory, which is relatively spared here.

B. Major neurocognitive disorder with Lewy bodies, early stage

Lewy body NCD features fluctuating cognition, recurrent visual hallucinations, and parkinsonism, none of which are described.

C. Frontotemporal neurocognitive disorder, behavioral variant ✓ KEY

Disinhibition, apathy, loss of empathy, and hyperorality with relatively spared memory and spatial skills fit the behavioral variant.

D. Bipolar I disorder, current episode manic, late onset

There is no elevated mood, decreased need for sleep, or episodic course; the change is gradual and progressive.

Rationale: The behavioral variant of frontotemporal neurocognitive disorder requires three or more of: behavioral disinhibition; apathy or inertia; loss of sympathy or empathy; perseverative, stereotyped, or compulsive behavior; and hyperorality or dietary change, along with prominent decline in social cognition or executive abilities. Learning, memory, and perceptual-motor function are relatively spared. Onset is often in midlife, so the presentation is easily mistaken for a psychiatric or "midlife" problem.

References: DSM-5-TR — Major or Mild Frontotemporal Neurocognitive Disorder, Criterion C1 (behavioral variant)

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 79-year-old with a known neurocognitive disorder now relies on her son to pay her bills and set out her daily medications because she makes repeated errors on her own. Under DSM-5-TR, what does this change MOST directly indicate?

A. Deficits now interfere with independence, so major NCD fits ✓ KEY

Needing help with complex instrumental activities such as bills and medications marks the functional threshold for major NCD.

B. A delirium is now present on top of her existing disorder

Delirium is an acute, fluctuating disturbance in attention and awareness, which is not described.

C. Her cognitive test scores must now fall in the severe range

The mild/major distinction rests mainly on functional independence, not on a single test cutoff.

D. The cause of her disorder must have shifted to vascular

A change in functional level says nothing about a change in etiological subtype.

Rationale: DSM-5-TR separates mild from major neurocognitive disorder chiefly by function. In mild NCD the deficits do not interfere with independence in everyday activities, although greater effort or compensatory strategies may be needed. In major NCD they do, at minimum requiring assistance with complex instrumental activities such as paying bills or managing medications.

References: DSM-5-TR — Major Neurocognitive Disorder, Criterion B; Mild Neurocognitive Disorder, Criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

A daughter caring for her father with moderate Alzheimer's disease says he asks several times each evening where his late wife is. Each time she reminds him that her mother died, he sobs as if hearing it for the first time. Which guidance should the counselor offer?

A. Keep reminding him of the death so that the fact is gradually relearned

Repeated reorientation to a painful fact he cannot retain causes fresh grief each time without lasting learning.

B. Ask his physician to raise his sedative dose for the evening hours

Medication is not the first response to distress that can be eased by changing how caregivers respond.

C. Avoid the subject entirely and leave the room whenever he raises it

Withdrawing leaves him alone with distress and does not meet the emotional need behind the question.

D. Respond to the feeling behind the question, then gently redirect him ✓ KEY

Acknowledging the emotion (for example, that he misses her) and redirecting reduces distress without repeated re-traumatizing news.

Rationale: In moderate dementia, a person often cannot retain new information, so repeated correction brings repeated fresh grief. Caregivers are usually taught to respond to the emotion behind a question, offer comfort, and redirect to a soothing activity. Counselors also support caregivers with psychoeducation, skills training, and attention to their own strain, as dementia guidelines recommend.

References: NICE guidelines — NG97 Dementia: supporting carers (psychoeducation and skills training) · Berk (Development Through the Lifespan) — Late adulthood: Alzheimer's disease and caregiving

► **Check:** Validation-and-redirect is standard caregiver teaching rather than a quoted NICE recommendation; SME to confirm locators.

☐ Approve ☐ Revise ☐ Retire Notes:

A 63-year-old with hoarding disorder faces eviction because her apartment is blocked by stacked items. Her landlord has offered to send a crew to clear it out over one weekend. Which approach is MOST consistent with effective treatment of hoarding?

A. Support the weekend cleanout, since clearing the space ends the disorder

Forced cleanouts may solve an immediate crisis but are distressing and often followed by rapid reaccumulation.

B. CBT building sorting and decision skills with practice in discarding ✓ **KEY**

CBT for hoarding targets motivation, organizing and decision-making skills, graded discarding, and reduced acquiring.

C. Insight-oriented talk about her childhood losses before any sorting

Exploring history alone does not change saving and acquiring behavior and delays needed work.

D. Have the counselor decide which items to discard to speed progress

Decisions made for the client undercut her own decision-making practice, which is central to treatment.

Rationale: Hoarding disorder involves persistent difficulty discarding possessions because of a perceived need to save them and distress at discarding, leading to clutter that compromises living areas. Cognitive-behavioral treatment for hoarding uses motivational work, skills training in organizing and decision-making, graded exposure to sorting and discarding, and practice resisting acquiring. Externally imposed cleanouts tend to be traumatic and rarely produce lasting change.

References: DSM-5-TR — Hoarding Disorder: diagnostic features and functional consequences · Corey (Theory & Practice) — Cognitive behavior therapy: skills training and exposure

► **Check:** Hoarding-specific CBT (Steketee & Frost) is not in the approved source library; treatment content cited to general sources. SME to confirm.

☐ Approve ☐ Revise ☐ Retire Notes:

53. nce-s-clf-182 · Assessment and Testing · obsessive-compulsive and related disorders · hard · In review

A 27-year-old is completely certain that her nose is grotesquely misshapen, although others see nothing unusual. She checks mirrors for hours and has had two rhinoplasties, which left her more distressed. She is now seeking a third surgeon. Which conclusion is MOST accurate?

A. BDD with absent insight; more surgery is unlikely to ease her distress ✓ **KEY**

Total conviction is coded as BDD with absent insight/delusional beliefs, and cosmetic procedures rarely relieve BDD.

B. Delusional disorder, somatic type, as her conviction is fixed and total

DSM-5-TR codes delusional appearance beliefs as BDD with absent insight rather than as delusional disorder.

C. BDD with good insight; a skilled surgeon will likely resolve her concern

Complete conviction is not good insight, and surgery typically fails to resolve or worsens BDD symptoms.

D. No disorder applies, because she has sought medical rather than mental care

Many people with BDD seek cosmetic rather than mental health treatment; the pattern fully meets BDD criteria.

Rationale: Body dysmorphic disorder involves preoccupation with perceived flaws not observable or slight to others, with repetitive behaviors such as mirror checking. Insight is specified as good or fair, poor, or absent insight/delusional beliefs; a person completely convinced of the flaw still receives BDD, not a psychotic diagnosis. Many people with BDD seek cosmetic procedures, which seldom improve and can worsen symptoms, so referral for CBT and medication evaluation is indicated.

References: DSM-5-TR — Body Dysmorphic Disorder: insight specifiers; associated features (cosmetic treatment)

☐ Approve ☐ Revise ☐ Retire Notes:

A 14-year-old with OCD spends hours arranging objects and touching doorframes until things feel "just right." His parents mention that between ages 7 and 10 he had repeated eye blinking and grunting that have since stopped. Which DSM-5-TR specifier should be added to his OCD diagnosis?

A. With poor insight

Insight specifiers depend on his beliefs about the obsessions, which are not described.

B. With panic attacks

No abrupt surges of intense fear are described.

C. With absent insight

Nothing suggests he is fully convinced his obsessional beliefs are true.

D. Tic-related ✓ KEY

The tic-related specifier applies to a current or past history of a tic disorder.

Rationale: OCD is specified as tic-related when the person has a current or past history of a tic disorder. Tic-related OCD is more common in males with childhood onset and often involves symmetry, ordering, and "just right" concerns rather than harm or contamination themes. Past tics count, so a careful developmental history matters.

References: DSM-5-TR — Obsessive-Compulsive Disorder: specify if tic-related

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-year-old with average intellectual ability reads slowly and makes many decoding errors. A school team is considering specific learning disorder with impairment in reading. Under DSM-5-TR, which requirement must be met?

A. A large gap between full-scale IQ and reading achievement scores

DSM-5 dropped the IQ-achievement discrepancy requirement; skills must instead be well below age expectations.

B. Difficulty persisting 6 months despite help aimed at that skill ✓ KEY

Criterion A requires difficulties persisting at least 6 months despite interventions that target them.

C. A formal diagnosis made only after the child reaches age 12

Specific learning disorder is typically identified in the early school years; there is no age 12 threshold.

D. Proof that the difficulty also appears in mathematics and writing

Impairment may involve one domain only; reading, written expression, and mathematics are specified separately.

Rationale: DSM-5-TR requires difficulties learning and using academic skills that have persisted for at least 6 months despite targeted intervention, with skills substantially and quantifiably below those expected for age (confirmed by standardized achievement measures and comprehensive clinical assessment). Onset is in the school-age years, and the difficulties are not better explained by intellectual disability, sensory problems, or inadequate instruction. The older IQ-achievement discrepancy model is no longer required.

References: DSM-5-TR — Specific Learning Disorder, Criteria A and B

☐ Approve ☐ Revise ☐ Retire Notes:

In DSM-5-TR, deficits in adaptive functioning for intellectual developmental disorder are assessed in which three domains?

A. Verbal, perceptual, and processing speed

These resemble index scores from intelligence tests, not adaptive functioning domains.

B. Sensory, motor, and expressive language

These are developmental skill areas, not the three DSM-5-TR adaptive domains.

C. Conceptual, social, and practical ✓ KEY

DSM-5-TR assesses adaptive functioning in the conceptual, social, and practical domains.

D. Cognitive, emotional, and behavioral

This is a general psychological triad, not the DSM-5-TR adaptive framework.

Rationale: Intellectual developmental disorder requires deficits in intellectual functions and in adaptive functioning, with onset during the developmental period. Adaptive functioning covers the conceptual (academic), social, and practical (self-management) domains. Severity is set by adaptive functioning in these domains rather than by IQ alone, because adaptive functioning determines the level of support needed.

References: DSM-5-TR — Intellectual Developmental Disorder, Criterion B and severity table

☐ Approve ☐ Revise ☐ Retire Notes:

For the past year, a 10-year-old girl has shown seven inattentive symptoms of ADHD, such as losing materials and drifting off during lessons, but only two hyperactive-impulsive symptoms. Symptoms began before age 12 and appear at home and school. Which presentation should be specified?

A. Predominantly inattentive presentation ✓ KEY

Inattention criteria are met while hyperactivity-impulsivity criteria are not, which defines this presentation.

B. Combined presentation

Combined presentation requires both symptom criteria to be met for the past 6 months.

C. Predominantly hyperactive presentation

She has only two hyperactive-impulsive symptoms, below the threshold.

D. Other specified ADHD

Full criteria are met, so a specified presentation of ADHD applies.

Rationale: ADHD presentations reflect the past 6 months: combined (both inattention and hyperactivity-impulsivity criteria met), predominantly inattentive (inattention met, hyperactivity-impulsivity not), or predominantly hyperactive/impulsive (the reverse). Children need six or more symptoms in a domain; those 17 and older need five. Girls are more often seen with the inattentive presentation, which can delay recognition.

References: DSM-5-TR — Attention-Deficit/Hyperactivity Disorder: specify current presentation

☐ Approve ☐ Revise ☐ Retire Notes:

Which of the following is a negative symptom of schizophrenia?

A. Persecutory delusions

Delusions are positive symptoms, an addition to normal experience.

B. Disorganized speech

Disorganized speech is a separate symptom domain, not a negative symptom.

C. Auditory hallucinations

Hallucinations are positive symptoms.

D. Avolition ✓ **KEY**

Avolition, a decrease in self-initiated purposeful activity, is a core negative symptom.

Rationale: Negative symptoms reflect a loss or reduction of normal functions. DSM-5-TR highlights diminished emotional expression and avolition as especially prominent in schizophrenia; others include alogia, anhedonia, and asociality. Negative symptoms account for much of the long-term disability in schizophrenia and respond less well to antipsychotic medication than positive symptoms.

References: DSM-5-TR — Schizophrenia Spectrum and Other Psychotic Disorders: key features (negative symptoms)

☐ Approve

☐ Revise

☐ Retire

Notes:

A 45-year-old in treatment for severe depression arrives with his wife, who says he has barely spoken or eaten for 3 days. In session he stays mute, holds a stiff, odd posture for long periods, and resists when the counselor gently offers him a cup of water. What should the counselor do FIRST?

A. Begin behavioral activation to restart his routine and appetite

Behavioral activation cannot address a possible medical emergency that needs urgent evaluation first.

B. Use silence and patience until he is ready to talk in session

Waiting delays care for catatonia, which can lead to dehydration and other serious complications.

C. Arrange urgent medical evaluation for possible catatonia today ✓ **KEY**

Mutism, posturing, and negativism suggest catatonia, which needs urgent medical care and is often highly treatable.

D. Ask his wife to keep a food log and report back next week

A week of monitoring is far too slow when he is not eating or drinking and may be catatonic.

Rationale: Catatonia is diagnosed when three or more of twelve psychomotor features are present, such as stupor, mutism, posturing, catalepsy, and negativism. It can occur with depressive, bipolar, psychotic, or medical conditions and may become life-threatening through dehydration, malnutrition, or autonomic instability. It typically responds to medical treatment, so the counselor's first job is to get the client to urgent medical and psychiatric evaluation.

References: DSM-5-TR — Catatonia Associated With Another Mental Disorder (Catatonia Specifier)

► **Check:** Placed under schizophrenia spectrum because catatonia sits in that DSM-5-TR chapter; the client here has depression.

☐ Approve

☐ Revise

☐ Retire

Notes:

A 24-year-old has just been discharged after a first episode of schizophrenia and is moving back home. His parents are frightened and often criticize him for "lying around," and his mother has given up her job to watch him constantly. Which intervention is MOST likely to reduce his risk of relapse?

A. Family psychoeducation that lowers criticism and overinvolvement ✓ KEY

High expressed emotion predicts relapse, and family interventions that reduce it lower relapse rates.

B. Advising the family to limit contact with him until he stabilizes

Reducing contact removes support and does not teach the family more helpful ways to respond.

C. Individual insight therapy exploring his early family relationships

Insight-oriented therapy is not an evidence-based relapse-prevention approach for schizophrenia.

D. Encouraging his parents to confront his low motivation more firmly

Firmer confrontation increases criticism, a component of high expressed emotion linked to relapse.

Rationale: Expressed emotion describes family attitudes of criticism, hostility, and emotional overinvolvement toward a member with a mental illness. High expressed emotion reliably predicts relapse in schizophrenia. Family psychoeducation and family intervention teach relatives about the illness, problem-solving, and calmer communication, and they reduce relapse and rehospitalization.

References: NICE guidelines — CG178 Psychosis and schizophrenia in adults: family intervention · Goldenberg (Family Therapy) — Psychoeducational family therapy and expressed emotion

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old arrives in a flamboyant outfit and calls the counselor "my new best friend" at their first meeting. She is upset when attention shifts away from her, speaks dramatically but vaguely, and her emotions change quickly. This pattern has lasted since her late teens. Which personality disorder BEST fits?

A. Narcissistic personality disorder

Narcissistic personality centers on grandiosity, entitlement, and need for admiration rather than shallow, shifting emotion.

B. Histrionic personality disorder ✓ KEY

Attention-seeking, rapidly shifting shallow emotion, impressionistic speech, and overestimating intimacy fit histrionic personality.

C. Borderline personality disorder

Borderline personality centers on abandonment fears, unstable identity, and self-harm, which are not described.

D. Dependent personality disorder

Dependent personality centers on needing others to make decisions and fear of being alone.

Rationale: Histrionic personality disorder is a pervasive pattern of excessive emotionality and attention-seeking beginning by early adulthood. Features include discomfort when not the center of attention, rapidly shifting and shallow emotions, use of appearance to draw attention, impressionistic speech lacking detail, theatrical expression, suggestibility, and viewing relationships as more intimate than they are. At least five of eight criteria are required.

References: DSM-5-TR — Histrionic Personality Disorder, diagnostic criteria

☐ Approve ☐ Revise ☐ Retire Notes:

Since adolescence, a 29-year-old has avoided almost all new relationships and job promotions, believing she is inferior and certain to be rejected. She also has marked fear of being judged when eating or speaking in front of others. How does DSM-5-TR describe avoidant personality disorder in relation to social anxiety disorder?

A. Social anxiety disorder must be ruled out before avoidant PD is given

DSM-5-TR does not make social anxiety disorder an exclusion for avoidant personality disorder.

B. Avoidant PD is diagnosed only when fear is limited to one situation

Avoidant personality disorder is pervasive across contexts, not limited to a single situation.

C. The two are unrelated, as one is a mood state and the other a trait

DSM-5-TR describes heavy overlap between them; neither is a mood disorder.

D. They overlap heavily and both may be diagnosed if each is fully met ✓ KEY

DSM-5-TR notes great overlap, possibly alternative conceptualizations, and both can be diagnosed when criteria are met.

Rationale: Avoidant personality disorder is a pervasive pattern of social inhibition, feelings of inadequacy, and hypersensitivity to negative evaluation beginning by early adulthood. DSM-5-TR notes so much overlap with social anxiety disorder that they may be alternative conceptualizations of the same or similar conditions. When criteria for both are met, both may be diagnosed; avoidant PD tends to reflect a broader, identity-level sense of inadequacy across most interpersonal settings.

References: DSM-5-TR — Avoidant Personality Disorder: differential diagnosis (social anxiety disorder)

☐ Approve ☐ Revise ☐ Retire Notes:

Early in counseling, a client with borderline personality disorder calls the counselor several times a week between sessions, often late at night, and feels rejected when calls are not returned at once. The counselor is growing frustrated. Which response BEST reflects an effective treatment frame?

A. Return every call at once so she does not feel abandoned in a crisis

Unlimited availability is unsustainable, can reinforce crisis behavior, and risks counselor burnout.

B. Tell her calls between sessions are not allowed under any condition

A blanket ban leaves her without coaching support and may feel like the rejection she fears.

C. Set clear, agreed limits on contact, with brief skills-coaching calls ✓ KEY

A collaborative, consistent frame with planned coaching contact supports skill use while protecting the work.

D. Refer her elsewhere, since frequent calls show she is not yet ready

Frequent contact is a treatment target, not a reason to abandon the client.

Rationale: Evidence-based treatments for borderline personality disorder share a clear, consistent treatment frame agreed with the client. In dialectical behavior therapy, between-session phone coaching is brief and focused on applying skills, with limits the therapist observes and explains openly. A predictable frame reduces fears of abandonment, discourages reinforcement of crisis behavior, and is supported by consultation for the counselor.

References: Linehan (DBT Skills Training Manual) — DBT treatment modes: between-session phone coaching and observing limits

☐ Approve ☐ Revise ☐ Retire Notes:

An adult client meets criteria for binge-eating disorder and asks which psychological treatment has the strongest evidence. Which approach should the counselor describe as first-line?

A. Eating-disorder-focused cognitive behavioral therapy ✓ KEY

CBT focused on eating, including guided self-help, is the first-line psychological treatment for binge-eating disorder.

B. A strict diet plan aimed mainly at quick weight loss

Dieting is not a treatment for binge eating and rigid restriction can intensify the binge cycle.

C. Long-term psychodynamic therapy as the sole treatment

Psychodynamic therapy lacks the evidence base that CBT has for binge-eating disorder.

D. Hypnosis to suppress cravings for specific trigger foods

Hypnosis is not an evidence-based first-line treatment for binge-eating disorder.

Rationale: Clinical guidelines recommend eating-disorder-focused CBT for binge-eating disorder, often beginning with a guided self-help program and moving to group or individual CBT if needed. Treatment targets regular eating, the binge cycle, triggers, and the over-evaluation of shape and weight. Weight loss is not the primary aim, and restrictive dieting can maintain binge eating.

References: NICE guidelines — NG69 Eating disorders: treating binge eating disorder (guided self-help; group CBT-ED)

☐ Approve ☐ Revise ☐ Retire Notes:

A 17-year-old has lost a significant amount of weight over a year through restrictive eating. She intensely fears weight gain and judges herself almost entirely by her shape. Her physician notes that, despite the loss, her weight remains within the normal range for her age and height. Which diagnosis BEST fits?

A. Anorexia nervosa, restricting type, with mild severity

Anorexia nervosa requires significantly low body weight, which is not present here.

B. Other specified feeding or eating disorder (atypical AN) ✓ KEY

Atypical anorexia nervosa is listed under OSFED when all AN criteria are met except low weight.

C. Avoidant/restrictive food intake disorder, adolescent onset

ARFID excludes restriction driven by concerns about body shape or weight.

D. Bulimia nervosa, low frequency and limited duration

No binge eating or compensatory behaviors are described.

Rationale: DSM-5-TR lists atypical anorexia nervosa as an example of other specified feeding or eating disorder: all criteria for anorexia nervosa are met except that, despite significant weight loss, weight is within or above the normal range. Medical risks can be serious despite a normal-appearing weight, so these clients need the same medical monitoring and specialized care as those with anorexia nervosa.

References: DSM-5-TR — Other Specified Feeding or Eating Disorder: atypical anorexia nervosa

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 14-year-old recently diagnosed with anorexia nervosa ask what treatment the eating disorder team will recommend. She is medically stable for outpatient care. Which description BEST fits family-based treatment (FBT) for adolescent anorexia?

A. The teen meets alone with a therapist to explore the roots of her illness

FBT is family based and does not focus on uncovering causes; individual insight work is a different model.

B. Parents step back from meals so that the teen regains a sense of control

FBT does the opposite in its first phase, placing parents in charge of restoring healthy eating.

C. The family first explores how parenting errors caused the eating disorder

FBT takes an agnostic stance on cause and explicitly avoids blaming parents.

D. Parents take charge of renourishment at first, then gradually hand it back ✓ **KEY**

FBT empowers parents to lead renourishment first, then returns control to the teen as she recovers.

Rationale: Family-based treatment (the Maudsley approach) is a leading evidence-based treatment for adolescent anorexia nervosa and is reflected in guideline recommendations for eating-disorder-focused family therapy. It proceeds in phases: parents take charge of restoring healthy eating, control over eating is gradually returned to the adolescent, and then broader adolescent development is addressed. The model does not blame parents and separates the illness from the young person.

References: NICE guidelines — NG69 Eating disorders: anorexia nervosa in children and young people (eating-disorder-focused family therapy)

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old in a major depressive episode brightens noticeably when friends invite her out, then sinks again. She sleeps far more than usual, eats much more, describes her arms and legs as feeling "like lead," and has long been crushed by small hints of criticism. Which specifier BEST fits?

A. With melancholic features

Melancholic features involve loss of reactivity to pleasant events, early waking, and decreased appetite, the reverse of this picture.

B. With anxious distress

Anxious distress involves tension, restlessness, worry, and fear, which are not described.

C. With atypical features ✓ **KEY**

Mood reactivity plus hypersomnia, increased appetite, leaden paralysis, and rejection sensitivity define atypical features.

D. With mixed features

Mixed features require manic or hypomanic symptoms during the episode, which are absent.

Rationale: The "with atypical features" specifier requires mood reactivity (mood brightens in response to positive events) plus at least two of: significant weight gain or increased appetite, hypersomnia, leaden paralysis, and a long-standing pattern of sensitivity to interpersonal rejection. Criteria for melancholic features or catatonia must not be met in the same episode. Despite its name, this pattern is common.

References: DSM-5-TR — Specifiers for Depressive Disorders: with atypical features

☐ Approve ☐ Revise ☐ Retire Notes:

A 32-year-old nurse with bipolar I disorder, stable on medication, is about to switch to rotating night shifts. Her past manic episodes followed weeks of disrupted sleep. Which counseling focus is MOST consistent with interpersonal and social rhythm therapy (IPSRT)?

A. Keep daily routines and sleep-wake times as steady as she can ✓ KEY

IPSRT stabilizes daily rhythms, especially sleep, because disruptions can trigger mood episodes.

B. Explore early memories to find the roots of her mood swings

Uncovering childhood roots is not the IPSRT focus on present rhythms and relationships.

C. Dispute the irrational beliefs that drive her elevated moods

Disputing irrational beliefs comes from REBT, not IPSRT.

D. Build a fear hierarchy for in-vivo exposure to her workplace

Exposure hierarchies treat anxiety disorders; they do not address rhythm disruption.

Rationale: Interpersonal and social rhythm therapy, developed by Ellen Frank, holds that disruptions to daily routines and sleep-wake cycles can trigger mood episodes in people with bipolar disorder. Clients track daily rhythms (for example, with a social rhythm metric), work to keep them regular, and address interpersonal problems and role transitions. A shift to night work is a classic rhythm disruption to plan for.

References: IPSRT — Social rhythm stabilization and the social rhythm metric

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old nursing student has fainted twice while watching blood draws and now avoids clinical rotations. She has specific phobia, blood-injection-injury type. When planning exposure, which added technique is MOST suited to this phobia type?

A. Progressive muscle relaxation before and during each exposure

Relaxation can deepen the drop in blood pressure behind fainting, so it is generally avoided for this type.

B. Applied tension, tensing large muscles to keep blood pressure up ✓ KEY

Applied tension counters the vasovagal blood pressure drop that causes fainting in this phobia type.

C. Interoceptive exposure to dizziness by spinning in a chair first

Interoceptive exposure targets fear of bodily sensations in panic disorder, not vasovagal fainting.

D. Slow diaphragmatic breathing to lower her heart rate and arousal

Lowering arousal does not address, and may worsen, the vasovagal response.

Rationale: Unlike other specific phobias, the blood-injection-injury type often involves a vasovagal response: a brief rise in heart rate and blood pressure followed by a sharp drop that can cause fainting. Applied tension teaches the client to tense large muscle groups to raise blood pressure while being exposed to feared stimuli. Relaxation-based methods are generally avoided because they can make fainting more likely.

References: DSM-5-TR — Specific Phobia: blood-injection-injury type (vasovagal fainting response)

► **Check:** Applied tension (Öst) is not described in DSM-5-TR; no approved source names it directly. SME to confirm or add a citation.

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old meets full criteria for PTSD after a violent home invasion. She also reports that she often feels as though she is watching herself from outside her body and that the world seems dreamlike and unreal. These experiences are not caused by substances or a medical condition. How should this be diagnosed?

A. PTSD plus a separate diagnosis of depersonalization/derealization disorder

When these symptoms occur as part of PTSD, they are captured with the specifier, not a separate disorder.

B. Acute stress disorder, as dissociative symptoms are central to that diagnosis

Full PTSD criteria are already met, and acute stress disorder is limited to the first month after trauma.

C. PTSD with delayed expression, given the presence of dissociative symptoms

Delayed expression refers to full criteria not being met until at least 6 months after the event.

D. PTSD with dissociative symptoms, noting depersonalization and derealization ✓ KEY

The dissociative specifier applies to persistent or recurrent depersonalization or derealization in PTSD.

Rationale: DSM-5-TR allows PTSD to be specified "with dissociative symptoms" when the person also has persistent or recurrent depersonalization (feeling detached from oneself, as an outside observer) or derealization (the world feels unreal, dreamlike, or distant), not attributable to substances or a medical condition. Recognizing this subtype matters because dissociation can affect pacing and stabilization in trauma-focused treatment.

References: DSM-5-TR — Posttraumatic Stress Disorder: specify with dissociative symptoms

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old with opioid use disorder has taken prescribed buprenorphine for 8 months and has not used illicit opioids. In a recovery group, peers tell him he is "not really clean" and urge him to stop the medication. He asks the counselor what to think. Which response is MOST consistent with best practice?

A. Agree that full recovery requires tapering off all opioid medicines soon

No fixed endpoint is required; stopping medication early raises the risk of relapse and overdose.

B. Suggest he follow the group's view, since peer support predicts recovery

Peer support is valuable, but the group's advice here conflicts with the evidence on medication for OUD.

C. Affirm that MOUD is effective treatment, and support talking to his prescriber ✓ KEY

Medication for OUD is evidence-based treatment, and medication decisions belong with the client and prescriber.

D. Advise him to keep his medication secret from others in the recovery group

Encouraging secrecy does not address stigma or help him find supportive recovery settings.

Rationale: Medications for opioid use disorder, such as buprenorphine, methadone, and extended-release naltrexone, reduce illicit use, overdose deaths, and other harms. Taking prescribed medication as directed is treatment, not "substituting one addiction for another." Counselors should challenge stigma, support informed choices, coordinate with prescribers, and help clients find MOUD-friendly recovery supports; duration of medication is individualized, and discontinuing it raises relapse and overdose risk.

References: SAMHSA TIP 63 — Part 1: MOUD effectiveness; Part 4: counseling and peer support for clients on MOUD

► **Check:** SME to confirm the TIP 63 part numbers in the locator.

☐ Approve ☐ Revise ☐ Retire Notes:

Two years after her husband died, a 70-year-old has returned to her volunteer work and made new friends. She still talks to him at his grave on their anniversary and keeps his watch on her nightstand. Her adult children ask the counselor to help her "finally let go." Which response BEST reflects current grief theory?

A. Agree that she must give up the attachment before she can heal

Older "letting go" models have given way to the view that healthy bonds with the deceased can continue.

B. Explain that a lasting bond with him can coexist with a full life ✓ KEY

Continuing bonds, and Worden's fourth task of finding an enduring connection, describe adaptive grief.

C. Assess her for prolonged grief disorder due to the grave visits

She is functioning well; occasional rituals of connection are not signs of prolonged grief disorder.

D. Suggest she put his watch away to signal the grief has ended

Keeping meaningful objects can be part of a healthy continuing bond; removing them is not required.

Rationale: Contemporary grief theory views ongoing connection with the deceased as compatible with healthy adaptation. Worden's fourth task of mourning is to find an enduring connection with the deceased while embarking on a new life, and the continuing bonds perspective holds that memories, rituals, and inner conversations can support coping. Concern arises when connection prevents re-engagement in life, which is not the case here.

References: Worden (Grief Counseling) — Task IV: find an enduring connection with the deceased while embarking on a new life

☐ Approve ☐ Revise ☐ Retire Notes:

Using James and Gilliland's six-step crisis model, a counselor has defined the problem with a client whose house burned down, ensured his safety, and offered support. The client is now calmer. Which step comes NEXT?

A. Obtaining his commitment to a plan

Commitment is the sixth and final step, after alternatives are explored and a plan is made.

B. Making a concrete plan with him

Planning is the fifth step and follows examining alternatives.

C. Redefining the core problem again

Defining the problem is the first step, which has already been completed.

D. Examining alternatives with him ✓ KEY

After the three listening steps, the fourth step is examining alternatives.

Rationale: James and Gilliland's six-step model has three listening steps (defining the problem, ensuring client safety, providing support) followed by three acting steps (examining alternatives, making plans, obtaining commitment). Assessment runs throughout all six steps, and safety is monitored continuously, but the sequence guides the counselor from understanding to action.

References: James & Gilliland (Crisis Intervention Strategies) — Six-step model of crisis intervention

☐ Approve ☐ Revise ☐ Retire Notes:

During a risk assessment, a client experiencing partner abuse discloses several things about her partner. Which disclosure MOST strongly signals elevated risk of lethal violence and calls for urgent safety planning?

A. He has choked her until she could barely breathe in the past ✓ **KEY**

Prior non-fatal strangulation is one of the strongest known predictors of later intimate partner homicide.

B. He often yells and calls her names when he gets home late

Verbal abuse is harmful but is a weaker predictor of lethality than strangulation.

C. He grew up in a home where his father hit his mother often

Childhood exposure to violence is a risk factor for perpetration but a weaker lethality marker.

D. He sulks and won't speak to her for days after arguments

Withdrawal after conflict does not signal elevated lethality.

Rationale: Lethality assessment in intimate partner violence focuses on factors linked to homicide, such as prior strangulation, access to firearms, threats to kill, escalating frequency or severity, extreme jealousy, and a recent or planned separation. Non-fatal strangulation markedly raises the risk of later homicide and may leave few visible injuries, so counselors should ask about it directly and respond with urgent safety planning and advocacy referrals.

References: James & Gilliland (Crisis Intervention Strategies) — Intimate partner violence: assessing lethality

► **Check:** The strangulation-lethality link comes from Campbell's Danger Assessment research, which is not in the approved library; SME to confirm the James & Gilliland locator.

☐ Approve ☐ Revise ☐ Retire Notes:

A 3-year-old has delays in speech, motor skills, and self-care, but cannot yet complete standardized intellectual testing in a valid way. The evaluating team plans to reassess in a year. Which DSM-5-TR diagnosis BEST fits for now?

A. Intellectual disability, mild

Severity cannot yet be reliably assessed, so a specified level of intellectual disability is premature.

B. Global developmental delay ✓ **KEY**

This diagnosis is reserved for children under 5 who miss milestones in several areas and cannot yet be reliably assessed.

C. Autism spectrum disorder, level 1

No social communication deficits or restricted, repetitive behaviors are described.

D. Language disorder, expressive type

Delays extend well beyond language to motor and self-care areas.

Rationale: Global developmental delay is diagnosed in children under age 5 when they fail to meet expected milestones in several areas of intellectual functioning and the clinical severity cannot be reliably assessed, often because the child is too young for standardized testing. It requires reassessment after a period of time.

References: DSM-5-TR — Global Developmental Delay

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old previously met full criteria for alcohol use disorder. For the past 5 months he has met no criteria except strong cravings, which come mainly when he passes his old bar. He lives at home, not in a controlled setting. Which specifier applies?

A. In sustained remission, since he has gone months without drinking

Sustained remission requires 12 months or longer without meeting criteria other than craving.

B. No remission specifier, because his ongoing craving is a criterion

Craving is the one criterion explicitly allowed during remission, so it does not block the specifier.

C. In early remission, as only craving has been present for 5 months ✓ **KEY**

Early remission applies after 3 months but less than 12 months with no criteria met except craving.

D. In early remission, in a controlled environment, given the time

He lives at home; the controlled-environment specifier is for settings where access is restricted.

Rationale: For substance use disorders, "in early remission" applies when, after full criteria were met, none have been met for at least 3 months but less than 12 months, with the exception of craving. "In sustained remission" requires 12 months or longer. "In a controlled environment" is added only when the person is in a setting where access to substances is restricted.

References: DSM-5-TR — Alcohol Use Disorder: remission specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old violinist is comfortable at parties, in meetings, and on dates, but for a year has felt intense dread before solo recitals, fearing the audience will judge her shaking hands. She has started declining solo work. If criteria for social anxiety disorder are met, which specifier applies?

A. With panic attacks

No abrupt surges of intense fear are described, and this specifier does not capture the restricted context.

B. Generalized type

DSM-5 removed the generalized specifier for social anxiety disorder.

C. With poor insight

Social anxiety disorder has no insight specifier in DSM-5-TR.

D. Performance only ✓ **KEY**

This specifier applies when the fear is restricted to speaking or performing in public.

Rationale: Social anxiety disorder may be specified "performance only" when the fear is restricted to speaking or performing in public. People with this pattern are often impaired mainly in professional or academic roles that require performing, while ordinary social interactions are comfortable. DSM-5 replaced the older "generalized" specifier with this narrower one.

References: DSM-5-TR — Social Anxiety Disorder: specify if performance only

☐ Approve ☐ Revise ☐ Retire Notes:

A 4-year-old was in a serious car crash 3 months ago. Since then she repeatedly crashes toy cars together while making siren noises, cries when buckled into her car seat, and clings to her parents at bedtime. She shows no visible distress during the crash play. How should the counselor understand the repetitive play?

A. Play reenactment can express intrusion symptoms in young children ✓ **KEY**

For children 6 and younger, intrusive memories may appear as play reenactment and need not look distressing.

B. The play is ordinary for her age and unrelated to the crash itself

Repetitive play that mirrors the trauma, alongside avoidance and clinging, points to a trauma response.

C. Because she seems calm during the play, PTSD can be ruled out here

DSM-5-TR notes that intrusive memories in young children may not appear distressing.

D. The play shows the trauma is resolved and needs no further attention

Ongoing reenactment with avoidance and distress at reminders suggests the trauma is not resolved.

Rationale: DSM-5-TR includes separate PTSD criteria for children 6 years and younger. Intrusion symptoms may take the form of play reenactment, and spontaneous intrusive memories may not appear distressing. The avoidance and negative-cognition clusters are combined, so only one symptom from that combined group is needed. Counselors should assess such play in context and involve caregivers in trauma-focused treatment.

References: DSM-5-TR — Posttraumatic Stress Disorder for Children 6 Years and Younger, Criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

A 73-year-old retired teacher with no history of anxiety reports four months of constant worry, a racing heart, restlessness and poor sleep. She recently began two new medications and says, "I just feel keyed up all the time." What should the counselor do FIRST?

A. Begin weekly CBT for generalized anxiety and give a worry log

CBT may help later, but new-onset anxiety in late life needs a medical cause ruled out before a psychological plan is set.

B. Teach slow breathing and suggest she cut back on her caffeine

Coping tips are reasonable but skip the key step of checking for medical and medication causes of her symptoms.

C. Encourage a medical review of her health and new medications ✓ **KEY**

Late-onset anxiety often has medical or medication contributors (e.g., thyroid, cardiac, drug effects), so medical review comes first.

D. Reassure her that worry is a normal response to growing older

Persistent anxiety is not a normal part of aging; dismissing it reflects ageist assumptions and delays care.

Rationale: Anxiety that first appears in later life is more often linked to medical illness, medication side effects or interactions, or early cognitive change than anxiety that began earlier. Older adults also tend to describe anxiety in somatic terms. Counselors coordinate a medical evaluation first, then provide evidence-based psychological treatment such as CBT, which is effective for older adults.

References: Berk (Development Through the Lifespan) — Late adulthood: physical health, medication use and mental health

☐ Approve ☐ Revise ☐ Retire Notes:

A home-health aide refers an 83-year-old man who lives alone. His home has spoiled food, piled-up mail and no working heat, and he has missed his heart medication for weeks. No one else is involved in his care. He tells the counselor, "I'm fine. Leave me be." Which response BEST fits the counselor's obligations?

A. Assess his safety and capacity and follow state rules on reporting to APS ✓ KEY

Self-neglect that threatens health is reportable to Adult Protective Services in most states, and capacity shapes the response; both must be assessed.

B. Honor his stated wishes and close the case, since he is a competent adult

His capacity has not been assessed, and serious self-neglect may trigger reporting duties regardless of his preference.

C. Arrange for him to move to a nursing home before the cold weather comes

Counselors do not unilaterally relocate adults; least-restrictive options and his rights must be weighed through proper channels.

D. Ask the aide to keep watching and decide again at the next monthly visit

No heat and missed cardiac medication are current dangers; waiting a month risks serious harm.

Rationale: Elder self-neglect, an older adult's failure to meet his own basic needs in ways that threaten health or safety, is the most common type of case reported to Adult Protective Services, and most states treat it as reportable. Counselors balance respect for autonomy against safety by assessing decision-making capacity and immediate risk, following state reporting law, and favoring the least-restrictive supports.

References: State law — Adult protective services statutes: self-neglect and vulnerable-adult reporting · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

► **Check:** Whether self-neglect is reportable, and who is a mandated reporter of vulnerable-adult maltreatment, varies by state; SME to confirm the "most states" framing.

☐ Approve ☐ Revise ☐ Retire Notes:

During a community talk on aging, an attendee says, "Getting old means your mind goes and you end up lonely and miserable." Which statement BEST reflects life-span research on typical aging?

A. Memory and reasoning decline steeply for most people after age sixty

Some abilities slow, but steep, global decline is not typical of normal aging; it suggests a neurocognitive disorder.

B. Most older adults become more depressed as their social circles shrink

Smaller networks reflect chosen focus on close ties; depression is not a normal outcome of aging.

C. Crystallized knowledge and processing speed both fall sharply with age

Processing speed slows, but crystallized knowledge such as vocabulary is stable or grows into late life.

D. Emotional well-being is often stable or better, though processing slows ✓ KEY

Older adults often report steady or improved emotional well-being, while fluid abilities like speed show gradual slowing.

Rationale: Research finds that emotional well-being tends to hold steady or improve in late adulthood. Carstensen's socioemotional selectivity theory explains that older adults focus on fewer, more meaningful relationships and positive emotional experience. Cognitively, processing speed and some fluid abilities slow, but crystallized knowledge stays stable, and severe decline is not a normal part of aging.

References: Berk (Development Through the Lifespan) — Late adulthood: socioemotional selectivity theory; fluid vs crystallized intelligence

☐ Approve ☐ Revise ☐ Retire Notes:

A high school teacher tells the school counselor that a 15-year-old in her class has cuts on her arm. The teacher says, "She is just doing it for attention, so I plan to ignore it so I don't reward it." How should the counselor BEST respond to the teacher?

A. Agree that ignoring it is wise, since attention can reinforce cutting

Ignoring self-injury leaves the student without assessment or help and rests on a common myth about its purpose.

B. Explain cutting usually eases distress, and arrange to see the student ✓ **KEY**

Self-injury most often regulates painful emotion, and the student needs a prompt, calm assessment that includes suicide risk.

C. Ask the teacher to tell the student to stop and to check her arms daily

Demands and body checks by staff tend to increase shame and secrecy and are not a substitute for assessment.

D. Suggest the teacher discuss self-injury with the whole class that day

Class discussion of self-injury can spread it through social contagion and exposes the student.

Rationale: Nonsuicidal self-injury is most often used privately to relieve intense negative emotion, not to gain attention. Even when it is not suicidal, it is a risk factor for later suicide attempts, so any student who self-injures needs an assessment that includes suicide risk. Counselors consult with staff to replace the "attention" myth with a calm, supportive referral pathway.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal self-injury disorder (functions and course) · Linehan (DBT Skills Training Manual) — Self-harm as an emotion-regulation behavior

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old has cut herself repeatedly for a year and has made one suicide attempt. She has intense mood swings and frequent conflict with her parents. Which treatment approach has the STRONGEST research support for reducing self-harm in adolescents like her?

A. Dialectical behavior therapy adapted for teens, with family skills work ✓ **KEY**

DBT for adolescents, which includes caregivers in skills training, has randomized trial support for reducing self-harm.

B. Client-centered therapy offered weekly in a supportive, open format

Supportive therapy builds rapport but has weaker evidence than DBT-A for reducing repeated self-harm.

C. Psychoeducation for her parents alone, with no individual sessions

Parent education helps but cannot replace skills training and individual treatment for a high-risk teen.

D. A peer support group where members discuss their self-harm stories

Groups that focus on sharing self-harm can increase contagion and are not an evidence-based first choice.

Rationale: Dialectical behavior therapy for adolescents (DBT-A) combines individual therapy, multifamily skills groups (mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness, and walking the middle path), phone coaching and team consultation. Randomized trials show it reduces self-harm and suicide attempts in adolescents with repeated self-harm.

References: Linehan (DBT Skills Training Manual) — DBT skills modules; adaptation for adolescents and families

► **Check:** DBT-A trial evidence (Mehlum et al., 2014; McCauley et al., 2018) is not in the approved source list; SME to confirm the Linehan locator covers the adolescent adaptation.

☐ Approve ☐ Revise ☐ Retire Notes:

A 76-year-old widow goes to bed at 8 p.m., lies awake for hours, naps twice a day, and gets up at 7 a.m. feeling unrested. She asks the counselor whether she should ask her doctor for a sleeping pill. Which recommendation is MOST appropriate?

A. A sleeping pill, since medication works best for insomnia in later life

Sedative-hypnotics raise the risk of falls, confusion and dependence in older adults and are not first-line.

B. More time in bed and longer naps so that she can catch up on lost sleep

Extra time in bed and naps weaken sleep drive and fragment night sleep further.

C. CBT for insomnia, including less time in bed and fewer daytime naps ✓ **KEY**

CBT-I is first-line for chronic insomnia at any age; limiting time in bed and naps builds sleep drive and consolidates sleep.

D. Acceptance that broken sleep is normal aging and needs no treatment

Sleep becomes lighter with age, but distressing insomnia with daytime effects is treatable and should not be dismissed.

Rationale: Cognitive behavioral therapy for insomnia is the recommended first-line treatment for chronic insomnia, including in older adults. Sleep restriction (or the gentler sleep compression) matches time in bed to actual sleep, and limiting naps strengthens sleep drive. Hypnotic medications carry added risks of falls and cognitive effects in later life.

References: AASM Clinical Practice Guideline (Insomnia) — CBT-I and sleep restriction therapy for chronic insomnia (strong recommendation)

☐ Approve ☐ Revise ☐ Retire Notes: _____

A factory worker was moved to a permanent overnight shift six months ago. He cannot stay asleep when he goes to bed at 9 a.m. and fights to stay awake at work around 4 a.m. His sleep was normal on his old day schedule. Which DSM-5-TR diagnosis BEST fits?

A. Hypersomnolence disorder

Hypersomnolence involves excess sleepiness despite adequate main sleep, not a mismatch with a work schedule.

B. Insomnia disorder

His sleep loss is explained by a circadian mismatch with his shift, which a circadian diagnosis better captures.

C. Delayed sleep phase type

Delayed sleep phase is a late internal clock, not a problem caused by night-shift work.

D. Circadian disorder, shift work type ✓ **KEY**

Insomnia during the sleep period and sleepiness during work, tied to a night-shift schedule, defines shift work type.

Rationale: Circadian rhythm sleep-wake disorder, shift work type, involves insomnia during the major sleep period and/or excessive sleepiness during the major awake period, associated with a shift work schedule such as night work. Counseling supports sleep scheduling, light management and protecting daytime sleep.

References: DSM-5-TR — Circadian Rhythm Sleep-Wake Disorders: shift work type

☐ Approve ☐ Revise ☐ Retire Notes: _____

A new principal asks the high school counselor to serve as the full-time state testing coordinator and to handle discipline for tardy students. The counselor is concerned that these duties will leave little time for students. Which response BEST reflects the ASCA National Model?

A. Accept both duties, since the principal sets the counselor's role

The ASCA model expects counselors to advocate for an appropriate role rather than simply absorb noncounseling duties.

B. Use data on student needs to propose shifting noncounseling duties ✓ **KEY**

The model calls for using data and an annual agreement with the principal to keep most time on direct and indirect student services.

C. Refuse both duties and file a complaint with the district office

Refusal and escalation damage collaboration; the model favors data-based advocacy with the principal first.

D. Take on discipline only, since it gives contact with at-risk students

ASCA lists handling discipline as inappropriate for counselors because it conflicts with the helping role.

Rationale: The ASCA National Model recommends that school counselors spend about 80% of their time in direct and indirect services to students. Coordinating schoolwide testing and administering discipline are listed as inappropriate duties. Counselors use data and an annual administrative conference to advocate for a role focused on academic, career and social/emotional development.

References: ASCA National Model — Appropriate vs inappropriate activities for school counselors · Gladding (Counseling: A Comprehensive Profession) — School counseling: the ASCA National Model

☐ Approve ☐ Revise ☐ Retire Notes:

An 11th grader studies hard and knows the material, but during exams her heart pounds, her mind goes blank, and she thinks, "If I fail this, my future is over." Her grades are dropping. Which intervention should the school counselor offer FIRST?

A. Relaxation training plus work on her catastrophic test thoughts ✓ **KEY**

Test anxiety responds well to combined arousal-reduction and cognitive restructuring, which target both parts of her reaction.

B. A referral for psychoeducational testing for a learning disability

She knows the material; the pattern points to anxiety during tests, not a learning disability.

C. More hours of studying each night so she feels better prepared

She is already prepared; more study does not address the anxiety that blocks retrieval.

D. A request to excuse her from exams for the rest of the semester

Avoidance maintains anxiety and does not build the skills she needs.

Rationale: Test anxiety has physiological (arousal), cognitive (worry and catastrophic thinking) and behavioral parts. Cognitive behavioral approaches that combine relaxation or systematic desensitization with cognitive restructuring, plus test-taking skills when needed, are well supported. Avoidance tends to strengthen the anxiety.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: test anxiety interventions · Corey (Theory & Practice) — Cognitive behavior therapy: relaxation training and cognitive restructuring

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-year-old in a gifted program reads at a high school level and debates ethics with adults, yet melts down when he loses a board game and struggles to make friends his age. His parents ask how he can be "so mature and so immature." Which concept BEST explains this?

A. A twice-exceptional profile that signals a hidden learning disability

Twice-exceptional means giftedness with a disability; nothing here indicates a disability.

B. Emotional immaturity caused by too much praise for his intellect

This blames parenting without evidence; uneven development is common in gifted children.

C. Asynchronous development across his intellectual and social growth ✓ **KEY**

Gifted children often develop unevenly, with intellect far ahead of social and emotional skills that match their age.

D. Early signs of a disruptive mood dysregulation disorder pattern

DMDD requires severe, frequent outbursts and persistent irritability across settings; this is not described.

Rationale: Asynchronous development means a gifted child's intellectual abilities run well ahead of social, emotional or physical development. This uneven profile can produce frustration, perfectionism and difficulty finding peers. Counselors normalize the pattern for families, support social-emotional skills, and help the child find intellectual peers.

References: Berk (Development Through the Lifespan) — Middle childhood: giftedness, creativity and talent

► **Check:** The term "asynchronous development" comes from gifted-education literature (Columbus Group; Silverman); SME to confirm Berk covers it or swap the reference.

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old Army veteran led a 12-person logistics team for six years. Applying for civilian jobs, his resume lists military job codes and unit titles, and he has had no interviews. He says, "Nobody out here values what I did." What should the counselor do FIRST?

A. Steer him toward entry-level roles until he gains civilian work history

This undervalues his experience and could lead to underemployment.

B. Give an interest inventory to find a field unrelated to his service

His issue is translating proven skills, not a lack of known interests.

C. Explore his anger at civilians as a possible sign of combat trauma

His frustration is an understandable response to rejection; nothing suggests a trauma reaction.

D. Help him translate his military duties into civilian skills and terms ✓ **KEY**

*Translating military roles into transferable civilian skills, using tools like the O*NET military crosswalk, is a key first step.*

Rationale: Many veterans have strong leadership, logistics and technical skills that civilian employers do not recognize when described in military language. Career counselors help veterans identify transferable skills, translate job codes and titles into civilian terms (for example, with the O*NET military crosswalk), and connect with veteran employment resources.

References: Sharf (Career Development Theory) — Career counseling with special populations: transferable skills and occupational information

► **Check:** Veteran-specific content may not be covered directly in Sharf; SME to confirm the locator.

☐ Approve ☐ Revise ☐ Retire Notes:

A career counselor is screening clients for a program that serves people who are underemployed. Which client BEST fits the definition of underemployment?

A. A nurse who left work by choice to care for her newborn twins

Leaving the labor force by choice is not underemployment.

B. An engineer stocking shelves part time who wants full-time work ✓ **KEY**

Working part time involuntarily in a job well below his training fits underemployment.

C. A teacher on summer break who will return to her job in the fall

A planned seasonal break from a full-time job is not underemployment.

D. A laid-off welder who has been looking for a job for six months

Someone without a job who is seeking work is unemployed, not underemployed.

Rationale: Underemployment describes working fewer hours than desired (involuntary part-time work) or in a job that does not use one's education, skills or experience. It is associated with lower well-being and job satisfaction, much as unemployment is, so counselors address both the practical job search and its emotional toll.

References: Sharf (Career Development Theory) — Adult career crises and transitions: unemployment and underemployment

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old woman with top performance reviews has been passed over three times for vice president. Each time, a man with a similar record was chosen. She tells her counselor, "I must not be leadership material." Which counselor response is BEST?

A. Focus on assertiveness training to correct her leadership weaknesses

This assumes a personal deficit and ignores the strong evidence of her performance.

B. Suggest she lower her goals to match what the company is likely to offer

Lowering goals accepts a barrier as fixed and undermines her career self-efficacy.

C. Help her weigh structural barriers to advancement, not just self-blame ✓ **KEY**

The pattern fits a glass ceiling; exploring systemic barriers counters self-blame and opens options for action.

D. Advise her to file a discrimination lawsuit against the firm at once

Legal action is her decision to make after exploring options, not the counselor's directive.

Rationale: The glass ceiling is an invisible barrier that limits women's advancement to top leadership despite qualifications. Women may internalize such barriers as personal failings. Counselors help clients recognize contextual and systemic barriers, preserve career self-efficacy, and consider strategies such as mentoring, sponsorship, advocacy or a move to another organization.

References: Sharf (Career Development Theory) — Career development of women: barriers to advancement · MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions: addressing systemic barriers

☐ Approve ☐ Revise ☐ Retire Notes:

A college sophomore has clear interests in health and science but has not declared a major. She says, "I'm bad at making big choices; I don't trust myself to pick right." A counselor using social cognitive career theory wants to build her career decision self-efficacy. Which strategy would be MOST powerful?

A. Hand her more printed details on each major so she can study them

She already knows her interests; more information does not build confidence in deciding.

B. Tell her firmly that she has what it takes to make a strong decision

Verbal persuasion is a weaker source of self-efficacy than direct success experiences.

C. Have her watch peers describe how they each chose their own majors

Vicarious learning helps, but it is less powerful than her own successes.

D. Set graded decision tasks she can complete and succeed at herself ✓ KEY

Mastery experiences, personal success at the task, are the strongest source of self-efficacy in Bandura's and SCCT's view.

Rationale: Social cognitive career theory (Lent, Brown and Hackett) draws on Bandura's four sources of self-efficacy: performance accomplishments (mastery), vicarious learning, verbal persuasion, and physiological and emotional states. Mastery experiences are the most powerful. For indecision rooted in low decision-making confidence, counselors structure small, achievable decision steps.

References: Sharf (Career Development Theory) — Social cognitive career theory: sources of self-efficacy; career decision self-efficacy

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor trained in divorce mediation is helping a couple reach a parenting and property agreement. In the third session, the wife privately asks the counselor to also see her for individual therapy for depression after the divorce is final. What should the counselor do?

A. Stay in the neutral mediator role and refer her to another therapist ✓ KEY

Taking one party as a therapy client would compromise mediator neutrality; a referral meets her need without a role conflict.

B. Agree, since therapy would begin only after the mediation has ended

Even later, the role change could appear to favor her and undermine the husband's trust in the agreement.

C. Begin individual sessions now so her depression does not slow progress

Concurrent therapy with one party creates a clear conflict of interest in mediation.

D. Ask the husband's permission and then see her alone if he agrees to it

Consent does not remove the conflict created when the mediator becomes one party's therapist.

Rationale: Divorce mediation is a neutral, task-focused process for reaching agreements; it is not therapy. When one party seeks a therapeutic relationship, the counselor clarifies roles and refers to avoid a conflict of interest that would compromise impartiality. The ACA Code addresses role changes and serving multiple clients whose interests may conflict.

References: ACA Code of Ethics — A.6.d Role Changes in the Professional Relationship; A.8 Multiple Clients

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 13-year-old are alarmed that she now argues about clothes, chores and bedtime and rolls her eyes at their rules. She has good grades, close friends, and still talks with them about school. What should the counselor tell the parents?

A. The bickering may signal early oppositional defiant disorder

ODD involves a persistent, pervasive pattern of hostility; mild, everyday conflict with good functioning is not ODD.

B. Mild conflict over daily issues is common in early adolescence ✓ **KEY**

Parent-teen conflict over everyday matters rises in early adolescence and usually reflects normal striving for autonomy.

C. Firm limits with no discussion will end the conflict quickest

An authoritarian style tends to escalate conflict; warmth with reasoned limits works better.

D. Letting her set her own rules will reduce tension at home

A permissive approach removes needed structure; teens still benefit from monitoring and clear limits.

Rationale: Parent-adolescent conflict over everyday issues typically increases in early adolescence as teens seek autonomy, and it usually declines later. Severe, constant conflict is not normal and warrants attention. Authoritative parenting, which pairs warmth with clear, explained limits and gradually expanded autonomy, is linked to the best adolescent outcomes.

References: Berk (Development Through the Lifespan) — Adolescence: parent-child relationships, autonomy and conflict

☐ Approve ☐ Revise ☐ Retire Notes:

A family lost their home and business in a wildfire. In counseling, they argue about what went wrong but also describe neighbors who helped and a shared faith that "we will rebuild." A counselor using Froma Walsh's family resilience framework would MOST likely focus on which task?

A. Finding the family member whose symptoms carry the family stress

Locating an identified patient fits other systems models, not Walsh's strengths-based resilience focus.

B. Tracing the family's patterns of fusion back over three generations

Multigenerational tracing is a Bowenian task, not the core of the resilience framework.

C. Strengthening shared meaning, connection and joint problem-solving ✓ **KEY**

Walsh's key processes are belief systems, organizational patterns and communication/problem-solving, all built on family strengths.

D. Rearranging the family hierarchy so the parents hold firmer power

Restructuring hierarchy is a structural family therapy goal, not the main resilience focus.

Rationale: Walsh's family resilience framework views families facing adversity as challenged, not damaged. It strengthens three domains of key processes: belief systems (making meaning of adversity, a positive outlook, transcendence and spirituality), organizational patterns (flexibility, connectedness, social and economic resources) and communication/problem-solving (clarity, open emotional sharing, collaborative problem-solving).

References: Goldenberg (Family Therapy) — Family resilience (Walsh): key processes in family functioning

► **Check:** Walsh's framework is primary in Walsh (2016), *Strengthening Family Resilience*; SME to confirm the Goldenberg locator.

☐ Approve ☐ Revise ☐ Retire Notes:

A 35-year-old client mentions that during worship at her Pentecostal church she speaks in tongues and sometimes feels the Holy Spirit move through her. She works, has close relationships, and shows no other unusual experiences. How should the counselor BEST understand these reports?

A. As possible psychotic symptoms that call for a psychiatric referral

Experiences accepted within the client's faith community, with intact functioning, are not evidence of psychosis.

B. As accepted religious practices within her own faith community ✓ **KEY**

DSM-5-TR cautions that experiences sanctioned by a person's culture or religion should not be judged as symptoms.

C. As dissociative episodes that need a structured diagnostic interview

DSM-5-TR excludes trance states that are a normal part of broadly accepted religious practice.

D. As avoidance of her problems that should become a focus of treatment

Nothing suggests her faith practice is causing problems; treating it as avoidance imposes the counselor's values.

Rationale: Clinicians must judge unusual experiences in the context of the client's cultural and religious norms. Practices such as glossolalia are normative in some traditions and are not symptoms when they occur within accepted religious contexts and do not cause distress or impairment. DSM-5-TR notes that dissociative disorder diagnoses do not apply to states that are a normal part of broadly accepted cultural or religious practice.

References: DSM-5-TR — Cultural concepts and distinguishing culturally sanctioned experiences from psychopathology; dissociative disorders exclusion for religious practice · ACA Code of Ethics — E.5.b Cultural Sensitivity

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old is referred eight months after a moderate traumatic brain injury from a car crash. He forgets what was discussed, tires quickly, and becomes irritable when sessions run long. Which adjustment to counseling is MOST appropriate?

A. Long open-ended sessions so he can explore at his own pace

Long, unstructured sessions tax attention and fatigue after brain injury.

B. Insight-focused talk therapy without memory aids or notes

Without memory supports, he is unlikely to retain or use session content.

C. Fewer sessions spaced far apart to reduce his frustration

Wide spacing weakens retention; short, frequent, structured sessions fit better.

D. Short, structured sessions with written summaries and aids ✓ **KEY**

Shorter sessions, concrete structure, repetition and written notes compensate for memory and fatigue problems after TBI.

Rationale: Clients with traumatic brain injury often have deficits in memory, attention, processing speed and emotional regulation, along with fatigue. Counselors adapt by using shorter, structured sessions, repetition, written summaries and external aids, involving family when appropriate, and coordinating with rehabilitation and neuropsychology providers.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling clients with disabilities: adapting counseling to cognitive impairment

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old client who was born deaf comes to counseling, with a sign language interpreter, for stress at work. She mentions proudly that she is active in the Deaf community. The counselor plans to begin by helping her grieve the loss of hearing. Why is this plan MOST problematic?

A. It imposes a loss frame on an identity she holds as cultural ✓ KEY

Many culturally Deaf people view deafness as a linguistic and cultural identity, not a loss to grieve.

B. Grief work is not suitable when an interpreter is in the room

Interpreters do not rule out any type of counseling work; the problem is the counselor's assumption.

C. Stage models say grief over hearing ends by early adulthood

No such rule exists; the issue is that she does not experience her deafness as a loss.

D. Counselors should refer deaf clients to a deaf counselor only

Culturally competent hearing counselors can serve Deaf clients; automatic referral is not required.

Rationale: Disability identity varies. Many people who are culturally Deaf (capital D) see themselves as members of a linguistic and cultural minority with its own language, such as ASL, rather than as people with a deficit. Applying a medical or loss model without asking the client imposes the counselor's worldview. Counselors explore how clients define their own identity and focus on the presenting concern.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling individuals with disabilities: disability identity and Deaf culture

☐ Approve ☐ Revise ☐ Retire Notes:

A 72-year-old transgender woman who transitioned in her forties must move into assisted living after a stroke. She says, "I fought my whole life to be myself. I'm terrified the staff will misgender me or that I'll have to hide again." Which counselor response is BEST?

A. Reassure her that staff are trained, so her worries are probably unfounded

Her fears reflect real risks documented for LGBTQ+ older adults; dismissing them invalidates her.

B. Suggest she keep her history private until she knows the staff better

Advising concealment adds to minority stress and puts the burden of others' bias on her.

C. Explore her fears and help her find and advocate for affirming facilities ✓ KEY

Validating her fears and helping her identify affirming care and supports addresses both her distress and real barriers.

D. Focus on her stroke recovery, since gender identity is a settled matter

Her identity is settled, but her safety and dignity in care are urgent present concerns.

Rationale: Transgender older adults often carry histories of discrimination and may lack family support. Many fear mistreatment, misgendering or pressure to go back into the closet in long-term care. Counselors validate these fears, help clients locate LGBTQ+-affirming providers and aging resources, and advocate for respectful, competent care.

References: MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions at the institutional level · Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ+ individuals; counseling older adults

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old bisexual man in a relationship with a woman says his family calls his bisexuality "a phase," while gay friends tell him he is "really straight now." He feels he belongs nowhere. Which concept BEST describes this source of minority stress?

A. Internalized homonegativity that he holds about his own identity

Internalized stigma is a self-directed belief; here the invalidation comes from others.

B. Bisexual erasure from both straight and gay communities ✓ KEY

Bisexual people often face invalidation from both heterosexual and gay or lesbian communities, sometimes called double discrimination.

C. Identity confusion, the first stage of Cass's identity model

Cass describes stages of gay and lesbian identity; it does not capture rejection from two communities.

D. Heterosexual privilege that protects him from any minority stress

Being in a different-gender relationship does not remove the stigma and invisibility bisexual people face.

Rationale: Bisexual people experience unique minority stressors, including bisexual erasure (denial that bisexuality is real or lasting) and binegativity from both heterosexual and gay or lesbian communities. This double marginalization can limit community support and is linked to higher rates of depression and anxiety than among gay and lesbian peers.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ+ individuals: bisexual invisibility and double discrimination

► **Check:** Confirm the Sue & Sue chapter discusses bisexual erasure/binegativity and double discrimination.

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old woman who entered the country without documentation seeks counseling for panic, poor sleep and constant fear that she will be separated from her children. At intake she hesitates and asks, "Will you tell anyone about my status?" What should the counselor do FIRST?

A. Explain confidentiality and its limits, which do not include her status ✓ KEY

Immigration status is not a mandated reporting category; explaining confidentiality and its true limits builds the safety she needs.

B. Record her status in detail so that the clinical file is fully complete

Counselors record what is clinically needed; detailed status notes add risk without clinical benefit.

C. Refer her to an immigration attorney before beginning counseling work

Legal referral may help later, but her immediate question is about confidentiality and her panic needs care.

D. Tell her you cannot discuss her legal status because of agency policy

Deflecting her question deepens fear and mistrust; she needs a clear answer about confidentiality.

Rationale: Undocumented clients live with chronic fear of detention and family separation, which can cause anxiety, depression and trauma symptoms. Fear of disclosure is a major barrier to care. Counselors clearly explain confidentiality and its actual limits (such as risk of harm or child abuse), note that immigration status is not a reportable matter, and document only what is clinically necessary.

References: ACA Code of Ethics — B.1.d Explanation of Limitations; A.2.b Types of Information Needed · Sue & Sue (Counseling the Culturally Diverse) — Counseling immigrants and refugees: undocumented status and fear of disclosure

► **Check:** No general mandate requires counselors to report immigration status, but some funded programs or states may have specific rules; SME to confirm the general framing.

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old woman who immigrated from rural China two years ago is brought in by her daughter. She reports headaches, fatigue and poor appetite, says her doctor found nothing wrong, and insists, "I am not crazy. I do not need to talk about feelings." Which approach is BEST?

A. Explain that her physical symptoms are really depression in disguise

Relabeling her experience early challenges her worldview and is likely to end engagement.

B. Ask her daughter to describe her mother's emotions on her behalf

This sidelines the client and may breach her preferences about what is shared within the family.

C. Tell her counseling can begin only once she accepts a diagnosis

Requiring acceptance of a diagnosis is a barrier and does not fit her cultural view of help-seeking.

D. Respect her view of the symptoms and offer practical, active help ✓ KEY

Starting with her own framing and offering concrete, problem-focused help builds credibility with less acculturated clients.

Rationale: Less acculturated clients from many cultures underuse counseling, may express distress through somatic complaints, and may see emotional talk as stigmatizing. Sue and Sue describe culturally responsive engagement: accept the client's explanation of the problem, coordinate with medical care, and offer active, structured, practical help. An early, felt benefit (what Sue and Sue call gift giving) builds the counselor's credibility.

References: Sue & Sue (Counseling the Culturally Diverse) — Asian Americans: somatic expression of distress; credibility and gift giving

☐ Approve ☐ Revise ☐ Retire Notes: _____

103. nce-s-clf-232 · Counseling and Helping Relationships · psychopharmacology basics for counselors · easy · In review

A client with schizophrenia has gained 25 pounds in the six months since starting a second-generation antipsychotic. He says he feels constantly hungry and is thinking of stopping the medication. What should the counselor do?

A. Support his plan to stop, since the weight gain is a serious problem

Stopping an antipsychotic abruptly risks relapse; medication changes belong to the prescriber.

B. Reassure him that the weight gain is cosmetic and will soon level off

Weight gain on these drugs signals metabolic risk and is not merely cosmetic.

C. Urge him to talk to his prescriber about the gain and metabolic checks ✓ KEY

Second-generation antipsychotics carry metabolic risks, and prescribers monitor weight, glucose and lipids and can adjust treatment.

D. Recommend a specific medication change that has fewer side effects

Recommending specific drugs is outside the counselor's scope of practice.

Rationale: Several second-generation antipsychotics, notably olanzapine and clozapine, are associated with weight gain, insulin resistance, diabetes and lipid changes. Guidelines call for monitoring weight, waist size, blood pressure, glucose and lipids. Counselors recognize these concerns, encourage clients to raise them with the prescriber rather than stop medication, and support healthy lifestyle changes.

References: APA CPG — Treatment of patients with schizophrenia (2020): monitoring for metabolic side effects of antipsychotics

☐ Approve ☐ Revise ☐ Retire Notes: _____

A father tells the counselor, "We gave our son a dose of his cousin's ADHD stimulant before a test, and he focused much better. So he must have ADHD, right?" Which response reflects an accurate understanding of stimulant medication?

A. Stimulants can sharpen focus in most people, so a response is not a diagnosis ✓ KEY

Stimulants improve attention in people with and without ADHD, so a good response cannot confirm the diagnosis.

B. A calming effect from a stimulant is a reliable sign that ADHD is present

The idea of a "paradoxical" response unique to ADHD is a myth; stimulants act similarly in those without ADHD.

C. Because stimulants are addictive, a good response is a sign of misuse risk

One positive response does not indicate misuse risk; the issue is that response is not diagnostic.

D. A good response confirms ADHD only if the effect lasts more than a week

Duration of response does not make it diagnostic; ADHD is diagnosed by DSM-5-TR criteria and history.

Rationale: A common misconception holds that stimulants calm or focus only people with ADHD. In fact, stimulants improve attention and concentration in most people, so response to medication cannot establish a diagnosis. ADHD is diagnosed through a careful history and DSM-5-TR criteria across settings. The counselor should also address the safety concern of giving a child someone else's prescription.

References: DSM-5-TR — Attention-deficit/hyperactivity disorder: diagnostic criteria and differential diagnosis · NBCC Study Supplement — Psychopharmacology: stimulant medications

► **Check:** The fact (non-specific stimulant response; Rapoport et al., 1978) is well established, but neither cited source states it directly; SME to confirm or substitute a pharmacology reference.

☐ Approve ☐ Revise ☐ Retire Notes:

A veteran completed prolonged exposure for PTSD. His fear when recalling a roadside ambush has dropped sharply, yet he still feels intense shame that he froze and a friend died, says he "doesn't deserve" good things, and avoids his family. What should the counselor focus on NEXT?

A. Repeat a full course of exposure, since fear must still be driving him

His fear has already habituated; repeating exposure does not target the guilt and shame that remain.

B. Address guilt and shame through meaning-making and self-forgiveness ✓ KEY

Moral injury centers on guilt, shame and broken meaning, which call for work on appraisals, meaning and self-forgiveness.

C. Reassure him that freezing was not his fault and move toward ending

Quick reassurance rarely shifts deep guilt and leaves the moral-injury symptoms untreated.

D. Refer him for a medication to reduce his remaining startle response

His remaining problems are guilt, shame and withdrawal, not hyperarousal.

Rationale: Moral injury describes lasting guilt, shame and loss of meaning after acts that violate one's moral code, including perceived failures to act. Fear-based exposure can reduce fear without resolving these reactions. Treatment adds work on guilt appraisals (as in cognitive processing therapy), meaning-making, self-forgiveness and repair (as in approaches such as adaptive disclosure), often with spiritual or community resources.

References: VA/DoD CPG — Management of PTSD and acute stress disorder: trauma-focused psychotherapies (PE, CPT)

► **Check:** Moral injury is not a formal diagnosis and the VA/DoD CPG does not specifically address it; SME to confirm the citation or add a moral-injury source (e.g., Litz et al., Adaptive Disclosure).

☐ Approve ☐ Revise ☐ Retire Notes:

A 42-year-old retired Navy chief landed a well-paid civilian job after 20 years of service. Six months in, he says, "The work is fine, but I feel pointless. Nobody has each other's back here, and nothing feels urgent." What is the counselor's BEST initial focus?

A. Help him find a higher-paying job with greater responsibility

Pay and rank are not his concern; he describes loss of purpose and belonging.

B. Screen him for PTSD, since his distress began after service

Nothing he describes suggests trauma symptoms; his concern is transition-related.

C. Teach him to adopt a civilian mindset and leave the Navy behind

Dismissing his military identity ignores a valued part of who he is.

D. Explore his loss of military identity, mission and camaraderie ✓ KEY

Leaving the military often means losing identity, purpose and close bonds, which are central transition tasks.

Rationale: Military-to-civilian transition involves more than finding a job. Veterans often lose a strong shared identity, a clear mission, structure and unit cohesion. Counselors explore these losses, help the veteran carry valued military strengths into civilian life, and support sources of purpose and belonging such as veteran organizations and service roles.

References: Sharf (Career Development Theory) — Adult career transitions; Schlossberg's model applied to job and role change

► **Check:** Veteran transition content may not appear in Sharf specifically; SME to confirm the locator.

☐ Approve ☐ Revise ☐ Retire Notes: _____

An 8-year-old from a low-income family has had a painful, untreated dental abscess for months and cannot see the board without glasses. The school nurse has arranged free dental care and free glasses three times, and the parent has not followed through. Which detail MOST supports a concern of neglect rather than poverty alone?

A. The family's low income and lack of dental insurance coverage

Lack of resources alone is not neglect; poverty must be separated from failure to provide care.

B. The child's trouble seeing the board in her daily classroom work

This shows an unmet need but does not show why the need is unmet.

C. Repeated failure to use free care that was arranged for the child ✓ KEY

When accessible care is offered and still not used for a serious need, the failure points to neglect rather than lack of means.

D. The length of time the abscess has gone without any treatment

Duration shows severity, but on its own could still reflect lack of access.

Rationale: Neglect is a failure to meet a child's basic needs, including medical and dental care, when the caregiver is able or has been offered reasonable means to do so. Many state definitions exclude conditions caused solely by poverty. Indicators include untreated medical or dental problems, chronic hunger, poor hygiene and frequent absence. Repeated refusal of accessible care for a serious need supports reasonable suspicion and a report.

References: State law — Child abuse and neglect statutes: definitions of neglect and poverty exclusions · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes: _____

Treatment Planning

17 items · NCE weight 9% (14 of 160 scored items)

108. nce-s-trt-052 · Counseling and Helping Relationships · SMART goals and measurable objectives · easy · In review

A counselor-in-training writes "Client will have healthier relationships" as the only entry under objectives in a treatment plan. Her supervisor explains how goals and objectives differ. Which statement BEST describes that difference?

A. A goal states a broad aim; objectives are measurable steps toward it ✓ **KEY**

Goals describe the desired end state in broad terms; objectives break the goal into specific, observable, time-limited steps that show progress.

B. A goal is set by the counselor; objectives are chosen by the client

Both goals and objectives are developed collaboratively; authorship is not what separates them.

C. A goal lists the interventions; objectives list the diagnoses treated

Interventions and diagnoses are separate parts of the plan; neither defines the goal or the objective.

D. A goal is reached in one session; objectives take the full treatment

This reverses the relationship: objectives are the shorter-term steps, and the goal is the longer-term outcome.

Rationale: In treatment planning, a goal is a broad, long-term statement of what the client hopes to achieve, such as healthier relationships. Objectives translate that goal into specific, measurable, time-bound steps (for example, using an "I" statement in two disagreements per week for four weeks, recorded in a log). The supervisee's entry is a goal, so it still needs measurable objectives beneath it.

References: Wiger (Documentation) — Treatment plans: distinguishing long-term goals from measurable short-term objectives

☐ Approve

☐ Revise

☐ Retire

Notes: _____

A 27-year-old client with severe social anxiety rarely speaks in groups larger than three people. At intake, her counselor writes the objective: "Client will give a 10-minute presentation to her 200-person department within two weeks." Which SMART criterion does this objective MOST clearly fail?

A. Specific, because it does not say which behavior will change

The behavior (a 10-minute presentation to a named audience) is clearly specified.

B. Measurable, because a presentation cannot be counted or observed

Giving a presentation is directly observable and easy to verify, so the objective is measurable.

C. Achievable, because the step is too large for her starting point ✓ KEY

Jumping from avoiding groups of three to presenting to 200 people in two weeks is not realistic; graded steps are needed.

D. Time-bound, because it does not include any deadline for change

The objective includes a clear deadline of two weeks, so it is time-bound.

Rationale: SMART objectives are Specific, Measurable, Achievable (attainable), Relevant and Time-bound. An objective can be precise and measurable yet still fail if it is not realistic given the client's current functioning. Setting an unattainable first step invites failure and discouragement; a graded series of objectives, such as speaking once in a small meeting, then a larger one, fits the client's starting point and the logic of gradual exposure.

References: Wiger (Documentation) — Treatment plans: objectives that are realistic and attainable for the client's current level of functioning

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old man seeks counseling for stress after losing his job. During the intake, he mentions drinking six to eight beers most nights but says, "That is not why I am here." The counselor believes the drinking is a major concern. How should the counselor approach the treatment plan?

A. List alcohol abstinence as the primary goal since it is most urgent

Imposing a goal the client has rejected undermines the alliance and is likely to increase resistance and dropout.

B. Decline to treat the job stress until he agrees to address drinking

Making care conditional on accepting the counselor's agenda is coercive and ignores the client's stated needs.

C. Leave drinking out of the plan and the sessions to keep the alliance

Ignoring a clinically significant concern avoids conflict but fails the client; it should be explored respectfully.

D. Start with his job stress goal and openly explore links to drinking ✓ KEY

Beginning with the client's priority builds engagement while the counselor transparently shares the concern and explores how drinking affects his stress.

Rationale: Collaborative treatment planning starts from the goals the client brings while allowing the counselor to share clinical concerns honestly. Here the counselor can begin with job-loss stress, voice the concern about drinking without imposing it, and use a motivational approach to explore how alcohol affects sleep, mood and coping. Plans built jointly and consistent with the client's circumstances are more likely to succeed.

References: ACA Code of Ethics — A.1.c Counseling Plans: counselors and clients work jointly to devise counseling plans · Miller & Rollnick (MI) — Focusing process: negotiating a shared agenda when client and counselor priorities differ

☐ Approve ☐ Revise ☐ Retire Notes:

A 63-year-old Vietnamese immigrant client nods and agrees to every goal the counselor proposes during treatment planning, then misses the next two sessions. She has said she was raised to defer to doctors and teachers. Which response is MOST likely to produce a truly collaborative plan?

A. Ask her to sign the plan as written since she already agreed to it

Her agreement may reflect deference rather than true endorsement; a signature does not make the plan collaborative.

B. Invite her priorities indirectly, such as what she hopes will change ✓ **KEY**

Open, respectful questions about her own hopes let her shape the plan without having to openly contradict an authority figure.

C. Refer her to a counselor who is also from a Vietnamese background

Shared ethnicity does not guarantee a good match, and referral skips the counselor's own task of adapting the process.

D. Let her family set the treatment goals because she defers to others

Family input may be welcome if she wants it, but handing goal-setting to others removes her voice entirely.

Rationale: Agreement is not the same as collaboration. When a client's cultural norms discourage disagreeing with authority, quick assent to every proposal and later missed sessions suggest the plan does not reflect her priorities. Culturally responsive planning reduces the power gap by asking about her hopes, concerns and preferred pace in indirect, respectful ways, checking understanding, and revising the plan with her.

References: Sue & Sue (Counseling the Culturally Diverse) — Culturally appropriate intervention: deference to authority and indirect communication styles · ACA Code of Ethics — A.1.c Counseling Plans: plans consistent with the abilities, temperament and circumstances of clients

☐ Approve

☐ Revise

☐ Retire

Notes:

A counselor works as a behavioral health care manager in a primary care clinic that uses the collaborative care model. She tracks every patient's PHQ-9 scores in a registry and reviews the caseload weekly with a consulting psychiatrist. What is the main purpose of the registry review?

A. To spot patients who are not improving so treatment can be adjusted ✓ **KEY**

Systematic caseload review flags patients not responding so the team can change treatment, the treat-to-target core of collaborative care.

B. To transfer every depressed patient to the psychiatrist's own care

In collaborative care the psychiatrist consults on the caseload; patients stay with their primary care provider and care manager.

C. To produce billing records that replace the clinical progress notes

A registry supports clinical decision-making; it does not replace progress notes or exist mainly for billing.

D. To find patients who no longer need any primary care involvement

The model keeps behavioral health care inside primary care; it is not designed to remove patients from it.

Rationale: Integrated behavioral health in primary care often follows the collaborative care model: a primary care provider, a behavioral health care manager and a consulting psychiatrist share a defined caseload. Measurement-based tracking in a registry and regular systematic caseload review identify patients who are not reaching targets, prompting changes in medication or psychotherapy. This treat-to-target approach is a key reason the model improves depression outcomes.

References: Gladding (Counseling: A Comprehensive Profession) — Clinical mental health counseling settings: integrated behavioral health in primary care · Lambert (Outcome Monitoring) — Measuring and monitoring progress to identify clients not on track

► **Check:** Collaborative care (registry, care manager, psychiatric consultant, treat-to-target) is well established in the AIMS/Unutzer literature; confirm Gladding edition discusses integrated care in this detail or swap to a more specific source if added to the library.

☐ Approve ☐ Revise ☐ Retire Notes:

During a routine visit, a family physician learns that a 35-year-old patient has been having panic attacks. She walks the patient down the hall, introduces him by name to the clinic's counselor, and explains why she thinks they should meet that same day. What is this practice called?

A. A cold referral

A cold referral gives the patient a name or number to call later, with no direct introduction.

B. A case conference

A case conference is a meeting among professionals about a client, not a live introduction of the client.

C. A warm handoff ✓ KEY

A warm handoff is a direct, in-person introduction from one provider to another, ideally during the same visit.

D. A consultation

Consultation is professional-to-professional advice; the patient is not transferred to the consultant's care.

Rationale: A warm handoff occurs when one provider personally introduces a patient to another, explaining the reason for the connection, often in the same visit. Compared with handing the patient a phone number, it transfers the trust built with the first provider, reduces stigma and logistical barriers, and increases the chance that the patient actually begins behavioral health care.

References: Gladding (Counseling: A Comprehensive Profession) — Clinical mental health counseling settings: referral and integrated care with medical providers

► **Check:** Confirm the Gladding edition names the warm handoff explicitly; the concept is standard in integrated-care literature (e.g., AHRQ).

☐ Approve ☐ Revise ☐ Retire Notes:

A 39-year-old client has met her treatment goals for generalized anxiety and will be discharged from an outpatient agency. Her counselor is preparing the discharge summary. Which entry is MOST essential to include?

A. A full transcript of the client's comments during the final session

Verbatim session content is not required and adds detail that is not needed for continuity of care.

B. Progress on each treatment goal and the aftercare recommendations ✓ KEY

A discharge summary documents why treatment ended, progress on each goal, status at discharge and the aftercare plan.

C. The counselor's personal reactions to the client across treatment

Counselor reactions belong in supervision, not in the client record that may follow her to future providers.

D. A copy of every weekly anxiety score the client has ever obtained

Scores are already in the record; the summary reports overall change rather than reproducing every data point.

Rationale: A discharge (closing) summary gives future providers and reviewers a concise picture of the episode of care: reason for referral and for discharge, diagnosis, services provided, progress toward each treatment-plan goal, condition at discharge, and specific aftercare recommendations and referrals. Linking discharge to the plan's goals shows that treatment ended for a clinical reason and supports continuity of care.

References: Wiger (Documentation) — Discharge summary: reason for discharge, progress toward goals, and aftercare recommendations

☐ Approve ☐ Revise ☐ Retire Notes:

A 46-year-old client with recurring suicidal thoughts keeps a handgun in his bedside drawer. He has no current plan or intent and is working on a safety plan with his counselor. He says the gun is for protection and he does not want to sell it. Which approach is MOST consistent with best practice?

A. Ask him to sign a contract promising not to harm himself with the gun

No-suicide contracts lack evidence of preventing suicide and do nothing to reduce access to lethal means.

B. Accept his choice and leave lethal means out of the safety plan to respect autonomy

Respecting autonomy does not mean skipping lethal means counseling; access to a firearm is a major risk factor.

C. Tell him counseling cannot continue until the firearm leaves the home

An ultimatum damages the alliance and may push him out of care, which raises rather than lowers risk.

D. Explore temporary off-site storage with someone he trusts during a crisis ✓ KEY

Collaborative lethal means counseling seeks temporary, acceptable ways to put time and distance between the client and the firearm.

Rationale: Making the environment safe is a core step of the Stanley-Brown Safety Planning Intervention. Lethal means counseling works collaboratively, respecting the client's values while finding acceptable options, such as temporary storage with a trusted friend, a locked safe with the key held by someone else, or ammunition stored separately. Because suicidal crises are often brief, added time and distance from a firearm can be lifesaving.

References: Stanley-Brown SPI — Step 6: Making the environment safe (lethal means counseling)

☐ Approve

☐ Revise

☐ Retire

Notes:

An agency compares two depression programs by dividing each program's total cost by the number of clients who achieved remission, finding \$2,400 per remission for one and \$3,100 for the other. Outcomes are not converted to dollar values. Which type of analysis is this?

A. Cost-benefit analysis

Cost-benefit analysis expresses outcomes in dollars (such as savings from fewer hospital days) and compares them with costs.

B. Summative needs assessment

A needs assessment identifies unmet needs before or during planning; it does not compare program costs per outcome.

C. Cost-effectiveness analysis ✓ KEY

Cost-effectiveness analysis compares programs on cost per unit of a nonmonetary outcome, such as cost per remission.

D. Formative process evaluation

Formative or process evaluation examines how a program is running so it can be improved, not its cost per outcome.

Rationale: Both analyses weigh costs against results, but they express results differently. Cost-benefit analysis converts outcomes into monetary terms so that benefits and costs can be compared directly, often as a benefit-cost ratio. Cost-effectiveness analysis keeps outcomes in natural units, such as remissions or symptom-free days, and reports the cost per unit of outcome, which suits counseling programs whose results are hard to price.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: cost-benefit versus cost-effectiveness analysis

☐ Approve

☐ Revise

☐ Retire

Notes:

A school district adopts an evidence-based bullying prevention curriculum, but after one year bullying reports have not declined. Teachers report that they often skipped lessons to make time for test preparation. What should the program evaluator examine FIRST?

A. Whether the curriculum's original research trials were published

The curriculum is already evidence-based; reviewing its publication history does not explain the local result.

B. Whether the curriculum was delivered with fidelity as designed ✓ KEY

Skipped lessons suggest poor implementation fidelity; a program not delivered as designed cannot be judged ineffective.

C. Whether a different curriculum would cost the district less

Cost comparisons may come later, but they do not address why this program did not produce the expected outcome.

D. Whether students liked the curriculum more than last year's

Satisfaction is a weak proxy for outcome and does not address the reported gaps in delivery.

Rationale: Fidelity (treatment integrity) monitoring checks whether a program is delivered as intended in dose, content and quality. When outcomes disappoint, evaluators first ask whether the program was actually implemented; otherwise a failure of delivery is mistaken for a failure of the program. Involving stakeholders such as teachers, students and administrators in the evaluation helps uncover implementation barriers like competing demands on class time.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: implementation (process) evaluation and treatment fidelity; stakeholder involvement

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher randomly assigns college students to receive either four or eight sessions of mindfulness training and then compares their scores on a perceived stress scale. In this study, what is the dependent variable?

A. Scores on the perceived stress scale ✓ KEY

The dependent variable is the outcome measured to see whether it changes in response to the manipulated variable.

B. The number of mindfulness sessions

The number of sessions is manipulated by the researcher, which makes it the independent variable.

C. The random assignment of the students

Random assignment is a design feature that controls for group differences; it is not a variable being measured.

D. The college the students are enrolled in

The college is a constant or possible extraneous variable, not the outcome under study.

Rationale: The independent variable is what the researcher manipulates or uses to form groups (here, four versus eight sessions). The dependent variable is the outcome measured to see whether it depends on the independent variable (here, perceived stress scores). A quick test: the dependent variable is the effect, measured after the cause is varied.

References: Erford (Research and Evaluation in Counseling) — Research design: independent and dependent variables

☐ Approve ☐ Revise ☐ Retire Notes:

A college counseling center compares students who signed up for a new resilience workshop with students who chose not to attend. At pretest, workshop students already reported more social support and higher motivation. Workshop students show greater gains at posttest. Which threat to internal validity is MOST important here?

A. Maturation of both groups over the full semester

Maturation would affect both groups similarly over the same period, so it does not explain the difference between groups.

B. Testing effects from taking the same pretest

Both groups took the pretest, so practice effects would not explain why one group gained more.

C. Instrumentation changes in the outcome measure

Nothing suggests the measure or its scoring changed between pretest and posttest.

D. Selection bias from groups that chose themselves ✓ KEY

Students chose their own group and differed at pretest, so preexisting differences may explain the gains.

Rationale: Selection is the main threat in quasi-experimental designs that compare nonequivalent groups without random assignment. When participants choose whether to receive the intervention, the groups can differ in motivation, support or other traits that also affect the outcome. Pretest differences are a warning sign; random assignment, matching or statistical controls such as ANCOVA reduce, but in quasi-experiments do not eliminate, this threat.

References: Erford (Research and Evaluation in Counseling) — Quasi-experimental designs: selection as a threat to internal validity

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher first analyzes outcome scores from 200 clients in a grief support program. She then interviews 15 clients whose scores did not improve to understand why the program did not help them. Which research design is she using?

A. An exploratory sequential mixed methods design

An exploratory sequential design starts with qualitative data and uses it to build or guide a later quantitative phase.

B. An explanatory sequential mixed methods design ✓ KEY

An explanatory sequential design collects quantitative data first, then qualitative data to explain the quantitative results.

C. A convergent parallel mixed methods study design

A convergent design collects both types of data at about the same time and compares them, rather than in sequence.

D. A practitioner-led action research design

Action research is cyclical inquiry by practitioners to improve their own practice; the defining feature here is the quant-then-qual sequence.

Rationale: Mixed methods research combines quantitative and qualitative data in one study. In an explanatory sequential design, the quantitative phase comes first and the qualitative phase follows to explain unexpected or important quantitative findings, such as why some clients did not improve. In an exploratory sequential design the order is reversed, and in a convergent design both strands are collected together and merged.

References: Erford (Research and Evaluation in Counseling) — Mixed methods designs: explanatory sequential, exploratory sequential and convergent

► **Check:** Design names follow Creswell & Plano Clark; confirm Erford uses the same labels (older texts say "sequential explanatory" and "concurrent triangulation").

☐ Approve ☐ Revise ☐ Retire Notes:

A research team plans a randomized trial of a new intervention for adults with severe depression, some of whom report suicidal ideation. One team member proposes a no-treatment control group to get the cleanest comparison. Which comparison condition is MOST ethically defensible?

A. Treatment as usual, so control clients still receive standard care ✓ KEY

An active treatment-as-usual comparison answers the research question without withholding needed care from high-risk clients.

B. A no-treatment group, provided participants give informed consent

Consent does not make it acceptable to withhold effective care from severely depressed, at-risk participants.

C. A placebo pill group, so neither group knows which they received

An inert placebo still withholds active treatment and is a poor match for a psychotherapy intervention.

D. A six-month waitlist group with no contact until the waiting ends

A long waitlist without monitoring delays care and leaves suicidal risk unassessed, which is not defensible here.

Rationale: Researchers must minimize harm to participants. When an effective standard treatment exists and participants are at serious risk, withholding care for the sake of a cleaner comparison is hard to justify. Comparing the new intervention with treatment as usual (an active control) keeps everyone in care. Waitlist or no-treatment controls are more defensible for low-risk problems, short delays, and with monitoring and a plan to offer treatment afterward.

References: ACA Code of Ethics — G.1.e Precautions to Avoid Injury: researchers take reasonable precautions to avoid causing physical, emotional or social injury to participants · Erford (Research and Evaluation in Counseling) — Ethics in research: control groups and withholding treatment

☐ Approve ☐ Revise ☐ Retire Notes:

A study comparing a new group intervention with a waitlist reports Cohen's $d = 0.50$ on a measure of anxiety. Using Cohen's conventional benchmarks, how large is this effect?

A. Negligible

Effects below about 0.20 are usually described as negligible; 0.50 is well above that.

B. Small

A small effect is about $d = 0.20$.

C. Medium ✓ KEY

Cohen described $d = 0.50$ as a medium effect.

D. Large

A large effect is about $d = 0.80$ or above.

Rationale: Cohen's d expresses the difference between two group means in standard deviation units. Cohen's conventional benchmarks are 0.20 for a small effect, 0.50 for a medium effect and 0.80 for a large effect. Unlike a p value, an effect size describes how much difference an intervention makes and does not depend on sample size.

References: Erford (Research and Evaluation in Counseling) — Effect size: Cohen's d benchmarks of .20, .50 and .80

☐ Approve ☐ Revise ☐ Retire Notes:

A trial reports that clients receiving a sleep intervention improved 5 points more on an insomnia scale than controls, with a 95% confidence interval for the difference of 2 to 8 points. Which interpretation is MOST accurate?

A. Ninety-five percent of treated clients improved between 2 and 8 points

The interval describes uncertainty about the average group difference, not the range of individual client changes.

B. The difference is significant at .05 because the interval excludes 0 ✓ **KEY**

A 95% interval that does not contain zero corresponds to rejecting the null hypothesis of no difference at $\alpha = .05$.

C. The study has a 5% chance of being repeated with the same 5-point result

Confidence level is not a probability of replicating the exact point estimate.

D. The intervention has a large effect because the whole interval is positive

An interval above zero shows a reliable difference, not a large one; effect size needs a standardized measure.

Rationale: A confidence interval gives a range of plausible values for the population parameter, here the true mean difference. If the procedure were repeated many times, 95% of intervals built this way would contain the true value. When a 95% interval for a difference excludes zero, the result is statistically significant at the .05 level. The interval's width also shows precision: narrower intervals reflect more precise estimates.

References: Erford (Research and Evaluation in Counseling) — Inferential statistics: confidence intervals and their relation to hypothesis testing

☐ Approve ☐ Revise ☐ Retire Notes:

A client's score on a depression measure drops from 38 to 26. The reliable change index for this measure is 8 points, and the cutoff separating clinical from nonclinical populations is 20. Using the Jacobson and Truax approach, how should this change be classified?

A. Recovered, since the change is larger than the reliable change index

Recovery requires both reliable change and a final score past the cutoff; 26 is still above 20.

B. Unchanged, since the final score is still in the clinical range

A 12-point drop exceeds the 8-point reliable change index, so the change is larger than measurement error.

C. Deteriorated, since the score has not yet fallen below the cutoff

Deterioration means reliable change in the worse direction; this client's score improved.

D. Improved, since the change is reliable but the cutoff is not crossed ✓ **KEY**

The 12-point drop exceeds the reliable change index, but 26 remains in the clinical range, so the client is improved, not recovered.

Rationale: Jacobson and Truax defined clinically significant change with two criteria: the change must be reliable (larger than expected from measurement error, judged by the reliable change index) and the client must move from the dysfunctional range into the functional range past a cutoff. Clients meeting both are recovered; reliable change without crossing the cutoff is improved; no reliable change is unchanged; reliable worsening is deteriorated. This distinguishes meaningful change for one client from mere statistical significance in a group.

References: Lambert (Outcome Monitoring) — Reliable change index and clinical cutoff (Jacobson & Truax) for classifying client change · Erford (Research and Evaluation in Counseling) — Clinical versus statistical significance

☐ Approve ☐ Revise ☐ Retire Notes:

Counseling Skills and Interventions

60 items · NCE weight 30% (48 of 160 scored items)

125. nce-s-cou-181 · Counseling and Helping Relationships · reality therapy and choice theory · easy · In review

Glasser pictured total behavior as a car, with basic needs as the engine and wants as the steering wheel. Which two components are the front wheels, the parts of behavior a person controls most directly?

A. Feeling and physiology

Feeling and physiology are the back wheels; they follow along and change indirectly when acting and thinking change.

B. Wants and needs

In the metaphor, needs are the engine and wants steer the car; they are not components of total behavior.

C. Acting and thinking ✓ KEY

Acting and thinking are the front wheels: the components over which people have the most direct control.

D. Perceiving and remembering

Perception plays a role in choice theory (the perceived world), but it is not one of the four wheels of total behavior.

Rationale: In choice theory, total behavior has four inseparable components: acting, thinking, feeling and physiology. Glasser argued that we have almost total control over acting and thinking and only indirect control over feeling and physiology. Reality therapists therefore focus on what the client is doing and thinking, expecting feelings and body states to follow.

References: Corey (Theory & Practice) — Reality Therapy: choice theory and total behavior

☐ Approve ☐ Revise ☐ Retire Notes:

126. nce-s-cou-182 · Counseling and Helping Relationships · reality therapy and choice theory · medium · In review

A 17-year-old on probation says nothing matters to him, yet he lights up when he talks about his grandmother and about someday becoming a firefighter. A reality therapist notes that these belong to his quality world. In choice theory, the quality world is BEST described as:

A. A personal album of the people, things and ideas that best satisfy his needs ✓ KEY

Glasser described the quality world as a set of specific pictures of the people, activities, possessions and beliefs that we believe will best satisfy our needs.

B. The stored record of past failures that has built up into a failure identity

Failure identity was an early Glasser concept about how people see themselves, not the store of need-satisfying pictures.

C. An unconscious storehouse of repressed wishes that drives his current choices

Repressed unconscious wishes are a psychoanalytic idea; choice theory emphasizes present, conscious choices.

D. The shared set of perceptions about reality that he holds with his peer group

The quality world is highly individual; it is not a set of perceptions shared with a group.

Rationale: The quality world is the core of choice theory's motivation: everyone carries a personal "album" of pictures of what would satisfy the needs for survival, love and belonging, power, freedom and fun. A reality therapist tries to learn what is in the client's quality world and to become part of it, then helps the client evaluate whether current behavior is getting him what he wants.

References: Corey (Theory & Practice) — Reality Therapy: the quality world

☐ Approve ☐ Revise ☐ Retire Notes:

A client agreed to walk for 20 minutes three mornings this week. She returns saying she "just couldn't find the time." Which response BEST reflects the reality therapy principles of accepting no excuses and using no punishment?

A. "That is disappointing. We agreed on this, so I expect you to make up the missed walks."

Expressing disappointment and imposing a make-up penalty is criticism and punishment, which reality therapists avoid because it damages the relationship.

B. "Tell me more about what made this week so hard. Your schedule may explain a great deal."

Inviting a long account of why the plan failed accepts the excuse and keeps the focus off what she will do next.

C. "Maybe this plan asked too much of you. Let's set exercise aside and work on something else."

Dropping the goal is giving up on the client, which Glasser explicitly cautioned against.

D. "Do you still want this? If so, let's make a smaller plan you are sure you can carry out." ✓ **KEY**

The counselor neither accepts the excuse nor punishes; she checks the want and helps make a simpler, more attainable plan.

Rationale: When a plan is not carried out, reality therapists do not accept excuses, do not criticize or punish, and do not give up. Instead they return to the client's wants, have the client evaluate what she is doing, and make a new plan that is simple, attainable and within her control (the SAMIC qualities of a good plan).

References: Corey (Theory & Practice) — Reality Therapy: planning and commitment; no excuses, no punishment, do not give up

☐ Approve ☐ Revise ☐ Retire Notes:

128. nce-s-cou-184 · Counseling and Helping Relationships · reflection, paraphrasing and summarizing · easy · In review

A 56-year-old client says, "My son moved across the country for work last month. The house is so quiet now, and I still catch myself setting four places at dinner." Which response is the BEST reflection of feeling?

A. "Your son moved away for work last month, and the house is quieter now."

This restates the content of what she said (a paraphrase) but does not name the emotion beneath it.

B. "You are feeling a real sense of loss and loneliness with him gone." ✓ **KEY**

It identifies the underlying emotions (loss, loneliness) that her words and the image of the extra place setting convey.

C. "How often do the two of you talk now that he lives so far away?"

A question gathers information and shifts focus to logistics rather than reflecting her feeling.

D. "Many parents feel this way at first, and it gets easier with time."

Reassurance and normalizing can feel dismissive and do not reflect her specific feeling back to her.

Rationale: A reflection of feeling names the emotion the client is expressing, often one implied rather than stated, so the client feels understood and can explore it further. Paraphrasing captures content, questions gather information, and reassurance can cut exploration short.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: reflection of feeling versus paraphrasing

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor opens the fourth session by saying, "Last week you talked about the tension with your sister, the late nights at work and how little sleep you are getting, and you decided to try leaving the office by six. Where would you like to begin today?" Which skill is the counselor using, and for what purpose?

A. Paraphrasing, to restate a single message in fresher and shorter words

A paraphrase restates one recent client message; this response pulls together material from an entire prior session.

B. Interpretation, to offer a new frame of reference for the client's issues

The counselor adds no new meaning or explanation; she simply organizes what the client already said.

C. Summarizing, to tie earlier material together and give the session focus ✓ KEY

Summaries condense several themes over a longer span and are often used to open a session, provide continuity and set direction.

D. Immediacy, to address what is happening in the relationship right now

Immediacy concerns the here-and-now counselor-client relationship, which this response does not address.

Rationale: Summarizing condenses several client statements, themes or feelings over a longer stretch of time. Counselors use it to begin a session (linking to the last one), to shift topics, to close a session, and to check shared understanding. It differs from paraphrasing in scope, and from interpretation in that it adds no new frame.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: summarization and its uses

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I'm completely over the breakup," as her eyes fill with tears and she twists a ring on her finger. Which counselor response is the BEST example of a skilled confrontation?

A. "You say you're over it, and yet your eyes are filling with tears." ✓ KEY

It describes the discrepancy between her words and her nonverbal behavior, tentatively and without judgment, and leaves her room to explore it.

B. "You clearly are not over it, and denying it will only make things harder."

This tells the client what she feels and labels her as denying; it is judgmental and likely to provoke defensiveness.

C. "Why are you crying right now if you really are over the breakup?"

"Why" questions about a discrepancy tend to feel accusatory and invite justification rather than exploration.

D. "It sounds like you have worked very hard to move on from this breakup."

This supportive reflection ignores the discrepancy entirely, so it is not a confrontation.

Rationale: In counseling, confrontation means pointing out a discrepancy (between words and nonverbal behavior, between statements, or between words and actions) in a caring, descriptive way. The "you say ... and yet ..." format names both sides without judgment and invites the client to examine the mixed message. Accusing, interpreting or asking "why" tends to raise defenses.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: confrontation of discrepancies

☐ Approve ☐ Revise ☐ Retire Notes:

Over six sessions, a client has said repairing his marriage is his top goal, yet he has cancelled each couples appointment he scheduled. The alliance is strong. The counselor gently notes this discrepancy, and the client is quiet, then says, "I guess I've been avoiding it because I'm afraid she'll tell me she wants out." What should the counselor do NEXT?

A. Restate the discrepancy more firmly so that its importance is not lost on him

He has already acknowledged the discrepancy; pressing it harder adds pressure without adding understanding.

B. Shift to a lighter topic to ease the tension that the confrontation created

Retreating just as he opens up signals that his fear is too much to talk about and loses the opening the confrontation made.

C. Ask him to commit now to attending the next couples appointment he schedules

Moving straight to a commitment skips exploration of the fear that drives the avoidance and risks hollow compliance.

D. Reflect his fear and explore what avoiding the sessions has protected him from ✓ **KEY**

Following a confrontation, the counselor attends to the client's response; he has accepted the discrepancy, so exploring the underlying fear deepens the work.

Rationale: A confrontation is only as useful as what follows it. The counselor watches how the client responds (denial, partial acceptance or full recognition) and builds on it. Here the client has recognized the discrepancy and named a fear, so empathic reflection and exploration of that fear come before planning new behavior.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: confrontation and evaluating the client's response

► **Check:** Applied-skills judgment item; confirm the Gladding locator on confrontation and that "explore before planning" is the clearly best sequencing.

☐ Approve ☐ Revise ☐ Retire Notes:

A client whose panic attacks resolved months ago has a history of losing caregivers in childhood. Three sessions before a jointly planned ending, she reports two new panic attacks and asks whether she can keep coming "for as long as it takes." What is the BEST response?

A. Agree to continue open-ended weekly counseling, since her symptoms have now returned

Reflexively extending treatment can confirm her fear that she cannot cope without the counselor and skips the meaning of the ending.

B. Explore her feelings about ending, normalize a brief return of symptoms and review gains ✓ **KEY**

A temporary return of symptoms near termination is a familiar reaction, especially with a history of loss; processing the ending and her skills is the core task.

C. Keep to the plan and avoid discussing the attacks, so as not to reinforce her symptoms

Ignoring the attacks and her feelings about ending leaves important material unprocessed and may feel like another abandonment.

D. Refer her to a new counselor now so that she has ongoing support once the sessions end

An abrupt transfer just before ending sidesteps the termination work and offers no reason why a new counselor is needed.

Rationale: Termination can stir up feelings of loss, particularly for clients with histories of abandonment, and a temporary regression is common. The counselor normalizes this, explores what ending means to the client, reviews her progress and coping skills, and may adjust the pace (for example, spacing sessions) rather than dropping or reflexively extending the plan.

References: Gladding (Counseling: A Comprehensive Profession) — Termination: client reactions and resistance to ending

☐ Approve ☐ Revise ☐ Retire Notes:

A client has met his counseling goals. Rather than stopping abruptly, the counselor proposes moving from weekly sessions to sessions every three weeks, followed by a booster session two months later. What is the MAIN purpose of this plan?

A. To let him practice new skills on his own while support is still available ✓ KEY

Spacing sessions and adding a booster lets the client rely on himself while the counselor can still help consolidate gains and plan for setbacks.

B. To keep him from forming an unhealthy dependence in the early sessions

The plan addresses the end of counseling, not the formation of dependence early in treatment.

C. To meet an ACA requirement that each ending include follow-up sessions

The ACA Code of Ethics does not require follow-up sessions for every termination.

D. To delay the ending until he no longer reports any symptoms whatsoever

The goals are already met; waiting for a symptom-free state is not the aim of tapering.

Rationale: Gradually spacing sessions (fading) and scheduling a booster or follow-up session are common termination practices. They allow the client to test and consolidate new skills independently, give room to address relapse prevention, and make the ending less abrupt.

References: Gladding (Counseling: A Comprehensive Profession) — Termination: fading sessions and follow-up

☐ Approve ☐ Revise ☐ Retire Notes:

A client reports frequent headaches that keep her home from social events. Her Adlerian counselor asks, "If these headaches went away tomorrow, what would you be doing differently in your life?" What is the PRIMARY purpose of this question?

A. To help her picture a future with no problem, as in a miracle question

The miracle question is a solution-focused technique for building goals; the Adlerian question has a diagnostic aim.

B. To uncover a repressed conflict that is being converted into pain

Conversion of repressed conflict is a psychoanalytic formulation, not the purpose Adlerians give this question.

C. To learn whether the symptom serves a purpose, such as avoiding a task ✓ KEY

"The Question" helps the counselor see whether the symptom has a social purpose, such as excusing the client from a life task she fears.

D. To rate how much the headaches interfere with her daily functioning

Rating functional impairment is an assessment task; the question is meant to reveal the goal of the symptom.

Rationale: Adlerians view symptoms as purposeful. "The Question" (asking what the client would do if the symptom were gone) helps distinguish problems that may be largely physical from those that serve a goal, such as avoiding friendship, work or love tasks. If the answer names something the client is avoiding, the symptom may be safeguarding her from it.

References: Corey (Theory & Practice) — Adlerian Therapy: "The Question" and the purposeful nature of symptoms

☐ Approve ☐ Revise ☐ Retire Notes:

A teacher consults a school counselor about a 9-year-old who tears up classmates' artwork and makes cruel remarks to the teacher. The teacher says she feels deeply hurt and wonders how a child could treat her this way. Using Dreikurs's goals of misbehavior, which goal is the child MOST likely pursuing?

A. Attention

An attention goal typically leaves the adult feeling annoyed or irritated, not hurt.

B. Power

A power goal typically leaves the adult feeling angry, provoked or challenged.

C. Display of inadequacy

Display of inadequacy leaves the adult feeling hopeless or discouraged as the child gives up.

D. Revenge ✓ KEY

When adults feel hurt, the child is often seeking revenge, trying to hurt others as he feels hurt.

Rationale: Rudolf Dreikurs, who extended Adler's ideas to parenting and schools, described four mistaken goals of misbehavior: attention, power, revenge and display of inadequacy. A practical clue is the adult's own reaction: annoyance (attention), anger (power), hurt (revenge) and hopelessness (inadequacy). Identifying the goal helps adults stop reinforcing it and encourage the child.

References: Corey (Theory & Practice) — Adlerian Therapy: Dreikurs and the goals of misbehavior

► **Check:** Dreikurs's four goals are standard; confirm that the cited Corey edition covers them (Gladding's child/school chapters are an alternative locator).

☐ Approve ☐ Revise ☐ Retire Notes:

A client sees himself as "a calm person who doesn't get angry." When his partner says he shouted during an argument, he insists he was "just being emphatic." In Rogers's theory of personality, this BEST illustrates:

A. Reaction formation against an unacceptable id impulse of anger

Reaction formation (expressing the opposite of an impulse) is a Freudian defense rooted in id-superego conflict, not Rogers's framework.

B. Distortion of an experience that threatens the self-concept ✓ KEY

Rogers held that experiences inconsistent with the self-concept are distorted or denied to awareness, maintaining incongruence.

C. Introjection of a parent's rules, swallowed whole without question

Introjection is a gestalt contact boundary disturbance about uncritically accepting others' values.

D. Minimization, a cognitive distortion that should be disputed

Minimization is a CBT term, and person-centered counselors do not dispute thoughts.

Rationale: In Rogers's theory, when an experience (here, anger) does not fit the self-concept, the person may perceive it selectively (distortion) or keep it out of awareness entirely (denial). The gap between self-concept and experience is incongruence, which produces vulnerability and anxiety. The core conditions make it safe enough to let such experiences into awareness.

References: Corey (Theory & Practice) — Person-Centered Therapy: incongruence, distortion and denial

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I suppose I'm kind of annoyed with my sister, but it's not a big deal," while clenching her fist. Which option BEST matches how a person-centered counselor and a gestalt counselor would each MOST likely respond?

A. Person-centered: reflect the annoyance she is starting to own; **gestalt:** ask her to give her fist a voice ✓ **KEY**

Person-centered counselors follow and empathically reflect the client's emerging feeling; gestalt counselors direct attention to present bodily awareness and often invite the client to speak as the body part.

B. Person-centered: ask her to exaggerate the clenched fist; **gestalt:** reflect the annoyance she now owns

The pairing is reversed: exaggeration is a gestalt experiment, and nondirective reflection is person-centered.

C. Person-centered: interpret her anger as displaced; **gestalt:** dispute her belief that anger is no big deal

Interpreting displacement is psychoanalytic and disputing beliefs is REBT; neither fits these two approaches.

D. Person-centered: advise her on how to talk with her sister; **gestalt:** explore early memories of the sister

Person-centered counselors avoid advice, and gestalt work stays with present experience rather than early memories.

Rationale: Both approaches are humanistic and value awareness, but they differ in counselor activity. A person-centered counselor stays nondirective, offering empathy so the client can more fully own her feeling. A gestalt counselor is more active, noticing nonverbal behavior and creating experiments, such as giving the fist a voice, to heighten here-and-now awareness.

References: Corey (Theory & Practice) — Person-Centered Therapy: empathic reflection and the nondirective stance · Corey (Theory & Practice) — Gestalt Therapy: attending to nonverbal behavior and experiments

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old client says, "I have no choice but to stay in a job I hate. My family expects it." Which counselor response is MOST consistent with existential therapy?

A. Dispute the irrational belief that he must meet his family's expectations

Disputing "must" beliefs is the core REBT technique, not the existential focus.

B. Explore how his early bond with his father shaped this need to comply

Tracing present behavior to early relationships is a psychodynamic focus.

C. Help him see that staying is a choice and explore what it means to him ✓ **KEY**

Existential counselors highlight freedom and responsibility, helping clients recognize that they are choosing even when they claim they have no choice.

D. Rehearse assertive statements he can use when his family raises it

Assertiveness rehearsal is a behavioral skills intervention.

Rationale: Existential therapy holds that people are free and therefore responsible for their choices. Statements such as "I have no choice" are seen as ways of avoiding that responsibility. The counselor helps the client own his choices, face the anxiety that comes with freedom, and decide how he wants to live.

References: Corey (Theory & Practice) — Existential Therapy: freedom and responsibility

☐ Approve ☐ Revise ☐ Retire Notes:

A 67-year-old client recently told his cancer is terminal says, "What's the point of anything now?" He is not suicidal. Which counselor response is MOST characteristic of an existential approach rather than a cognitive behavioral one?

A. "What evidence tells you that nothing has a point? Let's examine it together."

Testing the evidence for a thought is cognitive restructuring, a CBT method.

B. "Facing death has you asking what matters. What gives your days meaning now?" ✓ **KEY**

The existential counselor treats the question as a genuine confrontation with death and meaning and invites him to explore it.

C. "Let's schedule a few small, pleasant activities each day to lift your mood."

Scheduling pleasant activities is behavioral activation, a CBT strategy for low mood.

D. "Tell me about earlier losses in your life that this news is stirring up."

Linking present reactions to past losses reflects a psychodynamic focus.

Rationale: A CBT counselor might treat "What's the point?" as a hopeless automatic thought to test or as low mood to activate. An existential counselor sees it as an authentic encounter with the givens of death and meaninglessness and helps the client search for meaning, values and connection in the time that remains. Yalom argued that facing death honestly can deepen how one lives, and Frankl that meaning can be found even in unavoidable suffering.

References: Corey (Theory & Practice) — Existential Therapy: death, meaning and the search for meaning

☐ Approve ☐ Revise ☐ Retire Notes:

Two counselors treat clients who fear elevators. One teaches relaxation and works up a ranked list of feared scenes. The other has the client ride an elevator repeatedly for a long session, without leaving, until the anxiety subsides. Which statement accurately contrasts these approaches?

A. Flooding pairs relaxation with each step; desensitization keeps the client at peak fear

This reverses the methods: relaxation pairing belongs to systematic desensitization.

B. Both rely on reciprocal inhibition, but flooding is limited to scenes that are imagined

Flooding does not rely on a competing relaxation response, and it can be done in vivo or in imagination.

C. Desensitization blocks escape; flooding lets the client stop at any step of a hierarchy

This reverses them: flooding prevents escape, while desensitization moves gradually and lets the client pause.

D. Flooding is intense and blocks escape; desensitization is graded and paired with relaxation ✓ **KEY**

Flooding uses prolonged, high-intensity exposure with response prevention; systematic desensitization pairs relaxation with a graded hierarchy.

Rationale: Wolpe's systematic desensitization uses relaxation training, an anxiety hierarchy and gradual exposure so that relaxation inhibits anxiety. Flooding exposes the client to a highly feared stimulus for a prolonged period, in vivo or imaginal, while preventing escape, so anxiety extinguishes. Flooding often works faster but is more distressing and needs careful informed consent.

References: Corey (Theory & Practice) — Behavior Therapy: systematic desensitization, in vivo exposure and flooding

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor, a 13-year-old and his parents write and sign an agreement: on each school night he finishes his homework by 8 p.m., he earns 30 minutes of video games, and his father will check the work and deliver the reward. Which behavioral technique is this?

A. Contingency contract ✓ **KEY**

A contingency contract is a written agreement that specifies the target behavior, the consequence and who delivers it.

B. Token economy

A token economy uses tokens or points later exchanged for backup reinforcers; no tokens are involved here.

C. Response cost

Response cost removes a reinforcer after an unwanted behavior; this plan adds a reward for a wanted one.

D. Covert sensitization

Covert sensitization pairs an unwanted behavior with an imagined aversive event.

Rationale: Contingency contracts make the "if-then" relationship between behavior and consequence explicit. Effective contracts define the behavior in observable terms, specify the reinforcer, name who monitors and delivers it, and are agreed to by everyone involved, which increases the client's sense of ownership.

References: Corey (Theory & Practice) — Behavior Therapy: operant techniques and contingency contracts

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "My boss walked right past me this morning without saying hello. I'm going to get fired." Which counselor response BEST illustrates Socratic questioning?

A. "Your boss was probably just busy, so I really doubt you will be fired."

Reassurance supplies the counselor's conclusion instead of helping the client examine the thought himself.

B. "That is catastrophizing, so try replacing it with a positive thought."

Labeling the distortion and prescribing a replacement is didactic, not guided discovery.

C. "What are some other reasons your boss might have walked past you?" ✓ **KEY**

The open question guides the client to generate alternative explanations and weigh the evidence for himself.

D. "How did you feel in that moment when your boss walked past you?"

Exploring the feeling is useful but does not question the accuracy of the automatic thought.

Rationale: Socratic questioning, central to Beck's guided discovery, uses open questions to help clients examine evidence, consider alternative explanations and reach their own conclusions about a thought. Counselor-supplied reassurance or labeling may be accurate but tends to be less persuasive than conclusions the client reaches.

References: Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's Socratic dialogue and guided discovery

☐ Approve ☐ Revise ☐ Retire Notes:

A client with depression says she used to love her pottery class and will start going again "once I feel more motivated." Which response BEST reflects behavioral activation?

A. Agree to wait until her mood lifts, since pushing activity now could set her up to fail

Behavioral activation holds that waiting for motivation keeps clients stuck; action is expected to come first.

B. Schedule a pottery session this week and rate her mood and mastery after it ✓ **KEY**

BA works "outside-in": the client schedules valued activity according to a plan, not a mood, and tracks its effects.

C. First identify and dispute the core belief that is behind her lack of motivation

Disputing core beliefs is cognitive restructuring; BA targets behavior and avoidance directly.

D. Explore the earlier losses that may account for her loss of interest in pottery

Exploring past losses reflects a psychodynamic focus rather than activation.

Rationale: Behavioral activation treats depression by reducing avoidance and increasing contact with rewarding, valued activities. Clients monitor activities and mood, then schedule specific activities according to a plan rather than waiting to feel like it, often rating pleasure and mastery. Motivation tends to follow action rather than precede it.

References: APA CPG — Depression guideline (2019): behavioral therapy, including behavioral activation

☐ Approve ☐ Revise ☐ Retire Notes:

On a thought record, a client has written the situation, her automatic thought ("Everyone at the party thought I was boring") and her emotion (embarrassment, 85%). What does a cognitive therapy thought record typically ask her to do NEXT?

A. Trace the thought back to the childhood memory that first produced it

Finding the childhood origin of a thought is not the next step of a thought record.

B. Repeat a positive affirmation until the thought no longer feels true

Affirmations replace the thought without examining it, which is not cognitive restructuring.

C. Describe the thought in more detail without judging if it is accurate

Noticing thoughts without evaluating them is closer to mindfulness or defusion than to a thought record.

D. Weigh the evidence and write a more balanced, alternative response ✓ **KEY**

Thought records move from identifying the thought to evaluating it and writing an adaptive response, then re-rating the emotion.

Rationale: A thought record structures cognitive restructuring: situation, automatic thought, emotion and its intensity, then evaluation of the thought (evidence for and against, alternative explanations) leading to a balanced or adaptive response, and finally a re-rating of the emotion. It teaches clients to treat thoughts as hypotheses rather than facts.

References: Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's cognitive therapy and recording automatic thoughts

☐ Approve ☐ Revise ☐ Retire Notes:

Freud called it the "fundamental rule" of psychoanalysis: the client reports whatever comes to mind, however trivial, illogical or embarrassing it seems, without censoring it. This technique is called:

A. Free association ✓ KEY

Free association asks the client to say everything that comes to mind, opening a path to unconscious material.

B. Dream analysis

Dream analysis interprets the manifest and latent content of dreams; it is a separate technique.

C. Interpretation

Interpretation is the analyst's explanation of the meaning of material, not the client's uncensored reporting.

D. Catharsis

Catharsis is the release of pent-up emotion, which may occur but is not the reporting rule itself.

Rationale: Free association is the basic method of psychoanalysis. By suspending conscious censorship, the client lets associations lead toward repressed wishes and conflicts. The analyst attends especially to blocks, pauses and changes of topic, which may signal resistance.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: free association

☐ Approve ☐ Revise ☐ Retire Notes:

After gaining the insight that he ends relationships once they become close, a client begins arriving late and "forgets" a session. Weeks later he describes pulling away from a new partner in the same way. From a psychoanalytic perspective, which statement BEST describes what is happening and how to proceed?

A. The lateness reflects low motivation, so firm attendance rules with consequences should be set

Treating the lateness as a compliance problem misses its unconscious meaning, which is the analyst's main interest.

B. The repetition shows the insight was wrong, so the original interpretation should be dropped

Repetition after insight is expected; it does not by itself show the interpretation was wrong.

C. The lateness is resistance to be explored, and the repeated pattern calls for working through ✓ KEY

Lateness and forgetting after a painful insight are classic resistance, and insight is consolidated by working through the conflict repeatedly.

D. The relapse shows a need for catharsis, so his feelings should be discharged more fully

Emotional discharge alone was abandoned by Freud as a mechanism of lasting change.

Rationale: In psychoanalytic therapy, resistance is anything that keeps unconscious material from awareness, such as lateness, missed sessions or changing the subject, and it is interpreted rather than punished. A single insight rarely changes a pattern; working through is the repeated exploration of the same conflict as it appears in many situations and in the transference until change takes hold.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: interpreting resistance and working through

☐ Approve ☐ Revise ☐ Retire Notes:

A 40-year-old moved across the country at 20 and has not spoken to his parents in ten years. He says, "I've put all that behind me." His father also stopped speaking to his own family as a young man, and the client's marriage now swings between intense closeness and angry distance. How would a Bowenian therapist MOST likely understand and approach this?

A. His distance shows a high level of differentiation, so therapy should focus only on the marriage

For Bowen, physical or emotional distance is not differentiation; cutoff reflects unresolved fusion with the family.

B. The family's rigid boundaries should be restructured by having it enact the conflict in session

Boundaries and enactments are structural (Minuchin) concepts, not Bowen's.

C. The marriage problems call for a paradoxical directive to have the couple fight on purpose

Paradoxical directives are a strategic technique; Bowen coaching emphasizes calm, reflective self-work.

D. Cutoff manages unresolved fusion passed down the generations; he would be coached to reconnect ✓ **KEY**

Emotional cutoff handles fusion by distance, and its repetition across generations reflects multigenerational transmission; Bowen coaching helps clients re-engage their families while staying differentiated.

Rationale: Bowen saw emotional cutoff as a way of managing unresolved attachment by distance, not as a sign of maturity; the fusion tends to reappear in other close relationships, such as a marriage. Multigenerational transmission explains how such patterns and levels of differentiation pass down family lines. Therapy often involves a genogram and coaching the client to make calm, person-to-person contact with family of origin.

References: Goldenberg (Family Therapy) — Bowen family systems theory: emotional cutoff and multigenerational transmission process

☐ Approve ☐ Revise ☐ Retire Notes:

In a first session with a family, a counselor lets the father speak first as the family expects, picks up the family's own phrases and pace, and follows their story. Afterward she sketches a diagram showing a diffuse boundary between the mother and son and a rigid one around the father. In structural family therapy, these two activities are BEST described as:

A. Triangulating and then detriangulating the parental subsystem

Triangulation and detriangulation are Bowen concepts about drawing in a third person, not this sequence.

B. Joining the family, then mapping its boundaries and subsystems ✓ **KEY**

Accommodating to the family's style and hierarchy is joining; the boundary diagram is a structural map.

C. Prescribing the symptom, then reframing the family's problem

Symptom prescription and reframing are strategic techniques, and neither occurs here.

D. Building a genogram to trace patterns across three generations

A genogram traces multigenerational patterns (Bowen); a structural map shows current boundaries and alignments.

Rationale: Minuchin's structural therapist first joins the family, accommodating to its style through maintenance, tracking and mimesis, to become accepted as a leader of the therapeutic system. Family mapping then charts current structure: subsystems, hierarchy and boundaries that are clear, diffuse (enmeshed) or rigid (disengaged), guiding later restructuring interventions.

References: Goldenberg (Family Therapy) — Structural family therapy: joining, accommodating and family mapping

☐ Approve ☐ Revise ☐ Retire Notes:

At intake, a counselor explains to a client with a history of sexual assault what each part of the interview involves and why questions are asked. She lets the client choose where to sit and whether the door stays open, and says she may skip any question. These practices BEST reflect:

A. Trauma-informed principles of safety, transparency and choice that limit retraumatization ✓ KEY

Explaining procedures, offering choices and control, and attending to physical safety are core trauma-informed practices.

B. Early exposure to trauma cues so that habituation can begin right from the first session

Nothing here exposes the client to trauma cues; the aim is to reduce, not evoke, distress at intake.

C. Analytic neutrality and abstinence so that transference can develop without interference

Neutrality and abstinence are psychoanalytic stances and do not describe sharing control with the client.

D. Standardized intake procedures that apply the very same steps to every client at the agency

Offering individual choices is the opposite of a rigid, uniform procedure.

Rationale: Trauma-informed care assumes that many clients have trauma histories and organizes services to avoid retraumatization. Practices include physical and emotional safety, transparency about what will happen, and collaboration, choice and empowerment, so that clients regain a sense of control that trauma took away.

References: SAMHSA TIP 57 — Trauma-informed care principles: safety, trustworthiness, choice and avoiding retraumatization

☐ Approve ☐ Revise ☐ Retire Notes:

A veteran with PTSD writes an impact statement about why he thinks a friend died in an ambush and how it changed his beliefs about safety, trust, power, esteem and intimacy. His therapist then helps him identify "stuck points," such as "It was my fault," and examine them with structured worksheets. Which treatment is this?

A. Prolonged exposure therapy

Prolonged exposure (Foa) centers on repeated imaginal and in vivo exposure, not written impact statements and stuck-point worksheets.

B. EMDR

EMDR (Shapiro) uses bilateral stimulation while the client holds the memory in mind, within an eight-phase protocol.

C. Cognitive processing therapy ✓ KEY

CPT (Resick) uses the impact statement, stuck points and worksheets across five themes: safety, trust, power/control, esteem and intimacy.

D. Stress inoculation training

Stress inoculation training (Meichenbaum) builds coping skills in three phases; it does not target stuck points.

Rationale: Cognitive processing therapy, developed by Patricia Resick, is a trauma-focused cognitive therapy recommended in major PTSD practice guidelines. Clients write an impact statement, identify stuck points (often self-blame or overgeneralized beliefs), and use worksheets to challenge them, with later sessions on safety, trust, power and control, esteem and intimacy.

References: APA CPG — PTSD guideline (2017): cognitive processing therapy · VA/DoD CPG — PTSD guideline: trauma-focused psychotherapies (CPT, PE, EMDR)

☐ Approve ☐ Revise ☐ Retire Notes:

According to classic crisis theory, which statement BEST describes an acute crisis?

A. A sign of underlying mental illness that calls for long-term treatment

Crisis is viewed as a normal response to an overwhelming event, not evidence of psychopathology.

B. A property of the event, so anyone facing it will be in crisis

Crisis depends on the person's perception of the event and coping resources; the same event may not cause a crisis for everyone.

C. A chronic state that lasts until the person receives counseling

Crisis states are time-limited; a person reaches some new equilibrium with or without help.

D. A time-limited state that resolves within weeks, for better or worse ✓ **KEY**

Caplan described crisis as a temporary disequilibrium that usually resolves within about four to six weeks, in an adaptive or maladaptive direction.

Rationale: Crisis theory, building on Lindemann and Caplan, holds that a crisis arises when a person perceives an event as overwhelming their usual coping. The state is time-limited, commonly cited as about four to six weeks, and resolves at a higher, equal or lower level of functioning. Because it contains both danger and opportunity, brief, prompt intervention can tip the outcome toward growth.

References: James & Gilliland (Crisis Intervention Strategies) — Approaching crisis intervention: definitions of crisis and basic crisis theory

► **Check:** The "four to six weeks" duration is the commonly taught figure from Caplan; confirm wording against the cited James & Gilliland edition.

☐ Approve ☐ Revise ☐ Retire Notes:

Hours after a house fire, a client sits staring at the floor, cannot say where she will sleep tonight, and does not respond to questions about whom she might call. Her triage assessment shows severe impairment. Using James and Gilliland's continuum of crisis worker involvement, which stance is MOST appropriate?

A. Nondirective, letting her generate her own plan so that her sense of control is not taken away

A nondirective stance fits clients who are mobile and able to act; she cannot currently generate options.

B. Directive, taking charge to assess safety, give clear guidance and arrange immediate shelter ✓ **KEY**

When a client is immobile and severely impaired, the worker becomes directive, taking temporary control to meet urgent needs.

C. Collaborative, working as an equal partner to weigh options that she suggests for the night

Collaboration suits clients with partial mobility; she is not yet able to contribute options.

D. Deferred, holding off on any action until a full psychosocial history has been completed

Crisis work addresses immediate safety and needs first; a full history is not a prerequisite for action.

Rationale: James and Gilliland describe a continuum of worker involvement matched to the client's level of mobility, as gauged by triage assessment of affective, behavioral and cognitive functioning. Immobile clients need a directive worker; partially mobile clients, a collaborative one; and mobile clients, a nondirective one. The stance shifts toward less direction as the client regains functioning.

References: James & Gilliland (Crisis Intervention Strategies) — Six-step model: directive, collaborative and nondirective involvement; Triage Assessment Form

☐ Approve ☐ Revise ☐ Retire Notes:

Partway through a session, a court-referred client crosses his arms and says, "You're just like my probation officer, always telling me what to do." Which counselor response BEST reflects how motivational interviewing handles discord?

A. "I'm not telling you what to do. I'm only giving you the facts here."

Defending oneself argues with the client and tends to increase discord.

B. "If you don't engage here, I will have to report that to your officer."

A warning or threat is confrontational and escalates the conflict in the relationship.

C. "You're tired of being told what to do. Here, the choices are yours." ✓ **KEY**

Reflecting his frustration and emphasizing autonomy de-escalates discord without arguing.

D. "Let's get back to your plan. What will you change before next week?"

Pushing ahead ignores the strain in the relationship and is likely to deepen it.

Rationale: In current MI, discord refers to tension in the working relationship, distinct from sustain talk about the behavior. Counselors respond without arguing: reflecting, apologizing, affirming or shifting focus, and emphasizing the client's autonomy. This replaces the older idea of "rolling with resistance" and keeps the counselor from triggering further defensiveness.

References: Miller & Rollnick (MI) — Responding to sustain talk and discord; emphasizing autonomy

☐ Approve ☐ Revise ☐ Retire Notes:

Asked to rate on a 0 to 10 scale how important it is to her to stop vaping, a 19-year-old says, "Maybe a 3." Which follow-up question is MOST likely to elicit change talk?

A. "Why a 3 and not a 1 or a 2?" ✓ **KEY**

Asking why the number is not lower invites her to state her own reasons for change, which is change talk.

B. "Why only a 3 and not higher?"

Asking why the number is not higher invites her to defend the status quo, eliciting sustain talk.

C. "What would get you to a 10?"

Jumping to the top of the scale is a large leap that tends to bring out reasons it is not possible.

D. "Isn't a 3 a bit low for you?"

This implies judgment and reflects the righting reflex, which tends to provoke defensiveness.

Rationale: Importance and confidence rulers are a common MI strategy for evoking change talk. The key follow-up is to ask why the client chose that number rather than a lower one, which prompts her to voice her own reasons for change. Asking why the number is not higher tends to elicit sustain talk instead.

References: Miller & Rollnick (MI) — Evoking change talk: importance and confidence rulers

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor holds individual pre-group interviews for a 12-week interpersonal process group. She spends each interview asking about symptoms, history and current stressors, then tells applicants whether they have been accepted. Her supervisor says an essential element of screening is missing. Which element is it?

A. Requiring a formal DSM-5-TR diagnosis before any applicant joins

A diagnosis is not required for most counseling groups, and it says little about fit with a particular group.

B. Letting applicants ask questions and judge whether the group fits ✓ **KEY**

Screening is a two-way process: applicants learn about the group and leader and decide whether it meets their needs.

C. Ranking applicants by a personality inventory to pick the strongest group

Selection rests on fit with group goals and readiness, not on ranking applicants by a test score.

D. Asking applicants to agree to meet socially with members between sessions

Outside socializing is not encouraged in therapy groups and is not a screening goal.

Rationale: Pre-group screening selects members whose needs and goals fit the group, who will not impede the process and whose well-being will not be jeopardized by it. It is also a chance for prospective members to interview the leader, learn about the group's purpose and procedures and decide for themselves whether to join. A one-sided interview that only gathers clinical data misses that mutual decision.

References: ASGW Best Practice Guidelines — Planning: group and member preparation (screening and orientation) · ACA Code of Ethics — A.9.a Screening

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is preparing the informed consent discussion for a new adult therapy group. Which statement BEST belongs in that discussion?

A. The leader guarantees that every member will keep what is said confidential.

Leaders cannot guarantee that other members will keep confidentiality, and consent must state this limit.

B. Once you join, you are expected to attend until the final group session ends.

Members keep the right to leave; consent explains how to leave, not that leaving is barred.

C. You may feel pressure to share, but you decide what and how much to disclose. ✓ **KEY**

Naming the risk of group pressure and affirming the member's control over disclosure is core group consent content.

D. The leader will decide how much personal material each member shares weekly.

Members, not the leader, decide what they disclose; leaders protect members from coercion.

Rationale: Informed consent for a group covers its purpose, procedures and leader qualifications, the limits of confidentiality among members, and the psychological risks of participation, such as pressure to disclose, confrontation and changes that affect outside relationships. Members are told that they control their own disclosures and how to leave the group if they choose.

References: ASGW Best Practice Guidelines — Planning: group and member preparation (informed consent) · ACA Code of Ethics — A.2.b Types of Information Needed; B.4.a Group Work

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a community clinic is planning an open-ended outpatient interpersonal therapy group for adults. Which plan for size and session length is MOST consistent with Yalom's recommendations?

A. About 3 members, meeting weekly for 30 minutes

Three members is too few for rich interaction, and 30 minutes leaves no time to warm up and work.

B. About 15 members, meeting weekly for 45 minutes

Fifteen members is too many for interaction-focused work, and 45 minutes is too short.

C. About 12 members, meeting weekly for 3 hours

Twelve is above the usual range, and sessions beyond about 2 hours tend to tire members.

D. About 7 members, weekly for 80-90 minutes ✓ **KEY**

Yalom places the ideal interactional group near 7 or 8 members, with sessions of about 80 to 90 minutes.

Rationale: Yalom suggests that an interactional therapy group works best with about 7 or 8 members, within an acceptable range of roughly 5 to 10. Sessions need time for a warm-up period and for working through themes, so most groups meet for 80 to 90 minutes; beyond about 2 hours members tend to become weary. Groups for children or for highly impaired inpatients are usually smaller and shorter.

References: Yalom & Leszcz (Group Psychotherapy) — Creation of the group: group size and duration of meetings

☐ Approve ☐ Revise ☐ Retire Notes:

158. nce-s-cou-214 · Group Counseling and Group Work · group leadership skills and therapeutic factors · medium · In review

In the sixth session of a process group, members keep switching topics whenever anyone mentions loneliness. The leader is considering a self-disclosure. Which disclosure is MOST consistent with Yalom's guidance on leader transparency?

A. "I went through a lonely divorce years ago, so let me tell you how I got through it."

A detailed personal history shifts the focus to the leader and rarely helps members with their own here-and-now work.

B. "I feel a bit shut out as we keep changing topics. I wonder what others feel now." ✓ **KEY**

A brief here-and-now disclosure of the leader's reaction models openness and invites members to examine the process.

C. "I have struggles of my own lately, so I am one more member of the group tonight."

Presenting oneself as a member blurs the leader role and burdens the group with the leader's problems.

D. "I keep my reactions private so that you can project onto me more freely here."

A blank-screen stance is not what Yalom advises; judicious transparency aids the interactional group.

Rationale: Yalom favors leader self-disclosure that is focused on the here-and-now and offered in the service of members' work, such as sharing a reaction to what is happening in the room. Lengthy disclosure of the leader's past or current personal problems tends to redirect attention, burden members and blur the leader role, while total opacity deprives the group of useful modeling.

References: Yalom & Leszcz (Group Psychotherapy) — The therapist: transparency and the use of the self

☐ Approve ☐ Revise ☐ Retire Notes:

Near the end of a co-led depression group, a member quietly says she has stockpiled pills and "probably won't be around next week." Other members look stunned. What should the leaders do?

A. Note her comment and plan to raise it with her at the start of next week's session

Waiting a week ignores an imminent risk statement; risk must be assessed immediately.

B. Ask members to share reasons to live so the group can talk her out of the plan now

Peer persuasion is not a risk assessment and puts members in a role they cannot safely fill.

C. Remove her from the group and give her a referral for individual counseling instead

Removal without assessing and managing her immediate safety abandons her at a moment of risk.

D. Acknowledge it, have one leader assess her risk privately now, and help the group process ✓ **KEY**

Her safety comes first through an immediate individual assessment, while the other leader helps members with their reactions.

Rationale: A disclosure of suicidal intent during group calls for immediate action. The leaders acknowledge what was said, one leader (or the leader right after the group) conducts a private risk assessment and safety planning, and the group is helped to process its reactions now and in later sessions. Co-leadership makes it possible to attend to the individual without abandoning the group.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: handling a member in crisis; co-leadership · C-SSRS — Suicidal ideation with intent and plan: immediate risk assessment

► **Check:** Best-practice consensus rather than a single named standard; SME to confirm the Gladding locator.

☐ Approve ☐ Revise ☐ Retire Notes:

Three members of an interpersonal group have begun meeting for coffee before each session. In group they sit together, trade knowing glances and fall silent when one of them is challenged. Following Yalom, how should the leader BEST respond?

A. Ban all contact between members outside of group sessions for the rest of the group

Outside contact is not itself the problem, and a ban tends to drive it underground.

B. Invite the members to talk openly in group about their outside meetings and their effect ✓ **KEY**

Bringing the subgroup into the open turns a threat to cohesion into here-and-now material.

C. Meet privately with the three members and ask them to sit apart from each other in group

A private meeting adds secrecy and leaves the rest of the group out of the work.

D. Let the subgroup run its course, since cohesive pairs tend to strengthen the whole group

Secret subgroups whose loyalty outweighs the group's goals tend to undermine cohesion.

Rationale: Subgrouping becomes destructive when members' loyalty to the subgroup outweighs their commitment to the group's goals and when what happens outside stays secret. Yalom advises that extragroup contact be brought back into the group and discussed openly, so that it can be understood rather than forbidden.

References: Yalom & Leszcz (Group Psychotherapy) — Problem group members and subgrouping; extragroup socializing

☐ Approve ☐ Revise ☐ Retire Notes:

In a long-term group, Theo bristles at every comment from Ines, an assertive older member, and accuses her of "always looking down on me," though others see her remarks as mild. He later realizes she reminds him of his critical older sister. Drawing on Sullivan, Yalom calls Theo's distorted perception of Ines:

A. Parataxic distortion ✓ KEY

Parataxic distortion is perceiving another person through the lens of past relationships rather than as they are.

B. Consensual validation

Consensual validation is the corrective process of checking one's view against others' views, not the distortion.

C. Projective identification

This object relations term involves inducing one's projected feelings in another; Yalom uses Sullivan's term here.

D. Family recapitulation

Corrective recapitulation of the family group is a therapeutic factor, not the term for one member's distortion of another.

Rationale: Yalom borrows Sullivan's term parataxic distortion for the tendency to perceive others in the group through the template of earlier relationships, a form of transference that can occur between members as well as toward the leader. Feedback from other members, a process Sullivan called consensual validation, helps the member see the distortion and test it against reality.

References: Yalom & Leszcz (Group Psychotherapy) — Interpersonal learning: parataxic distortion and consensual validation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants to know whether her 10-week anxiety management group actually reduced members' symptoms. Which evaluation approach BEST answers that question?

A. Hand out a satisfaction survey at the last session of the group

Satisfaction tells whether members liked the group, not whether symptoms changed.

B. Record her own impressions of each member's progress each week

Leader impressions are subjective and open to bias; they do not measure symptom change.

C. Count how many members attended at least eight of ten sessions

Attendance shows engagement, not whether the group reduced symptoms.

D. Give a standardized anxiety measure before and after the group ✓ KEY

A valid symptom measure given at pre- and post-test directly assesses change in the target outcome.

Rationale: Outcome evaluation asks whether the group achieved its goals. Administering a reliable, valid measure of the target problem before and after the group (and, ideally, at points during it) provides evidence of change; satisfaction, attendance and leader impressions are process indicators, not outcome data.

References: ASGW Best Practice Guidelines — Performing and processing: evaluation of group outcomes · Erford (Research and Evaluation in Counseling) — Program evaluation: outcome evaluation with pretest-posttest measures

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is launching a video-based support group for caregivers who live across her state. Which step is MOST specific to the safe running of this telehealth group?

A. Ask members to keep their cameras turned off to protect each other's identities

Cameras off removes nonverbal cues and makes it harder to verify who is present and to monitor distress.

B. Skip pre-group screening, since online members can simply log off if it is a poor fit

Screening is as necessary for online groups as for in-person groups.

C. Get each member's location and emergency contact and set privacy rules for joining ✓ **KEY**

Knowing where members are, having emergency contacts and agreeing on privacy and no-recording rules address telehealth risks.

D. Use any free video app members already know, since familiarity improves attendance

Platforms must meet security and privacy requirements; convenience alone is not sufficient.

Rationale: Distance group work adds risks: members may join from places where others can overhear, could record sessions, and may be far from the counselor in a crisis. Counselors address these in informed consent and planning by verifying identity and location, collecting emergency contacts, using secure platforms and setting norms such as joining from a private space and not recording.

References: ACA Code of Ethics — H.2.a Informed Consent and Disclosure; H.3 Client Verification · ASGW Best Practice Guidelines — Planning: trends and technological changes

☐ Approve ☐ Revise ☐ Retire Notes: _____

164. nce-s-cou-220 · Career Development · career development theories (Holland, Super, Krumboltz) · hard · In review

For her first months as a hospital social worker, Amina followed every unit norm and sought her colleagues' approval. Now, secure in the role, she is pushing the team to adopt her ideas for discharge planning and asserting her own views. In Tiedeman and O'Hara's decision-making model, Amina is in which stage?

A. Crystallization

Crystallization is part of anticipation, when options are clarified before any choice is made.

B. Induction

Induction describes her earlier months, when she sought acceptance and conformed to the group.

C. Reformation ✓ **KEY**

In reformation the person, now accepted, acts assertively to change the group toward her own views.

D. Integration

Integration follows reformation, when individual and group goals are balanced into a stable sense of self.

Rationale: Tiedeman and O'Hara divided career decision making into anticipation (exploration, crystallization, choice and clarification) and implementation or adjustment (induction, reformation and integration). In induction the new member seeks acceptance; in reformation she becomes assertive and tries to change the group; in integration she achieves a balance between her own goals and the group's.

References: Sharf (Career Development Theory) — Tiedeman and O'Hara: anticipation and implementation stages of decision making

☐ Approve ☐ Revise ☐ Retire Notes: _____

165. nce-s-cou-221 · Career Development · career development theories (Holland, Super, Krumboltz) · medium · In review

A counselor uses Krumboltz's happenstance learning theory with a 30-year-old who wants to "figure out the one right career." According to this theory, how should the success of their counseling be judged?

A. By whether the client names one final occupation by the last session

The theory explicitly rejects a single career decision as the goal of counseling.

B. By the match between the client's interest code and a chosen field

Matching traits to occupations is a trait-factor goal; the theory uses assessments to spark learning.

C. By what the client accomplishes in the real world outside sessions ✓ **KEY**

Krumboltz held that success is assessed by the actions and learning the client achieves outside counseling.

D. By how certain the client reports feeling about future job choices

Certainty is not the aim; Krumboltz reframed open-mindedness and indecision as useful.

Rationale: Krumboltz's happenstance learning theory (2009) holds that the goal of career counseling is to help clients learn to take actions toward more satisfying lives, not to make a single decision. Assessments stimulate learning rather than match traits to jobs, clients learn exploratory actions that generate beneficial unplanned events, and success is judged by what the client accomplishes in the real world.

References: Sharf (Career Development Theory) — Krumboltz: happenstance learning theory and its four propositions

☐ Approve ☐ Revise ☐ Retire Notes:

166. nce-s-cou-222 · Career Development · career development theories (Holland, Super, Krumboltz) · medium · In review

A 47-year-old engineer has cut back his hours to care for his aging father, coaches his daughter's soccer team and is taking an evening course. He says work "matters less right now than it did five years ago." Which concept from Super's life-career rainbow BEST describes this?

A. Role salience, the shifting weight of concurrent life roles ✓ **KEY**

The rainbow shows several roles held at once across theaters, with their importance changing over the life span.

B. Career maturity, readiness for age-appropriate career tasks

Career maturity concerns readiness for developmental tasks, mainly in adolescence, not the balance of life roles.

C. Circumscription, the narrowing of acceptable occupations

Circumscription is Gottfredson's concept of eliminating occupations by sex type and prestige.

D. Congruence, the fit between personality and work setting

Congruence is Holland's concept of person-environment fit, not the weight of life roles.

Rationale: Super's life-career rainbow pictures the life span as arcs of roles such as child, student, leisurite, citizen, worker and homemaker, played in theaters such as home, community, school and workplace. Several roles are active at once, and their salience rises and falls over time, so a worker role can recede while caregiving, parenting and student roles grow.

References: Sharf (Career Development Theory) — Super: life-career rainbow, life roles and role salience

☐ Approve ☐ Revise ☐ Retire Notes:

A college sophomore scores very low on the Vocational Identity scale of Holland's My Vocational Situation. What does this score MOST likely indicate?

A. Her interests clash with the setting of her current campus job

This describes incongruence between person and environment, not vocational identity.

B. Her picture of her goals, interests and talents is unclear ✓ **KEY**

Holland defined vocational identity as having a clear and stable picture of one's goals, interests and talents.

C. She has a highly consistent code of adjacent types on the hexagon

Consistency describes how closely related a person's top types are, a different Holland concept.

D. She doubts her ability to finish the math courses for her major

Doubts about specific tasks reflect self-efficacy, a social cognitive career theory construct.

Rationale: Holland defined vocational identity as the possession of a clear and stable picture of one's goals, interests and talents. The My Vocational Situation measures vocational identity along with needs for occupational information and perceived barriers; low identity scores suggest a client who would benefit from clarifying self-knowledge before choosing among options.

References: Sharf (Career Development Theory) — Holland: vocational identity and the My Vocational Situation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor follows Fouad and Bingham's culturally appropriate career counseling model with a Somali American client. They have built a culturally appropriate relationship and identified her career issues. What is the NEXT step in the model?

A. Choose and apply culturally appropriate interventions

Interventions come after the impact of culture has been assessed and goals are set.

B. Set career counseling goals with the client together

Goal setting follows the assessment of cultural variables so that goals reflect them.

C. Support decision making, then plan implementation

Decision making and implementation are the final steps of the model.

D. Assess the impact of cultural variables on the issues ✓ **KEY**

After the relationship and the career issues, the model calls for assessing how cultural variables shape those issues.

Rationale: Fouad and Bingham's culturally appropriate career counseling model has seven steps: establish a culturally appropriate relationship, identify career issues, assess the impact of cultural variables on those issues, set counseling goals, make culturally appropriate interventions, support decision making, and implement and follow up. Placing cultural assessment before goals keeps goals and interventions grounded in the client's worldview.

References: Sharf (Career Development Theory) — Career counseling with culturally diverse clients: Fouad and Bingham's culturally appropriate model

► **Check:** Seven-step sequence from Fouad & Bingham (1995); SME to confirm that Sharf (6th ed.) presents the model and its step order.

☐ Approve ☐ Revise ☐ Retire Notes:

For more than a year a client has swung back and forth between staying in nursing and returning to art school, each time convincing herself the other path is right. In Pryor and Bright's chaos theory of careers, this pattern reflects which attractor?

A. Point attractor

A point attractor is movement toward a single fixed goal, not oscillation between two.

B. Torus attractor

A torus attractor is a complex but repeating routine that screens out change and chance.

C. Pendulum attractor ✓ KEY

A pendulum attractor describes swinging between two options or roles without resolution.

D. Strange attractor

The strange attractor is an open system that embraces complexity and chance, the pattern the theory favors.

Rationale: Pryor and Bright view careers as complex, dynamic systems shaped by nonlinear change and chance. They describe closed-system attractors (point, pendulum and torus) that represent attempts to control and simplify, and the strange attractor, an open-system pattern that combines stability with openness to change. A pendulum attractor is oscillation between two poles, such as two occupations.

References: Sharf (Career Development Theory) — Chaos theory of careers (Pryor and Bright): attractors

► **Check:** SME to confirm the chaos theory of careers is covered in the cited Sharf edition; attractor definitions follow Pryor & Bright (2011).

☐ Approve ☐ Revise ☐ Retire Notes:

170. nce-s-cou-226 · Career Development · career development theories (Holland, Super, Krumboltz) · medium · In review

Duc, 18, tells his counselor he will study pharmacy because his parents and grandparents decided it is best for the family, and he seems at ease with this. His family holds strongly collective social values. From the standpoint of Brown's values-based theory, how should the counselor view this choice?

A. As identity foreclosure that should be challenged so he chooses alone

Labeling a values-consistent family decision as pathology imposes an individualist norm.

B. As a sign of low career maturity needing decision-skills training

Deferring to family is consistent with collective values, not a deficit in maturity.

C. As secondary, since interests drive choice more than values do

Brown placed values, not interests, at the center of occupational choice.

D. As consistent with collective values, where family shares the choice ✓ KEY

Brown proposed that people with collective social values often make career decisions with, or in deference to, the family.

Rationale: Brown's values-based theory treats work values as central to occupational choice and recognizes that cultural values shape the decision process itself. People holding individualistic values tend to decide independently, while those with collective values tend to involve or defer to family or group. Counselors are urged to understand and work within the client's value system rather than impose an individualistic model.

References: Sharf (Career Development Theory) — Brown's values-based theory of occupational choice: cultural and work values

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is redesigning a career-choice workshop and wants to include the components that Brown and Ryan Krane's meta-analysis linked to the largest effects. Which set should she build in?

A. Written exercises, individual feedback, work information, modeling and support building ✓ KEY

These are the five critical ingredients Brown and Ryan Krane identified in career choice interventions.

B. Aptitude testing, computer guidance alone, job fairs, résumé drills and interview practice

Computer guidance used alone showed smaller effects, and the rest are not among the identified ingredients.

C. Group discussion, relaxation training, dream analysis, personality typing and goal rewards

These components were not among the ingredients linked to stronger career choice outcomes.

D. Interest inventories, self-scored results, labor statistics, cold calls and job shadowing

Self-interpreted inventories lack the individualized interpretation and feedback found to matter.

Rationale: Brown and Ryan Krane (2000) found five critical ingredients in effective career choice interventions: written exercises, individualized interpretation and feedback, information about the world of work, modeling opportunities, and attention to building support for choices in the client's social network. Interventions including more of these ingredients produced larger effects.

References: Brown & Ryan Krane (Career Interventions) — Five critical ingredients of effective career interventions · Sharf (Career Development Theory) — Effectiveness of career counseling: critical ingredients (Brown & Ryan Krane, 2000)

☐ Approve ☐ Revise ☐ Retire Notes:

172. nce-s-cou-228 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · easy · In review

Parents of a 16-year-old worry because over one school year he has tried out different friend groups, clothing styles, music and political views. He is otherwise doing well. From Erikson's perspective, how should the counselor understand this?

A. As normal exploration within the identity vs. role confusion stage ✓ KEY

Erikson saw adolescent experimentation with roles and values as a healthy part of forming an identity.

B. As a delayed crisis of the industry vs. inferiority stage of school age

Industry vs. inferiority concerns mastery and competence in middle childhood, not trying out identities.

C. As an early sign of the intimacy vs. isolation crisis of young adults

Intimacy vs. isolation follows identity and centers on close committed relationships.

D. As a warning sign that he lacks a self and needs intensive treatment

Role experimentation in a well-functioning teen is expected, not a sign of disorder.

Rationale: In Erikson's fifth stage, identity vs. role confusion, adolescents try out roles, values and commitments as they work toward a coherent sense of who they are. Erikson described a psychosocial moratorium, a period of permitted experimentation, as healthy; role confusion becomes a concern only when the young person cannot form any stable sense of self.

References: Berk (Development Through the Lifespan) — Adolescence: Erikson's identity vs. role confusion

☐ Approve ☐ Revise ☐ Retire Notes:

173. nce-s-cou-229 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · easy · In review

While talking with her grandmother on the phone, a 4-year-old nods in answer to questions and holds up a new toy, saying, "Look at this one!" Which feature of Piaget's preoperational stage does this show?

A. Animistic thinking

Animism is giving lifelike qualities to objects, which is not shown here.

B. Egocentrism ✓ KEY

She assumes her grandmother sees what she sees, failing to take another's viewpoint.

C. Lack of object permanence

Object permanence develops in the sensorimotor stage and is not what this shows.

D. Failure to conserve

Conservation concerns amounts that stay the same despite changes in appearance.

Rationale: Piaget described preoperational children as egocentric: they have difficulty distinguishing others' viewpoints from their own, as in the three-mountains task. A child who nods on the phone or holds up a toy for a listener who cannot see it assumes the other person shares her perspective.

References: Berk (Development Through the Lifespan) — Piaget's preoperational stage: egocentrism and the three-mountains problem

☐ Approve ☐ Revise ☐ Retire Notes:

174. nce-s-cou-230 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · medium · In review

A woman resolves a moral dilemma by focusing on preserving relationships and avoiding harm to everyone involved. Under Kohlberg's scoring she would likely be placed at Stage 3, below the postconventional level. Which critique, associated with Carol Gilligan, addresses this?

A. Kohlberg ignored the role of logical thought in moral judgment

Kohlberg built his stages on Piaget's account of cognitive development; logic was central.

B. Kohlberg's stages describe behavior rather than moral reasoning

Kohlberg studied reasoning about dilemmas, not behavior, and Gilligan did not argue otherwise.

C. Kohlberg built a justice bias from male samples, undervaluing care ✓ KEY

Gilligan argued that a theory based on males privileged justice and rated an ethic of care as less mature.

D. Kohlberg's stages must be completed fully before anyone advances

This restates Kohlberg's invariant sequence; it is not Gilligan's critique.

Rationale: Kohlberg's stages were derived from longitudinal interviews with boys and place reasoning based on abstract justice and rights at the postconventional level. Gilligan argued that this framework undervalues an ethic of care, which emphasizes relationships and responsibility, so care-oriented reasoning is often scored at Stage 3. Later research found smaller gender differences than Gilligan proposed, but the critique broadened the study of moral reasoning.

References: Berk (Development Through the Lifespan) — Moral development: Kohlberg's theory and Gilligan's ethic of care

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 10-year-old set many strict rules, rarely explain them beyond "because I said so," and show little warmth or responsiveness to his feelings. Which parenting style in Baumrind's framework is this, and which outcome does research MOST often link to it?

A. Permissive; impulsive behavior and poor self-control in children

Permissive parents are warm but make few demands, the opposite of this pattern.

B. Uninvolved; the poorest outcomes across nearly all measured areas

Uninvolved parents are low in both demands and warmth, but these parents are highly demanding.

C. Authoritative; high self-esteem and strong social competence

Authoritative parents pair high demands with warmth and explanation, which is missing here.

D. Authoritarian; anxiety, withdrawal and lower self-esteem ✓ KEY

High demands with low warmth and no explanation is authoritarian parenting, linked to anxious, withdrawn children.

Rationale: Baumrind classified parenting by demandingness and responsiveness. Authoritative parents are high in both; authoritarian parents are high in demands but low in warmth and explanation; permissive parents are warm but undemanding. Maccoby and Martin later added the uninvolved style, low in both. Authoritarian parenting is associated with anxiety, withdrawal, lower self-esteem and, in some children, defiance.

References: Berk (Development Through the Lifespan) — Early childhood: child-rearing styles (Baumrind)

☐ Approve ☐ Revise ☐ Retire Notes:

After a breakup, a 15-year-old tells her school counselor, "Nobody has ever hurt like this. My mom could never understand, because what I feel is completely different from anything she went through." In Elkind's account of adolescent egocentrism, this reflects:

A. The imaginary audience

The imaginary audience is the belief that everyone is watching and judging, not that one's feelings are unique.

B. The personal fable ✓ KEY

The personal fable is the belief that one's experiences and feelings are unique and that one is special.

C. Apparent hypocrisy

Apparent hypocrisy is voicing ideals without seeing that one's own behavior contradicts them.

D. Identity foreclosure

Foreclosure is Marcia's status of commitment without exploration, not an egocentric belief.

Rationale: Elkind described two forms of adolescent egocentrism. The imaginary audience is the sense of being constantly observed and evaluated by others; the personal fable is the belief that one is unique, so that no one else could understand one's feelings, and sometimes that one is invulnerable to harm. Both reflect the new capacity for formal operational thought turned back on the self.

References: Berk (Development Through the Lifespan) — Adolescence: imaginary audience and personal fable (Elkind)

☐ Approve ☐ Revise ☐ Retire Notes:

After a case presentation, a White supervisor tells a Black counseling intern, "You are so articulate. I wasn't expecting that." The supervisor means it as praise. In Sue's taxonomy of racial microaggressions, this remark is BEST classified as a:

A. Microassault

Microassaults are explicit, often deliberate attacks such as slurs, not well-meant comments.

B. Microinvalidation

Microinvalidations negate a person's experiences, as with color-blind statements.

C. Microinsult ✓ KEY

An ascription of intelligence that conveys surprise at competence is a classic microinsult.

D. Stereotype threat

Stereotype threat is a target's performance worry, not a type of microaggression.

Rationale: Sue described three forms of microaggression. Microassaults are explicit, usually conscious attacks. Microinsults are often unconscious communications that demean a person's heritage, such as expressing surprise that a person of color is articulate. Microinvalidations exclude or negate the thoughts, feelings or reality of people of color, such as claiming not to see race.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial microaggressions: microassaults, microinsults and microinvalidations

☐ Approve ☐ Revise ☐ Retire Notes:

Working with a Cambodian refugee, a counselor learns the client's own terms for her distress, asks how her community explains such symptoms and frames goals within those meanings. This approach reflects which perspective?

A. An etic perspective

An etic view applies concepts assumed to be universal across cultures.

B. An emic perspective ✓ KEY

An emic view seeks to understand the client from within her own culture's concepts and meanings.

C. An assimilation stance

Assimilation is an acculturation strategy of adopting the dominant culture.

D. A universalist stance

Universalism, like the etic view, emphasizes what is common across cultures.

Rationale: An etic (universal) perspective assumes that concepts and methods apply across cultures, while an emic (culture-specific) perspective seeks to understand behavior and distress through the meanings of the client's own culture. Culturally responsive counselors balance the two, using emic understanding to avoid imposing outside frameworks.

References: Sue & Sue (Counseling the Culturally Diverse) — Etic (universal) and emic (culture-specific) perspectives

☐ Approve ☐ Revise ☐ Retire Notes:

Linh, 17, was born in the United States to Vietnamese parents. At home and at her temple she has been taught to speak Vietnamese, honor her elders and keep Tet traditions, and she embraces these norms. The process by which she has been socialized into and kept her heritage culture is called:

A. Acculturation

Acculturation refers to changes from contact with a different, often dominant, culture.

B. Assimilation

Assimilation is an acculturation strategy of adopting the dominant culture over one's heritage.

C. Marginalization

Marginalization is low connection to both the heritage and the dominant culture.

D. Enculturation ✓ KEY

Enculturation is socialization into, and retention of, the norms of one's own heritage culture.

Rationale: Enculturation is the process of being socialized into and maintaining the norms, values and practices of one's heritage culture. Acculturation is the process of change that results from contact with another culture, usually the dominant one. For second-generation clients the two processes run side by side, and each can be assessed separately.

References: Sue & Sue (Counseling the Culturally Diverse) — Acculturation and enculturation in culturally diverse clients

► **Check:** Enculturation definition follows Kim & Abreu (2001); SME to confirm the Sue & Sue locator.

☐ Approve ☐ Revise ☐ Retire Notes:

A Black woman tells her counselor that she is repeatedly passed over at her firm. Her complaint was dismissed because the firm promotes many White women and several Black men. Which framework BEST helps the counselor understand her experience?

A. An additive model, summing her separate race and gender barriers

Adding single-axis barriers misses the distinct form of exclusion created where they meet.

B. Minority stress theory, applied to her single most salient identity

Minority stress concerns chronic stress from stigma; focusing on one identity misses the overlap.

C. Crenshaw's intersectionality, in which overlapping identities interact ✓ KEY

Intersectionality explains how race and gender combine into discrimination invisible to single-axis analysis.

D. Internalized oppression, seeing her concerns as self-blame to correct

Nothing suggests self-blame; recasting systemic exclusion as internal invalidates her experience.

Rationale: Kimberlé Crenshaw coined intersectionality to explain how Black women's discrimination claims fell through the cracks when courts examined race and sex separately. Overlapping identities interact to produce distinct experiences of privilege and oppression that single-axis or additive views miss, so counselors attend to the whole person and the systems acting on her.

References: Sue & Sue (Counseling the Culturally Diverse) — Multiple identities and intersectionality · MSJCC (Ratts et al., 2016) — Intersection of identities; privileged and marginalized statuses

☐ Approve ☐ Revise ☐ Retire Notes:

In a multicultural course, a White student says, "I worked hard for everything I have, so I don't have privilege." The instructor introduces Peggy McIntosh's concept of White privilege. Which statement BEST captures that concept?

A. Unearned advantages granted by systems, often invisible to those holding them ✓ KEY

McIntosh described privilege as unearned, systemic and largely invisible to those who have it.

B. Advantages held only by White people with wealth or high-status occupations

Racial privilege operates across class lines, although class shapes it.

C. Benefits that exist only when a White person holds conscious racial prejudice

Privilege operates through systems regardless of an individual's intent or beliefs.

D. Proof that White people's achievements reflect no individual effort or merit

Privilege does not deny effort; it describes advantages that exist apart from effort.

Rationale: McIntosh likened White privilege to an invisible knapsack of unearned assets, such as being able to assume one will not be followed in a store. Privilege is conferred by social systems regardless of intent or effort and is often invisible to those who hold it. Recognizing it is part of the counselor self-awareness called for in the multicultural and social justice competencies.

References: Sue & Sue (Counseling the Culturally Diverse) — White privilege and the invisible knapsack (McIntosh) · MSJCC (Ratts et al., 2016) — Counselor self-awareness: privileged and marginalized statuses

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor offers a 10-year-old a tray of sand and shelves of miniature figures, invites her to "make a world," and quietly witnesses the scene she builds. Dora Kalff developed this sandplay approach from which theoretical tradition?

A. Adlerian theory

Adlerian play therapy is Kottman's approach, built on lifestyle and mistaken beliefs.

B. Behavioral theory

Behavioral approaches target observable behavior through reinforcement, not symbolic worlds.

C. Person-centered theory

Child-centered play therapy (Axline, Landreth) is person-centered, not sandplay.

D. Jungian analytical theory ✓ KEY

Kalff built sandplay on Jung's ideas of symbols, the unconscious and the self.

Rationale: Margaret Lowenfeld created the World Technique, in which children build scenes in a sand tray with miniatures. Dora Kalff, a Swiss Jungian, adapted it into sandplay, viewing the tray as a "free and protected space" in which symbols from the unconscious emerge and the self can be expressed and healed.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: play therapy and sandplay

► **Check:** SME to confirm Gladding locator for sandplay; facts (Lowenfeld World Technique; Kalff Jungian sandplay) are standard.

☐ Approve ☐ Revise ☐ Retire Notes:

In the Adult Attachment Interview, a mother describes a childhood with a harsh, rejecting father. She recounts painful memories in a coherent, balanced way, reflects on how they affected her and shows some understanding of her father's struggles. Her interview is MOST likely classified as:

A. Dismissing

Dismissing interviews minimize attachment and offer idealized but unsupported or sparse memories.

B. Preoccupied

Preoccupied interviews are entangled and angry or vague, with long, incoherent passages.

C. Unresolved

Unresolved interviews show lapses in reasoning when discussing loss or trauma.

D. Autonomous ✓ KEY

A coherent, reflective account of adverse experiences is classified autonomous (secure), often called earned security.

Rationale: The Adult Attachment Interview, developed by George, Kaplan and Main, classifies adults by the coherence of their narrative about childhood rather than by how positive the experiences were. A coherent, reflective account of a difficult childhood is classified autonomous (secure), sometimes called earned security. Parents' AAI classifications predict their infants' Strange Situation classifications.

References: Berk (Development Through the Lifespan) — Attachment: continuity across generations and the Adult Attachment Interview

☐ Approve ☐ Revise ☐ Retire Notes:

Harlow raised infant rhesus monkeys with two surrogate mothers: one made of bare wire that held a feeding bottle and one covered in soft cloth without food. What did the monkeys' behavior show?

A. Attachment rests mainly on contact comfort, not feeding ✓ KEY

The monkeys clung to the cloth mother and ran to her when frightened, even though the wire mother fed them.

B. Attachment forms to whichever surrogate supplies food

This drive-reduction view is what Harlow's results contradicted.

C. Infants form no attachment to either surrogate mother

The monkeys formed a clear attachment to the cloth surrogate.

D. Imprinting occurs only in a narrow window after birth

Imprinting in a critical period is Lorenz's finding with geese, not Harlow's.

Rationale: Harlow's surrogate-mother studies showed that infant monkeys spent most of their time clinging to the soft cloth mother and sought her out when frightened, visiting the wire mother only to feed. The results challenged drive-reduction views that attachment arises from feeding and supported the importance of contact comfort.

References: Berk (Development Through the Lifespan) — Ethological theory of attachment; Harlow's contact comfort studies

☐ Approve ☐ Revise ☐ Retire Notes:

Core Counseling Attributes

16 items · NCE weight 8% (13 of 160 scored items)

185. nce-s-cor-049 · Social and Cultural Diversity · cultural humility · medium · In review

A counselor tells a 45-year-old Mexican American client, "It must help to have your big family around you right now." The client replies, "Actually, my family is part of the stress. My friends from work are who I lean on." Which counselor response BEST reflects cultural humility?

A. Families can be complicated, but they usually come through in a real crisis.

This holds on to the counselor's assumption about family support and quietly dismisses the client's correction.

B. I'm sorry to hear that. Maybe we can work on repairing things with them.

This sets a family-repair goal the client did not ask for and still treats family closeness as the expected norm.

C. Thank you for correcting me; I assumed. Tell me about who supports you. ✓ **KEY**

The counselor owns the assumption without defensiveness, thanks the client, and lets the client define her own support system.

D. That surprises me, since family is so central in Mexican American culture.

This applies a group generalization over the individual's own account, which is stereotyping rather than humility.

Rationale: Cultural humility treats the client as the expert on how culture shows up in her life. When a client corrects an assumption, the humble response is to accept the correction openly, acknowledge the error, and get curious about the client's actual experience. Group-level knowledge can inform hypotheses but cannot override what an individual client says about herself.

References: Sue & Sue (Counseling the Culturally Diverse) — Within-group differences; avoiding stereotyping in culturally responsive practice

☐ Approve ☐ Revise ☐ Retire Notes:

A Filipina American counselor begins working with a 58-year-old White, heterosexual, Christian man who owns a hardware store in a small farming town and is struggling after his business partner died. The counselor thinks, "His culture is the mainstream one, so there isn't much cultural material to explore." Which stance BEST reflects cultural humility?

A. Explore how his faith, rural roots, class and ideas of manhood shape his grief ✓ KEY

Everyone has a culture. Inviting him to describe how faith, place, class and gender norms shape his loss applies humility to a client from dominant groups.

B. Focus on grief alone, because culture matters mainly for marginalized clients

This treats dominant-group identity as culture-free, the very assumption the counselor should be questioning.

C. Rely on her knowledge of small-town life to anticipate his values and beliefs

Presuming to know his values from group knowledge is the opposite of the other-oriented, open stance humility requires.

D. Begin by discussing her own ethnic identity so he sees what culture involves

Centering the counselor's identity at the outset shifts focus away from the client and does not explore his worldview.

Rationale: Cultural humility is not reserved for clients from marginalized groups. Members of dominant groups also hold cultural identities, such as religion, regional and rural culture, social class and gender-role norms, that shape how they experience distress and help-seeking. Treating the majority culture as a neutral default is a form of cultural encapsulation; a humble counselor stays curious about every client's worldview.

References: Sue & Sue (Counseling the Culturally Diverse) — Everyone has a culture; Whiteness as an invisible default and cultural encapsulation · MSJCC (Ratts et al., 2016) — Client worldview: privileged and marginalized client identities

☐ Approve ☐ Revise ☐ Retire Notes:

Which description BEST captures cultural humility as distinct from cultural competence?

A. A body of knowledge about specific groups that a counselor masters in training

Mastering group-specific knowledge is closer to the knowledge component of cultural competence, not humility.

B. A credential earned after completing a set number of multicultural CE hours

Humility is an ongoing stance, not an endpoint that can be certified by hours of training.

C. A technique of matching clients with counselors who share their background

Matching may help some clients, but it is a service-delivery choice, not the counselor's own stance of humility.

D. A lifelong process of self-reflection, openness, and attention to power ✓ KEY

Cultural humility is defined as an ongoing commitment to self-reflection and self-critique, openness to the client's view, and redressing power imbalances.

Rationale: Cultural humility, a term introduced by Tervalon and Murray-Garcia, emphasizes lifelong self-evaluation and critique, an other-oriented openness to the client's cultural identity, and attention to power imbalances. It complements competence: rather than an endpoint of mastery, it is a continuing way of being with clients that keeps the counselor learning.

References: Sue & Sue (Counseling the Culturally Diverse) — Cultural humility versus cultural competence; lifelong self-reflection

► **Check:** Tervalon & Murray-Garcia (1998) originated the term; confirm the cited Sue & Sue edition discusses cultural humility in these terms.

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old man says, "I've been seeing someone else for six months. My wife has no idea. You probably think I'm a terrible person now." Which counselor response BEST reflects a nonjudgmental stance?

A. I don't think that, but you'll need to tell your wife before we can go on.

Making disclosure a condition of counseling imposes the counselor's values and sets an agenda the client has not chosen.

B. You expected me to judge you. I'd like to understand what's happening for you. ✓ KEY

This reflects his fear of judgment and conveys acceptance and curiosity without endorsing or condemning the affair.

C. Affairs are very common, so you really shouldn't feel bad about this at all.

Normalizing and reassuring takes a stance on the behavior and dismisses whatever guilt or conflict he may feel.

D. How do you think your wife would feel if she found out about all of this?

Though phrased as a question, it implies moral evaluation and shifts focus from his experience to an imagined accusation.

Rationale: A nonjudgmental stance means suspending moral evaluation so the client can explore his experience openly. It neither condemns nor approves the behavior. ACA Standard A.4.b asks counselors to avoid imposing their own values; responding to the client's fear of judgment and inviting exploration keeps the focus on his goals.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A 62-year-old client laid off from a factory job says, "Immigrants took all the jobs around here. People like that are ruining this country." The counselor, herself the daughter of immigrants, feels a flash of anger. Which response BEST reflects a nonjudgmental stance while staying ethical?

A. Losing that job hit hard. Tell me what these months have been like for you. ✓ KEY

This attends to the loss and fear beneath the statement, neither endorsing nor attacking his view, and keeps the focus on his experience.

B. I need to tell you that my own parents were immigrants, and that remark hurt.

Disclosing her hurt at this point centers the counselor's reaction and is likely to rupture the relationship before it forms.

C. The research shows immigrants don't really take jobs away from local workers.

Debating the facts turns the session into an argument and signals judgment rather than understanding.

D. It sounds like you're right that outsiders are causing a lot of your problems.

Agreeing with the generalization abandons neutrality and reinforces a blaming belief rather than exploring it.

Rationale: Nonjudgmental does not mean agreeing. The counselor sets aside her reaction in the moment, monitors it, and responds to the client's underlying experience of loss. Consistent with ACA A.4.b, she avoids imposing her values; she can process her own anger in supervision and later explore how his beliefs affect his goals.

References: ACA Code of Ethics — A.4.b Personal Values · Corey (Theory & Practice) — The counselor as a person: personal values and dealing with difficult client views

☐ Approve ☐ Revise ☐ Retire Notes:

A 67-year-old woman meeting by telehealth after eight months of sobriety says, "I drank at my granddaughter's wedding. I blew everything. I'm back at zero." Which counselor response BEST reflects a nonjudgmental stance?

A. A wedding was a risky place to be. Why didn't you use your coping plan there?

The "why didn't you" question carries blame and is likely to deepen her shame.

B. Well, eight months is good, but you'll need to start counting from day one.

This confirms her all-or-nothing view of the lapse and minimizes the progress she has made.

C. You're being hard on yourself. Let's look at what led up to it, together. ✓ KEY

This acknowledges her self-criticism without blame and invites shared, curious review of the lapse as something to learn from.

D. I'm a little disappointed, but I know you can get back on track this week.

Voicing disappointment adds the counselor's evaluation to her shame, even with encouragement attached.

Rationale: A nonjudgmental response to a lapse counters the abstinence violation effect, the guilt and "I've blown it" thinking that can turn a single lapse into a full relapse. Treating the lapse as information about high-risk situations, rather than as moral failure, keeps the client engaged and supports recovery.

References: Marlatt & Donovan (Relapse Prevention) — Abstinence violation effect; lapse versus relapse

☐ Approve ☐ Revise ☐ Retire Notes:

191. nce-s-cor-055 · Social and Cultural Diversity · awareness of power and privilege in the counseling relationship · medium · In review

A middle school counselor receives a 13-year-old sent by the assistant principal after a hallway fight. The student slumps in the chair and says, "Just write me up already. You all work for the office." Which counselor response BEST reflects awareness of power in this setting?

A. Remind the student that cooperating now will help keep the consequences lighter

Using discipline as leverage confirms that the counselor is an arm of the office and uses institutional power coercively.

B. Wait silently until the student is ready to explain what happened in the fight

Silence without explanation leaves the student's assumption about the counselor's role unaddressed.

C. Tell the student that everything said here will stay strictly between the two of them

Promising absolute confidentiality is inaccurate in a school setting and could later break trust.

D. Explain her role apart from discipline, what she would share, and offer choices ✓ KEY

Clarifying her role and the limits of confidentiality, and giving the student some control, directly addresses the power imbalance the student names.

Rationale: In schools, courts and other mandated settings, clients often see counselors as agents of the institution. Naming the counselor's role honestly, explaining what will and will not be shared, and offering real choices reduces the power differential and builds trust. Overpromising confidentiality or leveraging consequences each misuse the counselor's position.

References: MSJCC (Ratts et al., 2016) — Counseling relationship: power, privilege and oppression influencing the relationship · ACA Code of Ethics — B.1.d Explanation of Limitations

☐ Approve ☐ Revise ☐ Retire Notes:

192. nce-s-cor-056 · Social and Cultural Diversity · awareness of power and privilege in the counseling relationship · hard · In review

A 55-year-old Haitian Creole-speaking woman comes to an intake for depression with her adult son, who offers to interpret. As the session goes on, he answers some questions without asking her and gives brief summaries of her longer replies. What is the BEST course of action?

A. Continue with the son, since he knows her history and she seems comfortable

Comfort does not offset the filtering already happening; her voice and privacy are being shaped by a family member.

B. Arrange a trained interpreter, explaining it protects her privacy and voice ✓ KEY

A trained interpreter renders her words fully and neutrally, restoring her voice and confidentiality that family interpreting compromises.

C. Ask the son to translate word for word and to stop adding his own answers

Instructions do not remove the family role, power dynamics, or limits on what she can disclose in front of her son.

D. Rely on her limited English so that no third party needs to be in the room

Forcing her to use a language she does not command well silences emotional content and deepens the power imbalance.

Rationale: Language carries power: a client who cannot speak for herself in her own language loses control over her story. Family interpreters, however willing, may filter, edit or answer for the client, and their presence limits disclosure of sensitive material. Using a trained professional interpreter is the standard of practice that protects accuracy, confidentiality and the client's voice.

References: Sue & Sue (Counseling the Culturally Diverse) — Language barriers; problems with using family members as interpreters · ACA Code of Ethics — A.2.c Developmental and Cultural Sensitivity

☐ Approve ☐ Revise ☐ Retire Notes:

193. nce-s-cor-057 · Social and Cultural Diversity · awareness of power and privilege in the counseling relationship · medium · In review

A 50-year-old Black man who was once hospitalized involuntarily says at intake, "You've got all those letters after your name. The last one decided what was wrong with me and never asked me a thing." Which counselor response BEST addresses the power differential?

A. My training is extensive, so you can trust that I'll get the diagnosis right.

Leaning on credentials reasserts the expert power the client distrusts and dismisses his past experience.

B. That provider was probably following protocol, so try not to take it personally.

Defending the system invalidates his experience and aligns the counselor with the institution he distrusts.

C. Let's not focus on the past; what matters is the work we're going to do now.

Redirecting away from his experience avoids the trust issue he has raised and quietly silences it.

D. That sounds like it left you unheard. You know your life best; let's work together. ✓ KEY

This validates his experience, names him as the expert on his life, and invites a collaborative relationship that shares power.

Rationale: Credentials give counselors real institutional power, including the power to diagnose and document. Clients with histories of coercive or dismissive care, often compounded by systemic racism, may reasonably distrust that power. Acknowledging the experience and positioning the client as a collaborator and the expert on his own life helps balance the relationship.

References: MSJCC (Ratts et al., 2016) — Counseling relationship: counselor and client privileged and marginalized statuses · Sue & Sue (Counseling the Culturally Diverse) — Cultural mistrust and the credibility of the helping professional

☐ Approve ☐ Revise ☐ Retire Notes:

A 78-year-old man tells his 27-year-old counselor, "You're young enough to be my granddaughter. What could you know about burying a wife of 55 years?" Which counselor response BEST conveys empathy?

A. Age doesn't matter much here; I've been trained to work with grief at any age.

Citing training defends the counselor and sidesteps the doubt and loss the client is expressing.

B. You're right, I haven't lived that. I want to understand what losing her is like. ✓ **KEY**

This honestly acknowledges the generational gap and communicates a sincere wish to enter his world of loss.

C. I lost my grandmother last year, so I do know a little of what you're feeling.

This self-disclosure equates different losses and shifts the focus to the counselor's experience.

D. Would you feel more comfortable seeing a counselor who's closer to your age?

Offering a transfer at the first sign of doubt avoids the relationship rather than responding to his feelings.

Rationale: Empathy does not require shared experience; it requires trying to understand the client's world from his frame of reference. With a client of a different generation, the counselor can honestly acknowledge the difference and convey genuine interest in learning his experience, which often strengthens credibility more than defending one's qualifications.

References: Corey (Theory & Practice) — Person-centered therapy: accurate empathic understanding of the client's frame of reference

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at a refugee resettlement agency tells her supervisor that she cries alongside clients as they describe their losses, then feels drained and has trouble thinking clearly in session. The supervisor suggests she shift the balance of her empathy. Which shift is the supervisor MOST likely recommending?

A. More affective empathy, so that clients can see how deeply she shares their pain

She is already highly affectively attuned; adding more emotional sharing would deepen the overwhelm she reports.

B. Expressing sympathy and pity instead, since they are less taxing to the counselor

Sympathy feels for the client from outside their frame of reference and is not a substitute for empathy.

C. More cognitive empathy, grasping the client's view while keeping her own footing ✓ **KEY**

Cognitive empathy, accurate perspective-taking with emotional separateness, keeps the "as if" quality and protects clear thinking.

D. Emotional detachment, keeping reflections of feeling to an absolute minimum in session

Detachment would abandon empathy altogether and damage the relationship rather than rebalance it.

Rationale: Affective empathy is feeling with the client; cognitive empathy is accurately understanding the client's perspective. Rogers described empathy as sensing the client's world "as if" it were one's own without losing the "as if" quality. When affective resonance turns into personal distress, strengthening the cognitive side helps counselors stay present and effective while reducing burnout and vicarious trauma risk.

References: Corey (Theory & Practice) — Person-centered therapy: accurate empathic understanding and the "as if" quality · Davis (Multidimensional Empathy) — Perspective taking (cognitive) vs empathic concern and personal distress (affective)

☐ Approve ☐ Revise ☐ Retire Notes:

In person-centered counseling, offering unconditional positive regard means the counselor

A. values the client as a person without requiring approval of every behavior ✓ KEY

UPR is acceptance of the person's worth that does not depend on the client's behavior; it is not approval of all actions.

B. agrees with the client's decisions so the client feels supported and safe

UPR does not require agreeing with choices; acceptance of the person is distinct from endorsement of decisions.

C. offers frequent praise to raise the client's self-esteem in each session

Praise is conditional by nature, tied to things the counselor approves of, which is the opposite of unconditional regard.

D. keeps a neutral blank screen so that clients project feelings onto it

The blank screen is a psychoanalytic stance used to foster transference, not person-centered acceptance.

Rationale: Rogers described unconditional positive regard as a nonpossessive caring and acceptance of the client as a person of worth, without conditions. It allows clients to explore feelings and behaviors without fearing the loss of the counselor's regard. Acceptance of the person does not mean approval of every behavior.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance

☐ Approve

☐ Revise

☐ Retire

Notes:

A 23-year-old client starting treatment for panic attacks asks, "Just tell me straight. Is this going to fix me?" The counselor believes the approach is likely to help but cannot be certain it will work for him. Which response BEST demonstrates congruence?

A. Yes, this approach works for nearly everyone, so you'll be panic-free soon.

Overpromising misrepresents what the counselor actually believes and could undermine trust if progress is slow.

B. What do you think it would mean for you if this didn't end up working out?

Deflecting a direct question with a question avoids being genuine with a client who asked for honesty.

C. It's too early in our work for me to give you any kind of answer about that.

This withholds the counselor's honest, informed view, which she does in fact have.

D. I can't promise that, but many people improve, and we'll track how it goes. ✓ KEY

This matches what the counselor genuinely believes, offering honest hope without false certainty.

Rationale: Congruence means the counselor's outward communication matches inner experience. When a client asks a direct question, a genuine counselor answers honestly, which here means conveying realistic hope while admitting uncertainty. False reassurance and evasion are both incongruent and can erode the trust that genuineness is meant to build.

References: Corey (Theory & Practice) — Person-centered therapy: congruence or genuineness

☐ Approve

☐ Revise

☐ Retire

Notes:

A 48-year-old client with marked paranoid features stiffens when the counselor mentions his former employer. "How do you know where I worked? Have you been talking to people about me?" The detail came from the intake form he completed. Which response is MOST likely to protect the working alliance?

A. Reassure him warmly that she is on his side and that he is completely safe here

Effusive warmth and reassurance tend to raise suspicion in clients with paranoid features rather than ease it.

B. Gently note that his reaction may reflect a pattern of mistrust worth exploring

Interpreting his suspicion in this moment feels accusatory and is likely to deepen the rupture.

C. Calmly say it came from his intake form and offer to show him the form itself ✓ **KEY**

A plain, factual and transparent answer respects his concern and demonstrates that nothing is being hidden.

D. Apologize for bringing it up and move the conversation on to another subject

Dropping the topic without explanation leaves his suspicion intact and can look like evasion.

Rationale: Clients with paranoid features are highly sensitive to hidden motives. A straightforward, consistent and transparent style, explaining exactly where information came from and how it is used, builds trust better than warmth, interpretation or avoidance. Repairing a moment of suspicion this way strengthens the bond component of the alliance.

References: Gladding (Counseling: A Comprehensive Profession) — Working alliance: building trust and repairing ruptures · DSM-5-TR — Paranoid personality disorder: pervasive distrust and suspiciousness of others

► **Check:** The recommendation of a matter-of-fact, transparent (not effusively warm) style with paranoid clients is standard clinical guidance but is not stated in DSM-5-TR itself; DSM is cited for the feature only.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a community reentry program, herself a mother of young children, is assigned a 35-year-old man who served a sentence for physically abusing a child. She notices she is curt with him, ends sessions early, and dreads his appointments. What is the MOST appropriate action?

A. Refer him to a colleague right away, since her values conflict with his past

ACA A.11.b cautions against referral based solely on the counselor's personal values; she should first examine and manage the reaction.

B. Explore her reaction in supervision and watch how it is affecting his care ✓ **KEY**

Supervision helps her understand and manage negative countertransference so it stops shaping her behavior toward him.

C. Tell him her feelings openly so he understands the impact of his offense

Disclosing her reaction this way would be punitive and serve her needs rather than his treatment.

D. Keep the shorter sessions, since his offense limits what counseling can do

Continuing reduced care acts out the countertransference and denies him equitable treatment.

Rationale: Negative countertransference toward clients who have caused harm can show up as coldness, shortened sessions and avoidance. The counselor's task is to recognize the reaction, explore it in supervision, and keep it from compromising care. ACA A.11.b asks counselors to refrain from referring prospective and current clients based solely on personally held values; referral is for genuine lack of competence after efforts to manage the reaction.

References: Corey (Theory & Practice) — Countertransference: recognizing and managing negative reactions to clients · ACA Code of Ethics — A.11.b Values Within Termination and Referral

☐ Approve ☐ Revise ☐ Retire Notes:

At his first session, a 26-year-old graduate student from Ghana whose name is Kwabena says, "Most people here just call me Kevin. It's easier." Which counselor response BEST conveys respect?

A. I'm happy to use whichever you prefer. If Kwabena, will you help me say it? ✓ **KEY**

This leaves the choice with him and shows willingness to learn his name, conveying respect without pressure.

B. Kevin is fine with me, then. It will keep things simple for both of us here.

Accepting the anglicized name for convenience skips asking what he prefers and centers ease over his identity.

C. I'll call you Kwabena, because it's important to honor your true heritage.

Insisting on his birth name overrides his choice; respect means honoring his preference, whatever it is.

D. Do you feel people here haven't respected your culture since you arrived?

Jumping to a probing question about discrimination presumes a problem before learning what he wants to be called.

Rationale: Respect includes honoring how clients wish to be addressed. Rather than defaulting to convenience or imposing a choice, the counselor asks for his preference and offers to learn correct pronunciation. Small acts like this signal that the client's identity matters and that he directs how it is represented.

References: Gladding (Counseling: A Comprehensive Profession) — Building a counseling relationship: warmth and respect

☐ Approve

☐ Revise

☐ Retire

Notes: